

Communicating dementia across three generations within Chinese families

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Abstract of this entitled
“Communicating dementia across three generations within Chinese families”

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Background: Communication is fundamental for dementia care to address uncertainty and ambiguity. Family members play a constant role in the patients’ life and are their main communicative partners throughout the disease journey. Open and effective communication in the early stage can significantly benefit the patient and family unit in the long run. People with early stage dementia have largely retained the ability to communicate and have the desire to engage in family communication related to their condition. Additionally, demographic changes mean that adult grandchildren are increasingly likely to share caregiving responsibilities and exert influence on the family communication process. Because China features the world’s largest population of patients with dementia, it is also important to explore how the intrinsic Chinese communication culture complicates the communication process. However, how Chinese family members communicate about early stage dementia across three generations is poorly understood.

Objectives: This study adopted an intergenerational lens to examine how Chinese family members communicate about salient dementia-related issues. First, it aimed to understand three family communication processes in the early stage, including the initial decision to seek a dementia diagnosis, postdiagnosis disclosure and prognosis communication, and emotional communication. Second, it aimed to



understand the communicative role and responsibility of adult grandchildren. Third, it aimed to understand the influence of Chinese communication culture.

Methods: Guided by constructivist grounded theory, three rounds of qualitative interviews were conducted with one member in each of the three generations in the family: adult grandchildren, adult children, people with early stage dementia, and the patients' spouses. The first-round interview was conducted through convenience sampling to explore the general research idea. Then, the preliminary categories pointed to areas to explore during the second round of data collection, or theoretical sampling. Finally, I gathered third-round data to check and refine the emerging analytic categories until achieving data saturation.

Findings: A final sample of 28 people participated in this study. Research participants were recruited both online and from fieldwork settings. The family communication process was scrutinized by both the message sender and receiver. Problematic communication patterns were theorized as “disconnected communication”, subsumed by four core categories: unseeable, unspeakable, unhearable, and unmovable. In contrast, “connected communication” was seeable, speakable, hearable, and movable, with family members able to empathetically see each other, speak to each other, hear each other, and move each other to proactive action. The specific cultural context (interdependent self, belief in fate and fatalism), social context (medical system and patient autonomy), and historical context (intergenerational value differences) shaped intergenerational family communication in the Chinese context.

Conclusions: With the global paradigm shift in dementia care from the biomedical model to a more integrated person-centered model, it is imperative to involve people with early stage dementia in communication and recognize their self-consciousness. Adult grandchildren have also become more influential in family communication and care provision. Social work practice should promote connected



communication among intergenerational family members experiencing early stage dementia to enable early detection, diagnosis, and intervention and encourage open communication about the prognosis and emotional challenges. (499 words)

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Declaration

I declare that this thesis represents my own work, except where due acknowledgement is made, and that it has not been previously included in a thesis, dissertation or report submitted to this University or to any other institution for a degree, diploma or other qualifications.

Signed _____

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List of Abbreviations

AD: Alzheimer's disease

ADL: Activity of daily living

BPSD: Behavioral and psychological symptoms of dementia

C-A: Chinese American

CAT: Communication accommodation theory

CD: Communication deviance

ED: Emergency department

EE: Expressed emotion

EOL: End of life

GC: Grandchildren

GDP: Gross domestic product

GP: Grandparent

HCP: Healthcare professionals

IADL: Instrumental activity of daily living

ICD: International Classification of Diseases

ICU: Intensive care unit

MRI: Magnetic resonance imaging

PLwD: Person living with dementia

TACT: Trustworthiness, Auditability, Credibility, and Transferability



Chapter 1: Introduction

1.1. Background

Dementia is marked by severe impairment in two or more cognitive domains, when compared to the expected premorbid level of cognitive performance given the individual's age (ICD-11, 2023). Instead of a specific disease, dementia is an acquired syndrome with multiple possible causes (Gale et al., 2018). The risk of dementia substantively increases with older age, and it often starts in the seventh or eighth decade of life and worsens over time (Arvanitakis & Bennett, 2019). People living with dementia (PLwD) struggle with a variety of cognitive abilities, most notably memory, but also language, attention, orientation, judgment, and planning that manifest over three clinical stages (early/mild, middle/moderate, late/severe) (Arvanitakis & Bennett, 2019). Globally, the number of PLwD is expected to rise from 57.4 million in 2019 to 152.8 million in 2050 (Nichols et al., 2022). To date, there are no disease-modifying, pharmacologic treatments for neurodegenerative dementia (Gale et al., 2018). Clinicians from different disciplines focus on early detection of risk factors and therapeutic intervention to improve the quality of life for both PLwD and family caregivers (Gale et al., 2018). Currently, it is estimated that approximately 55 million people live with dementia, and this number will double every 20 years, reaching 78 million in 2030 and 139 million in 2050 (World Health Organization, 2021). WHO (2017) regards dementia as a global public health priority in their action plan 2017-2025.

Dementia is a chronic disease that requires longitudinal care, ongoing counseling, and continuous psychosocial support, placing a huge burden on the family, healthcare system, and society (Cantarero-Prieto et al., 2020; Gale et al., 2018; Etters et al., 2008). Family members, who help PLwD without pay, play a significant role in dementia caregiving and are critical to PLwD's quality of life (Brodaty & Donkin, 2009; Bertrand et al., 2006). However, caregivers of PLwD suffer from higher levels of depression and burden compared to nondementia



caregivers and non-caregivers (Moon & Dilworth-Anderson, 2015; Brodaty & Donkin, 2009; Bertrand et al., 2006; Pinquart & Sörensen, 2003). Dementia caregiving is not only more intense, but also associated with “ambiguous loss” (Boss, 1993) of the loved ones, who have reduced abilities to reciprocate and empathize. Therefore, primary caregivers (spouses and adult children) often require support and assistance from other family members. In recent years, there has been increasing recognition of the impact of dementia on the entire family unit (Braithwaite Stuart et al., 2022). In particular, demographic changes often mean adult grandchildren are increasingly likely to become primary caregivers and/or share care responsibilities with multiple family members (Venters & Jones, 2021). Globally, multigenerational family relations are more prevalent than the nuclear families in the 21st century (Bengtson, 2001). Grandchildren have replaced ancestors as the center of family, as stated by the neo-familism in contemporary China (Yan, 2016). The pervasiveness nature and relentless progression of dementia inevitably exert influence on the youngest generation and involve them in the caregiving process (Orange, 2001). Family dynamics across three generations are complicated and may be altered by dementia, posing barriers to effective health communication and care (Newman et al., 2017). Therefore, this study will adopt an intergenerational perspective to examine the provision of dementia care, specifically focusing on the experiences and perspectives of adult grandchildren.

Communication is fundamental for dementia care (Williams et al., 2022). It not only helps to transmit information but also provides mechanisms to connect with others, and establish, maintain, or change relationships (Watson et al., 2012). The ambiguity of dementia coupled with complex family dynamics necessitate health communication among multiple family members, PLwD, and professionals to maintain control over decision-making, help PLwD access appropriate care, and ensure the family’s quality of life. However, communication can be difficult since PLwD suffer from cognitive-communication impairment with declined language abilities over time, and family caregivers frequently lack the communication skills



and knowledge to manage communication problems (Watson et al., 2012). Nonetheless, early stage PLwD can still maintain conversations and largely retain the ability to communicate (Thoft & Ward, 2022). Various intervention and education programs can also improve caregivers' communication skills effectively (Morris et al., 2018; Swan et al., 2018). Therefore, effective communication with early stage PLwD is highly possible and desirable; if left undiscussed, family members may face greater difficulties in providing quality care and be more stressed when PLwD completely lose communication ability in the later stage.

In addition, in the context of Chinese families, communication about dementia across generations is complicated by the intrinsic Chinese communication culture, such as preference for implicit communication, non-verbal communication, and face-directed communication strategies, and so on. Deeply influenced by Confucianism, the high-context communication culture and collectivist orientation render Chinese communication subtle and even ambivalent, which may lead to difficulties in understanding each other and communicating dementia-related issues. As China hosts the world's largest aging population as well as number of people living with dementia, it is imperative to understand how health communication is practiced within families, the socio-psychological processes involved, and the impact of culture (Jia et al., 2020). Therefore, examining the communication process among early stage PLwD and intergenerational family members in the Chinese context is highly important.

1.2. Research Gap and Study Objective

There are three research gaps. First, previous studies mostly take communication breakdown with PLwD as the starting point of research and elicit caregivers' experiences and viewpoints to repair and improve communication skills or to measure its impact on caregiver burden (Purves & Phinney, 2012). These studies are constructive in understanding how dementia-related communication problems burden family caregivers (Savundranayagam et al., 2005) and what interventions



can effectively help improve communication skills for both PLwD and caregivers (e.g., Savundranayagam & Orange, 2014; Small & Ann, 2012). However, communication is seldom considered a process in which family members communicate about salient issues related to dementia. Epstein et al. (1993) proposed that the communication process includes three sub-concepts: clear and consistent messages, open emotional expression, and collaborative problem-solving, which altogether help stimulate family functioning and adaption towards dementia (Kim et al., 2018; Walsh, 1998). Therefore, it is important to understand how families solve problems by communicating initial symptoms, initiating dementia screening, disclosing the diagnosis to others, managing emotional arousal, and so on. Second, although previous studies involve PLwD, they usually designate a single individual (the primary caregiver, usually the spouse or adult child) as the “family caregiver” and isolate PLwD from the broader family network (Braithwaite Stuart et al., 2022; Hicks & Lam, 1999). Since primary caregivers provide the majority of care, investigating the dyadic communication or two-generational interaction is crucial to ensuring caregiving quality (Kokorelias et al., 2022). However, the responsibility and role of younger family members, particularly adult grandchildren, in preserving family relationships and facilitating communication is of greater importance given the intensive caregiving demand and the value of intergenerational relationships (Braithwaite Stuart et al., 2022). Yet, how family dynamics, conflicts, and resilience across three generations facilitate open communication and positive problem-solving receive little attention (Kim et al., 2018). Third, most literature on dementia communication is based on Western societies, without considering the unique communication culture present in non-Western cultures influenced by collectivist and family-oriented values (e.g., in China, Japan, and Korea). Neglecting the particularities of communication practices and their cultural roots may prevent family members across generations from effectively communicating dementia-related issues, making decisions, or expressing emotions.



Given that early stage PLwD largely retain the ability to communicate and the importance of family communication in countering the uncertainty and ambiguity associated with dementia, this study will contextualize key communication tasks in the family settings and explore how intergenerational family members communicate with each other and early stage PLwD under the Chinese communication culture. In summary, this study aims to:

1. Understand three family communication processes: (1) initial decision to dementia diagnosis, (2) how family inform others about the diagnosis and discuss the uncertainties and (3) emotions arousals, among family caregivers, early stage PLwD and other family members.
2. Understand the communicative role and responsibility of adult grandchildren, in relation to adult children and PLwD during the three communication processes.
3. Understand the influence of Chinese communication culture on family communication about dementia across three generations.

The structure of this study is as follows. **Chapter 2** is the literature review. It first introduces the complexity of dementia and caregiving stakeholders, family dynamics, and intergeneration relationships, both globally and in China. Then, it focuses on the family communication process in dementia and other diseases and conditions, as well as Chinese communication culture. Then, it reviews the three existing theoretical perspectives, the components, and critiques of each theory. **Chapter 3** discusses the epistemology, research question, and methodology, methods, and the demographic information of the research participants. **Chapter 4** presents the key research findings on what, how, and why intergenerational family communication becomes problematic using four core categories: unseeable, unspeakable, unhearable, and unmovable. **Chapter 5** concludes the study with a discussion, practical implications, reflections, strengths, and limitations.



Chapter 2: Literature Review

2.1. Dementia and Care Relationships

In this section, I first explain the complexities of dementia and then argue for the necessity of health communication between PLwD, their families, and healthcare professionals. Then I summarize the existing literature on dementia communication by topics, settings, and communication partners and identify the gaps. Healthcare professionals, spouses, and adult children are key communication partners, and their care relationships with PLwD are demonstrated. Despite the importance of healthcare communication, the family remains the most important partner throughout the disease journey. Therefore, this section concludes with a discussion of family dynamics and intergenerational relationships in dementia caregiving, with an emphasis on the role of grandchildren in the family communication process in Chinese families.

2.1.1. Complexity of Dementia

Dementia is marked by uncertainty. Even the definition of dementia is unsettled and evolving, with multiple coexisting definitions (Portacolone et al., 2014; George et al., 2013, 2011). Since there are no definitive biomarkers for dementia, patients with suspected dementia are often described as “probable” to reflect this uncertainty (Ennis et al., 2019). With over 100 sub-types, each type of dementia can have heterogeneous profiles of cognitive and non-cognitive symptoms, adding difficulty to dementia diagnoses (Aldus et al., 2020). The insidious nature of dementia might also leave cognitive decline unrecognized and possible active interventions neglected (Zhao et al., 2016). In addition, common disorders in older people, such as depression, apathy, and dementia, frequently overlap, and each can be mistaken for the other two or confused with other psychiatric disorders, resulting in phenomena such as pseudodementia, pseudo-pseudodementia and pseudodepression (Brodaty & Connors, 2020). Depression can be either a risk factor for dementia, predementia syndromes of cognitive decline, or psychological



responses to the diagnosis of dementia (Muliyala & Varghese, 2010). The interaction between physical declines and depression enhances the long-term risks of cognitive impairment and dementia, further compounding the presence of dementia symptoms, formal diagnosis, and disease management (Seitz-Holland et al., 2023).

Therefore, dementia diagnosis is a long process, fraught with uncertainties, that sometimes takes many years (Gauthier et al., 2021). Many PLwD remains undiagnosed, with diagnosis usually occurring long after signs and symptoms are evident, especially behavioral problems (Gamble et al., 2022). Globally, Alzheimer's Disease International (2021) estimated that 75% of PLwD are not diagnosed, and in some low- and middle-income countries, this rate can be as high as 90%. Even in high-income countries with advanced healthcare services, diagnosis rates (20–50% in primary care) remain relatively low (Prince et al., 2011). Early dementia detection is particularly challenging since it is difficult to distinguish it from normal cognitive aging and other psychiatric symptoms in early stages (Gauthier et al., 2021). Although early diagnosis may help families better prepare for care strategies and maximize patients' participation (Bradford et al., 2009), ethicists (e.g., Jodi Halpern) also question the benefit of early diagnosis considering its undefined causes and untreatable symptoms. In addition, early diagnosis can result in certain disadvantages, such as depriving PLwD's autonomy in decision-making and social exclusion due to stigma (Portacolone et al., 2014). Even with a formal diagnosis, the benefits of early diagnosis may not be realized as family caregivers often report a lack of information for continued support from professionals and difficulty understanding and adjusting to behavioral, physical, and psychological changes in PLwD (Stokes et al., 2014; Brodaty et al., 1990). They also need the knowledge to interpret the patient's behavioral cues, or they may misattribute PLwD's behavior and add further to their own distress (Savundranayagam & Orange, 2014; Polk, 2005).



The ambiguity of dementia screening, diagnosis, and symptoms necessitate clear and constant health communication among family members, PLwD, and professionals to maintain control over decision-making, help PLwD access appropriate care, and ensure the family's quality of life. However, paradoxically, one area of functioning that is most profoundly changed by dementia is communication, as a result of a progressive decline in cognitive abilities and language abilities (Banovic et al., 2018). Language difficulties occur both at the semantic level (the meaning of the words) and pragmatic level (the ability to adapt language to specific social interaction), and such problems worsen as the disease progresses (Banovic et al., 2018; Richter et al., 1995). Caregivers are usually unprepared for such communication breakdowns and report feelings of loneliness, anger, frustration, and mourning for the loss of conversation and declined relationship quality (Murray et al., 1999; Small et al., 2000). This also negatively affects PLwD's sense of self and personal wellbeing and reduces the quality of interaction and relationships with others (Perkins et al., 2022; Pozzebon et al., 2016). PLwD commonly experience difficulties expressing their needs and participating in conversations (Young et al., 2016). In Alzheimer's disease (AD), for example, changes in language and communication emerge early in the course of the disease, most often being manifested in word-finding difficulties in social communication, such as day-to-day interaction with family and friends (Bayles & Kaszniak, 1987). Repetition and temporary digressions from the conversation topic are common due to deteriorating working memory and pragmatic competence (Orange, 2001; Garcia & Joannette, 1997; Mentis et al., 1995). Nonetheless, early stage AD patients are still capable of participating in and helping to maintain conversations and can understand most spoken and written language unless the syntax, grammar, or storyline is highly complex (Orange, 2001). In the middle stage, AD patients show pronounced anomia, and everyday communication problems increase in magnitude (Kemper & Hummert, 1997). They may output more socially inappropriate language, use stereotyped social greetings, provide little meaning in their expressed language due to semantic emptiness, and become lost in multi-partner conversations



(Orange, 2001; Watson et al., 2012). In the later stages, AD patients display various communication performances, such as mutism, producing nonsense words and utterances, relying heavily on nonverbal communication, and eventually losing the ability to communicate. Dementia-related communication changes create everyday life disruptions and take a severe toll on family relationships as well as the emotional physical and psychological well-being of both PLwD and family members (Nickbakht et al., 2023). Families frequently regard communication challenges as one of the most challenging aspects of dementia (Nickbakht et al., 2023).

The need to communicate, even at a very basic level, is central to human relationships and continues throughout the course of dementia (Orange et al., 1998; Richter et al., 1995). Communication is not simply a matter of speaking and listening but a symbolic exchange process that is indissolubly linked with our deeper sense of being human in social life (Chen & Miike, 2006). Furthermore, effective health communication is important in delivering high-quality health care, including both self-care and shared care (Kreps, 2018). Although communication among PLwD and involved parties (e.g., family members, healthcare professionals, nurse and care staff, and community members) can be challenging (Banovic et al., 2018), PLwD should not be prematurely excluded from communicating dementia-related issues, as this may leave them disempowered and disassociated. Eliciting views and feelings from PLwD despite their disability are at the core of personhood and interpersonal values and can be achieved with humane and appropriate approaches (Aggarwal et al., 2003). Family members who act as surrogates for PLwD cannot consistently and accurately identify their values and thoughts (Black et al., 2013). Instead, PLwD can reliably report their care values and preference, well-being, and quality of life even in middle-severe dementia (Whitlatch et al., 2009). In particular, people with mild dementia can describe and share their lived experience of dementia and their strategies to manage life and illnesses (Thoft & Ward, 2022; Steeman et al., 2007); they are also capable of successfully resolving



communication breakdowns with caregivers despite declining cognitive, linguistic, and conversation abilities (Orange et al., 1998). In recent years, a growing body of research has considered dementia communication as a collaborative, communicative challenge rather than focusing on individual deficiencies only (Jones, 2015; Shakespeare, 1998). It has been found that PLwD's linguistic deficits vary depending on the context, goals, and complexity of the interaction (Small et al., 2000). The co-construction of communication with PLwD is contingent on the skills and knowledge of communicative partners and their support, compassion, and care (Banovic et al., 2018; Young et al., 2016). The collaborative nature of conversation highlights that PLwD's ability to produce meaningful conversations is not just a function of their own cognitive ability but is also interactionally produced (Kindell et al., 2017). Sadly, some relatives expressed lack of communication motivation and no longer seek to communicate together due to the PLwD's communication difficulties which often lead to annoyance (Braithwaite Stuart et al., 2022). Understanding communication as collaborative and interaction problems as co-constructed helps caregivers to pay attention to the contextual resources and environmental prompts to scaffold and support PLwD communication using their retained pragmatic abilities (Jones 2015; Hydén, 2014, 2013, 2011).

Communication with early stage PLwD about important dementia-related issues is possible and desirable. Although there are changes to communication with PLwD, this does not result in a complete loss of ability to communicate (Killick & Craig, 2011). Family members should try to involve early stage PLwD in decision-making processes as much as possible (Kokorelias et al., 2022). PLwD in the early stage also recognize themselves as having agency and may desire more say in decision-making (Hamann et al., 2011; Adler, 2010). However, existing research in linguistics and speech therapy neglects these communication issues and how family members and PLwD can discuss important matters together, nor do they examine how caregivers communicate with other family members and the related



consequences on family resilience and problem-solving abilities (Kim et al., 2018). In conclusion, the complex nature of dementia necessitates health communication among PLwD, family members, and professionals to manage the uncertainties effectively, and it is possible to involve PLwD in the early stage in this communication process but research into this, as well as guidelines and strategies, are lacking.

2.1.2. Key Communication Tasks During the Dementia Experience

Table 1 summarizes the existing literature on communication about dementia in a continuum as the condition progresses by different healthcare and community settings. Under each topic and setting, I list the communication participants in the particular study. Communication can happen between PLwD and healthcare professionals (HCP), sometimes with family caregivers; it can also be family caregivers with HCP, nurses, or care staff only; or communication can occur at home with the primary caregiver or other family members. The following section introduces the key communication partners and their caregiving relationships with PLwD, including HCP, spouse, adult children, and grandchildren. Family dynamics and intergenerational relationships are also addressed here to reflect the complex roles and responsibilities of different family members as the condition progresses.

Table 1. Summary of Existing Literature on Dementia Communication Topics, Locations, And Partners.

Communication topics	Where did the communication happen?			
	Inpatient	Outpatient	Nursing home	Home
1 Memory symptoms (before diagnosis)		PLwD and HCP (2004) - Family and HCP and PLwD (2010)		



	Communication topics	Where did the communication happen?			
		Inpatient	Outpatient	Nursing home	Home
2	Diagnostic disclosure (from HCP to family, and within family members)		- Family and HCP and PLwD (2019) - PLwD and HCP (2000)		PLwD and multiple family members (2015)
3	Illness uncertainties and information seeking		- Family and HCP and PLwD (2020) - Family and nurse (2013)		
4	Genetic risk and testing (Younger-onset dementia)				PLwD and spouse, sibling, children (2023)
5	Risk communication (safety concerns, e.g., falls, leaving home unsupervised, home appliance accidents)		Family and HCP and PLwD (2019)		- Family and PLwD (2018)
6	Pain management		Family and HCP (2022)		
7	Hospital/ICU/ED communication	- PLwD and HCP/nurse (2017-2022) - PLwD and nurse (2021)	PLwD and family (2022) [ED]		
8	Family acceptance and adaption				Among family members (1998, 1996)



Communication topics	Where did the communication happen?			
	Inpatient	Outpatient	Nursing home	Home
9 Sibling caregiving arrangement				Among siblings (2022)
10 Everyday care (ADL, IADL, e.g., bathing, eating, medication)			PLwD and staff (2020, 2016, 2011)	PLwD and spouse (2013, 2003, 2000)
11 Relationship negotiation (Negotiating position, identity work, narrative construction, meaning making)				PLwD, spouse, adult children, family members (2022, 2016, 2012, 2011, 2009, 1996)
12 Relationship maintenance				- PLwD and grandchildren (2019, 2018, 2017) - PLwD and spouse (2018)
13 Coping Covid-19				PLwD and adult children (2023)
14 Emotion management			PLwD and adult children (2020)	
15 Formal support arrangement				Family and social worker and PLwD (2016)
16 Home service provision				PLwD and home care staff/HCP (2022, 2021)
17 Moving to nursing home			PLwD and spouse,	



Communication topics	Where did the communication happen?			
	Inpatient	Outpatient	Nursing home	Home
			children (2016)	
18 Nursing home collaboration			Family and staff (2021)	
19 End-of-life care		PLwD and HCP (2018) [Chinese American]	Family and staff/HCP (2016, 2015, 2014)	
20 Social support and friendship	Online space to share experience: Online communities for PLwD (Johnson, 2020); PLwD and caregiver blog (Harper, 2019); Facilitated friendship (Ward, 2011)			

Note. HCP: Health Care Professionals; ED: Emergency Room; ICU: Intensive Care Units

2.1.2.1. Healthcare Care Professionals and Formal Caregivers

Three out of four communication settings are healthcare locations: inpatient, outpatient, and nursing home, which mostly involve HCP and formal caregivers. Effective healthcare communication enhances positive patient outcomes, such as treatment adherence, illness recovery, and the doctor-patient therapeutic relationship, leading to patient empowerment (Street et al., 2013). As summarized in **Table 1**, healthcare professionals play an important role in the diagnostic period and disease management, including discussing the early memory loss symptoms, informing the diagnosis and necessary resources, going through the uncertainties of the disease with the family, and managing pain and symptoms of hospitalized PLwD. HCPs are also concerned about daily risks for PLwD, such as poor personal hygiene, medicine mismanagement, leaving home unsupervised, abuse, home appliance accidents, and so on. (Taylor et al., 2018). When PLwD need formal caregivers or are situated in nursing homes, care staff are crucial communicative partners in addressing daily needs, providing quality care, and initiating difficult



conversations such as end-of-life arrangements with the family. However, in the past 20 years, the traditional biomedical approach and the authority of professionals over dementia care have been increasingly challenged (Downs, 2000). Kitwood's (1997) person-centered care and the "care for the carer approach" emerged, emphasizing the subjective experience of PLwD and the physical and psychological needs of informal caregivers (Adams & Gardiner, 2005). These two approaches further converged to "relationship-centered care" (Keady & Nolan, 2003; Nolan et al., 2003), with PLwD and family caregivers placed at the center of policy and research (La Fontaine & Oyebode, 2014). In recent years, the "partnership approach" (Adams, 1999) or "triadic interaction" (Fortinsky, 2001) among professionals, PLwD, and family caregivers has received increasing recognition due to the interdependencies and reciprocities of "dementia care triads" as the condition progresses (Adams & Gardiner, 2005). Therefore, as shown in **Table 1**, most healthcare communication involves caregivers, either in three-way patient-companion-professional communication, provider-caregiver communication, or communication between caregiver and PLwD. Caregivers can compensate for the cognitive loss of PLwD and provide HCP with information and observation through their consistent caregiving involvement (Hao & Ruggiano, 2020). Communication and collaboration among patient, companions, and professionals is crucial for successful integrated dementia care and patient empowerment (Smith et al., 2021).

However, the presence of family caregivers also poses challenges to triadic healthcare interactions. Healthcare professionals struggle to balance divergent communicative needs and conflicting roles between patients and caregivers (Robinson et al., 2010). For example, caregivers, rather than the patients themselves, are more likely to initiate discussions about dementia, deliver biomedical information about the patient, and have more corridor conversations with professionals before diagnosis delivery (Sugarman et al., 2007; Hunsaker et al., 2010). PLwD are also frequently aware of their incompetence in interactions and may attempt to save face using compensatory strategies (e.g., emotional reactions,



blaming lack of attention) and decreased involvement (Dooley et al., 2015). As a result, “protective caregivers” often speak twice as much as patients in primary care, which may lead to patient disempowerment and frustration (Sabat et al., 2011). In addition, professionals noticed communication dilemmas while interacting triadically. For example, in diagnosis delivery, HCPs should not use mitigating language which could leave room for patients and caregivers to misinterpret the diagnostic situation, yet nor should HCPs use overly straightforward approaches to improve understanding since doing so may result in negative emotions and conflict. Notwithstanding, these power inequalities, family tensions, and acceptance issues must and can only be solved through better communication between PLwD and family caregivers. Although it is important to understand how patients and HCP communicate, such communication is mostly limited to the diagnosis and institution stages. Family, however, plays a constant role in the patient’s life and is the main communicative partner in the “long haul” of the disease journey. Thus, the role of family in the patient’s everyday life is worth further investigation (Hao & Ruggiano, 2020).

2.1.2.2. Primary Caregivers and Family Dynamics

Table 1 summarizes the communication partners in the disease journey, in which family members, primarily spouses and adult children, are indispensable in different communication topics and settings. Spouses are the predominant care providers for PLwD and have a central role in dealing with the challenges of dementia (Gilbert et al., 2023; Braun et al., 2009). In the United States, more than 40% of dementia caregivers are spouses (Alzheimer’s Association, 2016). This number is estimated to continue to increase due to longer life expectancies, declining birth rates, and the resulting low availability of adult child caregivers (Williams et al., 2018). PLwD who share a positive life history with their spouse tend to be emotionally connected, work together on dementia-related problems, and make meaning of what is happening together; whereas others lacking such history experience strained relationships, work apart on problems, and try to minimize



and deny dementia related emotions and changes (La Fontaine & Oyeboode, 2014). Dementia poses challenges for spousal caregivers in coping with the emotional impact of behavioral changes, and they are more likely to become disengaged from their social networks due to the demand for care (Donnellan et al., 2017; Pozzebon et al., 2016). Despite the influence of previous marital quality, dementia also significantly impacts spousal relationships, such as communication, reciprocity, and opportunities for shared activities after diagnosis (Ablitt et al., 2009). The feeling of “loss of partner” is central to the day-to-day experience of spouses living with PLwD (Pozzebon et al., 2016).

While it is well evident that marital relationships become increasingly positive and strong as couples enter old age (Brubaker, 1990; Carstensen et al., 1995), and communication skills are the main predictors of marital satisfaction and relationship quality (Weiss & Heyman, 1997), how marital satisfaction and communication are affected when one has cognitive or physical impairment can be very different. In the context of dementia, for example, Gallagher-Thompson et al. (2001) found that caregiving dyads tend to have less interaction and expression of support than non-caregiving couples. Nonetheless, caregiving wives were most facilitative when planning care and daily tasks, and AD husbands had the most interactions to build rapport (e.g., smiling) during that task, indicating differentiating interaction patterns, problem-solving skills, and emotional closeness among the afflicted couple. Spousal caregivers also reported relationship strain due to PLwD’s communication impairment, which may lead to misunderstandings, conflict, isolation, and loss of intimacy (Williams et al., 2018). Reduced marital satisfaction may result in neglecting PLwD and even violence (Balfour, 2014). As such, many studies have examined spousal relationships with dementia and have developed interventions to improve meaningful interaction, emotional connection, and mutual support, reduce conflict and negativity, and enhance relationship equality (Gilbert et al., 2023).



Although spouses are usually the initial primary caregiver, their adult children often become more involved in caregiving as their parents grow older and have declining health (Kokorelias, 2022). Adult children usually work as “gatekeeper communicators” for their older parents with dementia, deciding what information to disclose and how to explain their parents’ behavior, explaining PLwD’s communication to others, and coaching others to communicate with PLwD more effectively (Miller-Ott, 2018). In terms of role allocation, adult children tend to be care managers in charge of caregiving arrangements, while spouses are often care providers (Brodaty & Donkin, 2009). However, the overall burden was higher among adult children than the spousal caregivers, in order of husbands, wives, daughters, and then sons showing increasing burden and worsened mental health (Conde-Sala et al., 2010). Adult children felt guilty for not living together with PLwD (Conde-Sala et al., 2010), and they struggled more in making decisions for PLwD than spouses (Samsi & Manthorpe, 2013). These findings suggest that adult children and spouses have different perceptions of the caregiving role, with spouses tending to regard caregiving as a marital duty, but adult children seeing these tasks as important changes to their lifestyle (Conde-Sala et al., 2010). Recently, more research has examined the experience of adolescents (11-18 years) and young adult children (19-35 years) caring for their parents with dementia, especially in young-onset dementia cases (e.g., Chirico et al., 2021). These young caregivers more frequently brought up issues in family relationships, such as their changing role as children and their efforts to keep the family together (Chirico et al., 2021). In contrast to the dyadic-focused spousal caregiving relationship, adult children report more tension and distress with other family members, such as denial, different understandings of dementia, different understanding and approach to caregiving, and communication breakdowns (Tatangelo et al., 2018).

Although dementia involves and impacts other family members, such as siblings, in-laws, grandchildren, and other relatives, the overwhelming majority of studies focus on the relationship and communication between the spouse/adult children and



PLwD alone rather than the roles, impact and skills of the entire family network (Smith et al., 2022; La Fontaine & Oyeboode, 2014). Research has rarely considered how intergenerational family relationships mediate and manage the impact of dementia, despite ample evidence demonstrating that multiple generations are affected (e.g., Tolkacheva et al., 2010; Garwick et al., 1994). Previous family relationships, family history, family organization, family communication, caregiving motivation, and the family vision of the future all shape family dynamics and influence the caregiving process (Esandi et al., 2021). This is exemplified by Carey Henderson, an author with dementia, who emphasized the importance of family in the context of dementia:

“One of the things about this — it’s in the family, and the family has not only me and my wife, but we have our children and our children have their spouses...In other words, this thing about Alzheimer’s is not just about two people. It’s about a whole mess of people” (Henderson & Andrews, 1998, p. 65).

Informal caregiving is a complex and interdependent phenomenon involving the decisions and actions of multiple family members, and also can be influenced by community and social factors (Ihara et al., 2012). Given the increasing recognition of the impact of dementia on the family unit, other family members, particularly grandchildren, should be involved to bridge the gap between PLwD and their circle of family members (Braithwaite Stuart et al., 2022; Portacolone et al., 2014).

2.1.2.3. Grandchildren and Intergenerational Relationships

Family Structure and Intergenerational Solidarity

The family system is composed of various lateral (e.g., affinal) and vertical (e.g., intergenerational) ties (Becker & Steinbach, 2012). Bengtson (2001) argued that multigenerational family relations would be more important than nuclear family relations in the 21st century. Major social change has significantly transformed the family structure in the past few decades and proved Bengtson’s argument (Bernal



& De la Fuente Anuncibay, 2007). Although Bengtson's theory is situated in the United States, multigenerational ties are of greater importance globally (Blanton, 2013). Due to the declining mortality, "top-heavy" families with more grandparents than grandchildren have become more common (Hagestad, 2006). An increased life expectancy leads to a greater confluence of multiple generations and lengthens the duration of family ties, especially the grandparent-grandchildren (GP-GC) relationships (Bernal & De la Fuente Anuncibay, 2007; Hagestad, 2006). The parent-child relationship may last 6 to 7 decades, and the GP-GC relationship may last for 3 to 4 decades (1 to 2 decades can be grandparent and adult grandchildren), rendering vertical family ties along generational lines more durable, stable, and important than horizontal ones (Hagestad, 2006). These extended kinship networks represent a latent and rich source of help that can be activated in times of need (Silverstein et al., 1995), but also indicate prolonged years of caring for dependent older family members and potentially longer years of family conflicts across generations (Bengtson, 2001). Meanwhile, as fertility declines, the intergenerational structure becomes less "bottom-heavy," and the number of siblings or cousins in each generation is shrinking (Hagestad, 2006). The shape of the family structure grows long and thin into a 'beanpole family', and the long year of shared lives increases the need for interaction, support, and mutual influence across more than just two generations (Bengtson, 2001).

The increased number of generations and decreased absolute number of relatives have strong implications for intergenerational solidarity, especially as the GP-GC relationship becomes more socially prominent and personally meaningful (Harper, 2005). It is not uncommon to see a granddaughter helping their mother with the housework, so their mother can spare time to care for their grandmother (Romøren & Hagestad, 1988), grandparents encouraging their children by telling them they are doing a good job in taking care of the grandchildren (Hagestad, 2006), grandchildren better understanding their parent's thoughts by conversing with grandparents (Streltzerb, 1979), or conflict between grandchildren and parents



negatively affecting the GP-GC relationship (Monserud, 2008). Recent evidence also shows that the middle generation is not merely a “lineage bridge” (Hill et al., 1970). While parental attitudes toward grandparents may be transmitted to children and persist in their lives, a positive connection between GP-GC in emerging adulthood can also influence grandchildren’s provision of care for their own parents in established adulthood (Hwang et al., 2022). The triadic interactions between grandparent, adult children, and grandchildren are inherently dynamic and complex, with a natural entanglement of values, feelings, and attitudes across generations (Stelle et al., 2010). In summary, intergenerational relationships, in the format of both help given or received, solidarity or conflict, will become more important and prevalent in modern family life (Bengtson, 2001).

Grandparent-Grandchildren Relationships

Family research has traditionally focused on the early stages of individual and family life span and neglected aging grandparents, who are viewed as dependent and contradict the norm of self-reliance in the West (Barranti, 1985). Before the mid-20th century, independent and reciprocal GP-GC relationships separate from the middle generation were an exception due to the limited time GP-GC shared together (Robertson, 1975). It was not until the 1960s that the role of grandparents came into research, with an emphasis on their voluntary bonds with grandchildren and a “roleless role” in the absence of sanctioned norms, obligations, and prescribed expectations (Barranti, 1985; Clavan, 1978). Due to increased marital instability and decreased functioning of the nuclear family, grandparents play an increasingly important role in fulfilling family functions in the 21st century (Bengtson, 2001). It is important for grandparents to simply “be there” representing a “reserve army” and “protection mechanisms” for the family in times of crisis (Hagestad, 2006). A growing body of research explored grandparents raising grandchildren, which evolved from focusing on the stress, strain, and costs associated with grandparenting to a more balanced view of the exchange of support across generations comprising of both cost and reward (Hayslip & Kaminski, 2005). Until



recently, research into early adult grandchildren found that the GP-GC relationship is a continuous source of mutual assistance, including emotional, financial, and instrumental support (Scharf, 2016). Emotional closeness, relationship quality, communication, and willingness to maintain the relationship all contribute to long-term GP-GC solidarity despite geographical and generational distances (Michałek-Kwiecień, 2022).

The GP-GC emotional attachment, formed in early childhood and developed throughout life, is unique as it is exempt from psycho-emotional intensity and parent-child responsibility (Kornhaber & Woodward, 1981). Grandparents' unconditional love, nurturance, protection, and acceptance of grandchildren confer a natural source of social immunity that grandchildren cannot obtain from anyone else, even parents (Kornhaber & Woodward, 1981). Both GP-GC make great efforts to protect their relationships, such as developing "silent pacts" in conversation topics, such as avoiding discussing sexual topics, religious belief, or racial issues (Hagestad & Kranicfeld, 1982). Although GP-GC may decrease contact as the grandchildren transit into adulthood, their close emotional bonds are continuous, and the support inherent in their relationships can be reactivated in times of need (Wetzel & Hank, 2020). Their relationship quality is relatively stable over time and not usually influenced by their socioeconomic characteristics (e.g., employment, wealth) or the challenges of modern society (e.g., social mobility) (Becker & Steinbach, 2012). These strong emotional bonds between GP-GC render it one of the most important relationships for both parties (Miron et al., 2019). Not only do grandparents play an important role in grandchildren's life, but grandchildren also act as social reference points for grandparents, preserving their memory, providing them with a sense of continuity, increasing their sense of well-being and morale, and facilitating communication and interaction (Miron et al., 2019; Barranti, 1985). Adult children can be an important source of intergenerational solidarity and combat the challenge of the aging population, but few studies have examined the



intergenerational relationships in the caregiving contexts across three generations of adults (Mebane & Pezzuti, 2020).

Grandchildren as Auxiliary Caregivers

In the caregiving environment, grandchildren bring relationship histories with their grandparents to care and companion (Stelle et al., 2010). Those with positive history with grandparents have higher levels of cohesion and lower levels of conflict, are most voluntarily involved in caregiving, and are more likely to describe their caregiving experiences as gratifying and rewarding (Blanton, 2013). Their established feelings of attachment enable them to maintain relationships and develop coping strategies (Fruhauf et al., 2006; Orel, 1999). On the contrary, grandchildren who have poor intergenerational relationships with grandparents preceding the appearance of age-related frailty are less involved and focus more on the stressful and conflictual aspects of caregiving (Blanton, 2013). Regarding caregiving roles, primary caregivers manage day-to-day caregiving demands and bear the greater emotional burden of slowly losing a parent (Blanton, 2013). As auxiliary caregivers, grandchildren primarily contribute more time and energy to the emotional and social needs of their grandparents and even their parents (Blanton, 2013). Grandchildren maintain companionable relationships, bolster spirits, prevent grandparents from saying and thinking things negatively, preserve their dignity, calm fears, create pleasant experiences, and talk about past positive memories (Piercy & Chapman, 2001). Early research has also found that adolescents seek to help their institutionalized grandparents in a positive way (Streltzerb, 1979). Although they occasionally provide instrumental help (e.g., ADL, IADL), grandchildren are aware that the demands placed on them for physical care are much less than those of their parents (Blanton, 2013). Grandparents also identified grandchildren as an important source of positivity as they do not treat them differently due to dementia, which is strikingly different from how they may treat their adult children (Dooley et al., 2021). They enjoy the company of grandchildren as they alter low moods and bring reciprocal joy and support (Dooley et al., 2021).



However, while grandchildren regard their caregiving involvement as beneficial to the patient and the family, they also experience complex emotions such as anger, frustration, and sadness mingled with compassion, grief, and fear, indicating the impact of the caregiving burden (Orel, 1999). Sometimes they are caught in the middle of conflictual family relationships, receiving their mother's stress and frustration, act as their mother's confidants, or they are patronized by family members and even excluded from the caregiving experience (Blanton, 2013; Orel et al., 2002; Creasey et al., 1989). It is worth mentioning that the roles of grandchildren as auxiliary caregivers are inherently heterogeneous, with different stages in childhood, adolescence, emerging adulthood, and established adulthood, each with different abilities to adapt to the caregiving environment (Venters & Jones, 2021).

In particular, when the grandparent is affected by neurogenerative diseases such as dementia, Perion et al. (2021) argued that the experience of grandchildren in the secondary caregiving role is more positive in nature and may help them forge closer family relationships (Celdran et al., 2012). Grandchildren are satisfied with positively impacting their grandparents' emotional welling, believe nobody can do a better job than the family, and regard caregiving as a form of personal growth even if it is not always pleasant (Megido et al., 2023; Perion et al., 2021). However, other studies also demonstrated that the experience could be both positive and negative (Newman et al., 2017). Grandchildren face great uncertainty, fear, and anxiety as a result of the unpredictable nature of the condition's progression and the resultant ambiguous loss; they may experience altered or interrupted family relationships as the disease progresses (Newman et al., 2017) and may experience worsened GP-GC relationships (Celdran et al., 2011; Creasey et al., 1989). Grandchildren can also be concerned about being unable to maintain the relationship as they may not know what to say or how to behave, lack emotion-regulation strategies in the face of constant change, and lack perspective-taking skills (Miron et al., 2017). When mothers are involved in dementia care for



grandparents, grandchildren may feel the disruption of daily routines and shared leisure time, resulting in sadness, anger, tension, and avoidance of the caregiving situation (Howard & Singleton, 2001). In conclusion, grandchildren contribute, experience, and perceive the role of caregiving differently from spousal and adult child caregivers. Therefore, it is important to incorporate grandchildren into the wider family system and training programs plan (Celdrán et al., 2011) and provide them with information and resources to cope with dementia (Megido et al., 2023; Alm et al., 2014).

Traditionally, the dementia caregiving task lands on the patients' spouses and adult children. As the average life expectancy after being diagnosed with dementia can be very long (10 to 12 years for AD), dyadic care with a spouse or adult children is no longer sufficient (Gale et al., 2018; Lubinski & Orange, 2000). With increasing life span, the relationship between PLwD and the non-adjacent generation, grandchildren, may extend for decades. Thus, adult grandchildren are likely to become primary caregivers and share care responsibilities with multiple family members (Venters & Jones, 2021). However, while examining communication tasks confronted by families with dementia, limited research has focused on the role of grandchildren or younger family members (Braithwaite Stuart, 2022). Given the unique relationship between GP-GC and the value of intergenerational relationships, this study will include grandchildren while examining the family communication process.

2.1.3. Dementia Care in the Chinese Context

2.1.3.1. Stem Family as Care Providers

China hosts the world's largest population of patients with dementia, and the number will increase dramatically in the next 30 years (Jia et al., 2020; Wang et al., 2019). The annual cost per person and the percentage of the national GDP that dementia accounts for exceed global averages, imposing heavy financial burdens on families and society (Xu & Qiu, 2018; Wimo et al., 2017). Among the 10–11



million patients with dementia aged 60 in China, more than 60% have Alzheimer's disease, and around 70–80% have not received treatment (Jia et al., 2020). Although memory clinics and dementia advocacy campaigns have been developed in the past decade, help-seeking in dementia care still faces various barriers, such as lack of education, negative perceptions of dementia, and complex clinical features of different types of dementia (Zhao et al., 2016). Primary care settings do not routinely screen for cognitive impairment (Hu, 2014). Seeking diagnosis is generally delayed for 2 years after first recognizing cognitive symptoms, even in well-established memory clinics (Zhao et al., 2016). 93.1% of PLwD in China were undetected (Chen et al., 2013).

In China, approximately 85% of PLwD are cared for by family caregivers, and dementia further burdens family caregivers due to inadequate healthcare system support and effective treatments (Jia et al., 2016). Previous studies have identified various challenges in dementia care, such as increasing economic costs of dementia (Xu et al., 2016), low levels of dementia knowledge (Liu et al., 2019), and fewer working-age adults available for caregiving (Chan et al., 2013). In addition to the general stress factors encountered by dementia caregivers across different cultures, Chinese caregivers face unique challenges imposed by their social-cultural environments, such as the legal obligation to care for aged parents, the deep-rooted importance of filial piety governing the parent-child relationship, and the familial values such as interdependence and harmony (Zhang et al., 2019). However, the traditional perception of caregiving responsibility that puts aside one's own needs has been challenged and younger caregivers have shown contradictory and ambivalent attitudes towards filial piety (Zhang et al., 2019). Additionally, demographic changes, such as the urban-rural divide and migration, the one-child policy, and the resulting 4-2-1 family structure, have reduced the capacity and availability of the family caregiving system, with family members less likely to take full responsibility for caregiving as favored by traditional culture (Zhang et al., 2019).



Nonetheless, the intergenerational ties are strong and stem families remain stable in modern China (Zhang & Xie, 2022a, 2022b). Stem family consists of three generations: a married couple (older parents), one of their children, the child's spouse, and the younger couple's children (Fumagai, 2019). Although the nuclear family remains the most common family structure, represented by two thirds of all families, its dominance is decreasing and becoming increasingly unstable (Zhang, 2019; Yang & He, 2014). In comparison, it is becoming more common for younger generations to live with their parents when they enter adulthood and get married: in 2010, approximately one-third of young people aged 20 to 29 in urban areas and two-thirds in rural areas lived in stem families, rendering grandchildren as a relatively untapped but growing source of caregiving for grandparents in old age (Wong, 2018). The emergence of neo-familism in China also indicates the shifting family foci from ancestors to grandchildren (Yan, 2016): For both grandparents and adult parents, their ultimate goal of hard work and the meaning of life relies on the happiness and success of the youngest generation. Two generations selflessly devote themselves to raising the third generation, and the family resources, love, and care flow downwards rather than upwards (Yan, 2018). The centeredness of grandchildren has replaced filial piety, and the importance of children's well-being has replaced the respect for older relatives (Yan, 2018). Therefore, grandchildren are not only influenced by their grandparent's dementia but can also exert great influence on the disease journey, given their increasing significance to both adult parents and grandparents. Research also shows grandchildren may co-exist with one grandparent in their 30s under the 4-2-1 family structure (Jiang & Sánchez-Barricarte, 2011). Therefore, the stability of the stem family, the prominent role of grandchildren in modern families, and the long years of co-existence may set the stage for an expanding pool of grandchildren as informal caregivers with great influence on the family unit.



2.1.3.2 Intimate Turn in Family Life

The intergenerational relationship between older parents and adult children in Chinese families has changed profoundly (Gruijters, 2017). While a few decades ago, Chinese families were bounded by patriarchal norms and collectivist values; nowadays, Yan (2016) argues that the intergenerational and intramarital power balance has shifted with the emergence of “*intergenerational intimacy*”, an egalitarian parent–child relationship emphasizing emotional bonds rather than filial obligations. Reciprocity instead of submission has become the foundation of two-way exchanges regarding material, financial, and emotional support, which is coined as the “*intergenerational contract*” by Ikels (1993) (Liu, 2017). Although the traditional value of filial piety has declined among the younger generation, traditional patterns of family relations and family support remain strong as adult children generally maintain intensive social relations with their parents in the format of reciprocity (Gruijters, 2017). Family members still have prescribed roles and authority determined by a hierarchy of gender, generation, and age (Mann, 2001). Intergenerational solidarity also persists because of the state’s reluctance or inability to provide social welfare and public services, which reinforces filial norms and drives families to be the main providers of care (Liu, 2017). Globally, the interaction between intergenerational relations and political economy is well evident in that older people are more dependent on adult children, have more interactions and are emotionally closer with them if they live in states with less generous social welfare (Hank, 2007; Lowenstein & Daatland, 2006). Compared to developed countries, PLwD living in developing countries live in larger households with their extended families, indicating indivisibility from the care of family members (Prince, 2004). Cultural values emphasizing filial piety are consistent with a lack of public resources, as families survive through mutual exchange across generations (Sun, 2002).

Beyond the intergenerational relationship between older parents and adult children is the three-generational interaction, including grandchildren. The GP-GC



relationship has special significance in China due to filial piety and increased longevity (Xu et al., 2014). Early research has focused on grandparents providing daily care for grandchildren (particularly among left-behind children in rural China) and, in return, expecting respect and care from grandchildren when they enter adulthood (Xu & Chi, 2015; Yee et al., 2009). A high level of structural and functional solidarity has been identified between GP-GC, as evidenced by a high rate of co-residence and care provided by grandparents, extensive childcare involvement by non-coresidential grandparents, and a sizable number of skipped-generation households (Chen et al., 2011). Recent evidence has shown that grandparents also receive various types of support from their grandchildren, such as instrumental, financial, and emotional support, indicating that grandchildren may play an increasingly important role in supporting their grandparents multidimensionally (Xu & Chi, 2018). Grandparents also tend to receive more care support from grandchildren when they have no son available, or their daughters do not provide assistance (Silverstein & Xu, 2016). However, the *patrilineal preference* in Chinese culture may influence by whom and how much such support is provided, particularly in rural China. For example, the middle generation is a mediator in facilitating the GP-GC relationship, particularly the father's emotional closeness with his parents (Cong & Silverstein, 2011). Paternal grandparents frequently have closer relationships with their grandchildren than maternal grandparents, particularly paternal grandmothers, who care for and interact more with grandchildren (Cong & Silverstein, 2012). As the grandchildren enter adulthood, grandparents are more likely to provide and receive support from the grandchildren of their first-born sons rather than their first-born daughters (Cong & Silverstein, 2008). The patrilineal preference may influence family dynamics, GP-GC relationships, intergenerational communication patterns, caregiving preferences, and past and future conflicts.

Given the important role of grandchildren functionally and emotionally, Xu and Chi (2018) advocated that the younger generation should work with parent generations



to support their grandparents for the benefit of family and society. However, recent research mostly focuses on the impact of intergenerational relationships, particularly intergenerational support, on older people's depressive symptoms (e.g., Fu & Ji, 2020), life satisfaction (e.g., Wu, 2022), and health status (e.g., Shu et al., 2021). Yet little is known about how the changing intergenerational relationships shape family communication patterns, and how, and to what extent, grandchildren communicate with grandparents with dementia and the wider family system. Therefore, this study aims to understand the role of adult grandchildren in Chinese families during the family communication processes about dementia. Their views will be the entry point for qualitative interviews to reflect on how dementia influences the younger generations and how they shape the disease journey differently in relation to adult parents and grandparents. Recognizing the importance of involving adult grandchildren in dementia care and communication, in the next section, I will highlight what is left undiscussed in families with dementia and how such communication can be complicated by the Chinese communication culture.

2.2. Communicating Dementia in The Family Context

Considering the importance of family communication about dementia, this section first summarizes the dementia-related family communication topics covered in previous research and highlights the gaps. Due to the lack of material regarding dementia communication, I then examined family communication in other diseases and conditions, including cancer, schizophrenia, and end-of-life care, to provide further insights related to dementia communication research. This section then introduces Chinese communication culture and practices, its intricacies and subtleties, and how this is manifested in health communication with Chinese American families.



2.2.1. Family Communication about Dementia

Communicative engagement is vitally important for PLwD and the people around them, especially family members (McNaney et al., 2017). As summarized in **Table 1**, family communication topics can be categorized into decision making, the personhood & relational aspect of communication, and the emotional aspect of communication (Alsawy et al., 2020).

2.2.1.1. Decision Making Communication

Decision-making communication aims to achieve quality care by communicating important decisions and understanding each other's thoughts. Family caregivers consistently make decisions about the health and social needs of PLwD, from the initial diagnosis to everyday care management and long-term care arrangements (Stevenson & Taylor, 2018). Research in this category explores the experience of decision-making, how decisions were negotiated, and how family dynamics changed, mainly using retrospective interviews.

At the very beginning of the diagnosis, the family needs to come to terms with dementia and decide how to convey the diagnosis to others, which generally requires a new narrative of self and family (Weeks et al., 2015). PLwD recognize the importance of disclosing their condition but often find it difficult to initiate and maintain the conversation and manage the negative emotional consequences (Weeks et al., 2015). When, who, where, and how to tell or not tell others is important for family discussion, but early works mainly only focus on physicians' diagnostic disclosure to the patients (e.g., Marzanski, 2000; Pinner, 2000). The family can unite in announcing the diagnosis, but at times, this is also a source of distress rooted in long-established family dynamics and relationships (Weeks et al., 2015). For patients with young-onset dementia (before 65 years old), family members also need to discuss the potential genetic risks and whether to consider genetic tests (Poulton et al., 2023). However, any communication surrounding



genetic risk is complex and sometimes may be unspoken and kept hidden for a long time, even with one's own children (Ducharme et al., 2013).

After the initial diagnostic period, dementia issues merge into everyday family routines. Thus, the major communication item is everyday care, including most ADL and IADL activities such as grooming, socializing, eating, spending money, and so on. (Miller et al., 2016). Communication aids such as “Talking Mats” were developed and have proven effective in facilitating decision-making about daily living (Murphy & Oliver, 2013). Additionally, everyday life involves various risks for PLwD. For example, family caregivers are mostly concerned about dangers such as driving, falls, financial risks, becoming lost, and using electrical appliances; In comparison, PLwD are concerned about their daily activities, hobbies and socializing, mental health and medicines, and risks to and from others (Stevenson et al., 2019; Stevenson & Taylor, 2018). These different concerns and perceptions of risk necessitate discussion between PLwD and family members. In the later stage, the family may have to discuss with PLwD whether to move to a nursing home, but PLwD frequently resent their lack of agency in these discussions since such decisions are usually made after their judgment ability is largely impaired (Lord et al., 2016).

In summary, PLwD value the opportunity to express their views and participate in everyday decision-making (Daly et al., 2018). However, shared decision-making declines over time in tandem with cognitive function declines, and caregivers may gradually take over decision-making (Samsi & Manthorpe, 2013). Therefore, early discussions with PLwD on important decisions is important to ensure congruent understanding and may reduce caregiver stress and uncertainty in the long run.

2.2.1.2. Personhood and Relational Aspects of Communication

Personhood & relational aspects of communication aim to build strong and effective relationships with self and family by communicating dementia-related changes.



Since dementia changes the complex weave of responsibilities, roles, identity, and relationships within the family, the first sub-category is negotiating family positions (social construction of self and family within conversations) (Kindell et al., 2017; Kitwood, 1997).

Research has explored how PLwD (Benbow & Kingston, 2016), spouse (Hayes et al., 2009), adult children (Hoppe, 2022), couples (Nilsson, 2022; Molyneaux et al., 2012), and family members (Purves, 2011; Perry et al., 1996) negotiate changes associated with dementia, re-position themselves and each other, and reframe the dementia experience through narrative construction. This process includes not only communication with others, such as the co-construction of couplehood in dementia (Molyneaux et al., 2012), but also communication with self, such as how the spousal caregiver identity affects a couple's intimacy and their own positions in the marriage (Hayes et al., 2009). "Self" is therefore a product of co-construction, rather than residing in individual PLwD or caregivers (Kindell et al., 2017). Producing a narrative is valuable for both PLwD and family caregivers and contributes to the quality of dementia care (Benbow & Kingston, 2016). Through telling and retelling the story, old meaning develops and evolves into new meanings that can address existential issues (Greenhalgh & Hurwitz, 1999).

The second sub-category under the relational aspect of communication is maintaining relationships. The goal of these conversations is not only to convey or exchange messages but also to have an interactive function that enables being together in the moment (Müller & Mok, 2014; Hydén et al., 2013). Since an important function of communication in marriage is to maintain the marital relationship, research has examined spousal communication and developed facilitative communication strategies to foster engagement and strengthen relationships (Williams et al., 2018). Another line of research that has received attention in recent years is on GP-GC relationship maintenance. Despite the strong desire to maintain meaningful interaction with their grandparents with dementia,



grandchildren frequently found it difficult to initiate and maintain conversations due to lack of life knowledge and generational gaps (Welsh et al., 2018). Therefore, grandchildren are found to consciously make an effort to maintain conversations and connections with grandparents with dementia who are relationally present, both through non-verbal communication (Miron et al., 2019) and technology-aided communication, such as picture boards (Welsh et al., 2018; Finke et al., 2008).

In summary, these studies foreground the importance of conversation as a fundamental part of relationships and accentuate the importance of preserving family networks and quality interaction (Purves & Phinney, 2012; Hummert, 2007). However, more research is needed to explore how grandchildren maintain relational closeness with PLwD (Miron et al., 2019).

Existing caregiving literature has predominantly examined the PLwD-caregiver relationship rather than the relationship between multiple caregivers, both on the instrumental and relationship-maintenance levels. The communication interventions mostly focus on PLwD, caregivers, and healthcare professionals without addressing the communication needs of other family members (Morris et al., 2018). While some caregivers receive a degree of respite from formal caregiving services, most rely on family members' support to manage caregiving tasks, experience loss, and relieve caregiving stress (Speice et al., 1998). However, the caregiving task shared among multiple family members is always unequal, and tension between family members can be a major source of stress (Smith et al., 2022; Panyavin et al., 2015). Communication challenges and role changes between family caregivers and other family members are also evident (Smith et al., 2022). However, only one study has specifically examined the communication process of how siblings discuss caregiving responsibilities and how this affects their relationships (Kokorelias et al., 2022). Siblings can either become closer to each other due to the increasing demand for caregiving communication and increasing involvement in each other's lives, or poor communication can result in relationship strain



(Kokorelias et al., 2022). In addition, younger family members often feel “left out” in family discussions about dementia and the changes happening to their loved ones since they are often infantilized when it comes to dementia, and family members are concerned about not being able to make them understood what is going on (McNaney et al., 2017). Young family members often want to be more involved in family discussions as a recognized and valuable member of the care relationship, as they identify themselves as advocates for PLwD and support them in seeking to remain independent as much as possible (McNaney et al., 2017). There appears to be a lack of research on communication between family caregivers and other family members, so this study will incorporate grandchildren into the communication dynamics and examine their communication role and responsibility with parents and grandparents as the condition progresses (Smith et al., 2022).

2.2.1.3. Emotional Aspect of Communication

The emotional aspect of communication aims to maintain good mental health for both caregivers and PLwD by effectively communicating positive and negative emotions. Surprisingly, only one article examined how communication occurs during a caregiver’s nursing home visit and how they emotionally supported and connected with PLwD (Levy-Storms & Chen, 2020). There is a significant lack of research on how families communicate about the negative, positive, and ambivalent emotions that widely exist around dementia. While examining the dementia narrative by caregivers and PLwD, Benbow & Kingston (2016) noticed they frequently talked about their emotions and how these were impacted by the condition. Caregivers and PLwD use words such as “devastated”, “worried”, “down”, “fraught”, “short-tempered”, “stressed”, “guilty”, “upset”, and “embarrassed” to describe their emotions, while feeling ashamed, frustrated, and hopeless are emotions specifically experienced by PLwD (Benbow & Kingston, 2016). The accounts of PLwD in previous research also demonstrated their continued emotional awareness and their awareness of the impact of dementia on the people who care for them (Ablitt et al., 2010; Burgener & Twigg, 2002).



Family members also face strong emotional issues in accepting the diagnosis (Knopman et al., 2000). Nonetheless, caregivers can enact emotional management strategies to reduce PLwD's negative emotions despite the strain on their own emotional energies (Gilbert et al., 2021). Being able to talk about these emotions with family and friends is important for validating emotions rather than just processing them internally, as this helps the family come to terms with dementia and better adapt to it (Weeks et al., 2015). Younger family members also expressed that opportunities to talk about dementia with others are rare yet expressed a desire to engage people experiencing similar circumstances (McNaney et al., 2017). While professionals can provide them with information, it is family and friends that are reserved for, and can more successfully provide, emotional support by having conversations about dementia (Wellman & Wortley, 1990). Earlier research by Speice et al. (1998) also noticed that caregivers who have the chance to talk to other family members about their illness experiences feel more emotionally supported and less depressed. However, little research has examined the emotional communication process and how families can talk through negative emotions and enhance positive emotions through communication across generations.

In conclusion, the personhood & relational aspects of communication and emotional aspects of communication are both highly pertinent for PLwD, but previous research has mostly focused only on decision making between spouse/adult children and PLwD (Alsawy et al., 2020; Williams et al., 2018). Although there are strategies and interventions to improve communication skills, they do not directly address family relationships and emotional-based needs (Purves & Phinney, 2012). The empirical evidence is scattered, and how the family members communicate other important issues, such as the presence of dementia symptoms, negotiating for diagnostic checking, and managing emotional arousal with family members other than spouses or adult children remain unexplored. This will be the focus of the current study.



2.2.2. Family Communication in Other Diseases and Conditions

Research on family communication about dementia is limited, with most studies consider communication difficulties/breakdown as a predictor of the caregiving burden, not an outcome or a process itself (Harris et al., 2009). Nonetheless, a large body of research has examined family communication in other diseases and conditions, such as cancer, schizophrenia, and end-of-life illness, which can provide valuable insights into studying family communication about dementia.

The importance and complex nature of communication has been recognized early in the context of cancer, including doctor-patient interactions, healthcare team communications, and communication within the family (Hay et al., 2009). In particular, the construct of “family” is critical in health communication about cancer, since family history may lead to discussions about risk, family members serve as the primary support unit for patients, and the family context lays the foundation for healthy behavior promotion (Bowen et al., 2017). The onset of cancer may prompt family members to assume different roles and bring the family together, and it is believed that including family members from and within each generation is useful for promoting health interventions and reducing disease outcomes for the patient (Bowen et al., 2017). In the family context, research has examined topics across the continuum from cancer diagnosis to survivorship, such as disclosure of genetic susceptibility (Harris et al., 2009), family response to the diagnosis (Hay et al., 2009), lack of communication about genetic risk (Bowen et al., 2017), deliberation about whether to obtain genetic testing in families, screening and prevention behaviors (Hay et al., 2009) and later-stage cancer communication (Zhang & Siminoff, 2003). Key barriers to communicating about cancer and associated emotions are also identified, including lack of intimacy and closeness, avoidance of distress and disappointment, preoccupied ideas or concerns, desire for mutual protection, poor mental state, and family-specific communication rules (e.g., maintaining privacy and gendered roles). Even if the patient or caregiver wants to



initiate a conversation, the deliberate effort of others to remain silent is still prominent. Thus, intra-familial communications among cancer subjects are frequently discordant and guarded (Zhang & Siminoff, 2003).

In addition, Wittenberg et al. (2012, 2017) identified four caregiver types (Manager, Carrier, Partner, and Lone) and explored their specific communication challenges to support family-centered cancer care. Across these four caregiver types, researchers notice common primary communication tasks within the family, such as initiating conversations about prognosis and end-of-life wishes, asking for and receiving help, sharing burdens with others, and the need for self-care (Wittenberg et al., 2017). Nonetheless, *managers* and *carrier* caregivers find communication difficult since they want to protect the patient by avoiding communication about disease and prognosis, and *lone* caregivers, who lack family support, report difficulty communicating with health providers (Wittenberg et al., 2017). These barriers and difficulties emerge from implicit and explicit family rules that govern appropriate communication topics, discussion frequency, and a hierarchy among family members regarding their communication roles. The rules can be categorized into two general family communication patterns, ***family conversation*** (facilitative communication among family members), and ***family conformity*** (conforming to the powerful family members) (Wittenberg et al., 2017).

Another pertinent line of research explores how expressed emotion (EE), an overt abrasive behavior of caregivers, impacts family communication with schizophrenia patients. EE measures *critical*, *hostile*, or *emotionally overinvolved* attitudes the caregivers hold toward mentally or physically ill family members and is a known risk factor of schizophrenia relapse (Kavanagh, 1992). High EE is associated with poor illness course and is closely associated with family communication deviance (CD), such as lack of clarity and distorted communication styles (Kymalainen & Weisman de Mamani, 2008). Research shows that communication patterns among family members with high EE usually involve more intense and negative verbal



exchanges (Amaresha & Venkatasubramanian, 2012). Culture also plays a role since family members are more likely to show high EE when the patient cannot maintain certain cultural norms and values endorsed by their family and ethnicity (Kymalainen & Weisman de Mamani, 2008). Eventually, high EE and CD may prevent family communication from managing problems effectively and expressing thoughts coherently. Although some research has studied EE in the field of dementia, they mostly focus on its impact on caregivers' mental health, burden, distress, and the psychological well-being of PLwD (e.g., Safavi et al., 2023; Wong & Zelman, 2020; Safavi et al., 2018), rather than going further to examine the communication process during emotional arousal and explore the underlying communication patterns.

In addition to cancer and schizophrenia, another general field of relevant family communication study is that covering end-of-life (EOL) with terminally ill patients since communication is an essential prerequisite for good EOL care and decision-making. Various studies have examined barriers to EOL discussion among family members, such as unpredictability experienced at EOL, denial of dying, and family conflicts (Gerber et al., 2020). Family roles also play a part, in which spouses have the most say in decision-making, which may override the care preference of adult children (Gerber et al., 2020). Therefore, professional coordinators with communication skills and EOL knowledge are important to facilitate family communication since death is mostly taboo subject in modern societies, and caregivers are emotionally distressed (Gerber et al., 2020; Saini et al., 2016; Moore et al., 2015).

In conclusion, although these research areas cannot be directly applied to dementia due to its distinct clinical features, learning from family communication about other diseases can provide valuable insights into health communication, such as the impact of cultural values, different communication roles, and power relationships in shaping family communication.



2.2.3. Communication Culture in China

Chinese communication is subtle and intrinsic. As shown in **Figure 1**, Chinese communication has four important components, including its cultural foundation, cultural beliefs and taboos about communication, the speaking rules, and communication goals, all of which are profoundly influenced by Chinese culture. This section will first explain why harmony is the foundation of Chinese communication and why it cannot be taken at face value. Then I will explain how Chinese culture constructs the “Chinese self”, which further shapes interpersonal relationships and establishes communication goals. Finally, I summarized the speaking rules determined by the cultural beliefs and taboos and how they are manifested in health communication between Chinese American and healthcare professionals in the immigration context.

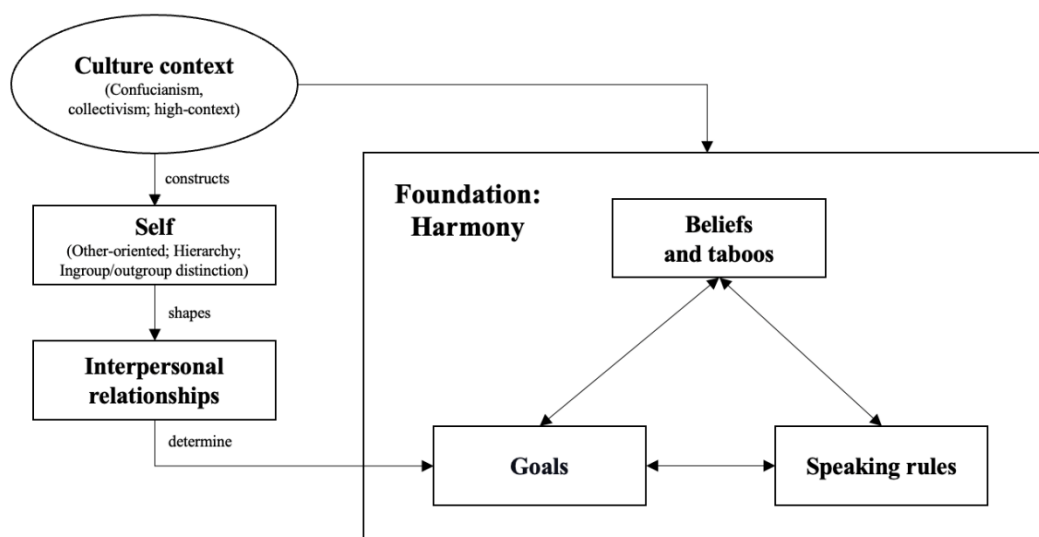


Figure 1. The formation of Chinese communication culture

2.2.3.1. Harmony as a Foundation of Chinese Communication

Chinese communication is subtle and intrinsic. Communication theory research, to explain and predict human interactions, has traditionally been Eurocentric and mostly set in Western contexts (Tian & Yu, 2021; Miike, 2007). Recognizing the

lack of resonance of Western derived theory with the non-Western world, Asante (1980) first brought up the concept of “Afrocentricity” to facilitate indigenous African communication research. This was followed by “Asia-centricity”, proposed by Miike (2003) to understand Asian communication including concepts particular to Asian languages principles from Asian religion-philosophical traditions, and lessons from Asian historical experiences (Miike, 2008). An emic approach was later employed to study a specific culture or area, such as Chinese (e.g., Guo-Ming Chen) and Japanese (e.g., Yoshitaka Miike) communication.

Chinese communication culture broadly reflects the East Asian communication orientation influenced by Confucianism, including process orientation, differentiated linguistic codes, indirect communication emphasis, and receiver-centered nature (Yum, 1988). It also corresponds to social constructivism in that, compared to North America, communication in East Asia is an infinite process of interpretation, and meaning is subject to the interpretation, listening skills, and sensitivity of the receiver rather than the sender (Yum, 1988). Beyond the Asiatic interpretation, Western scholars have also viewed communication from different cultural orientations and proposed that Chinese culture and language is high-context (Hall, 1976) and has a high level of collectivism (Hofstede, 2001). According to Hall (1976), in a high-context culture, people are deeply involved with each other, personal feelings are strongly controlled, and information is widely shared through simple messages with deep meaning. In contrast, people in low-context cultures have relatively fragile bonds with others and are somewhat alienated, necessitating that communication between people needs to be more explicit and nonpersonal (Kim et al., 1998). With intimate human relationships and well-structured, established and recognized social hierarchy and norms, communication in high-context cultures relies more on the physical context and less on the verbal message itself. In other words, one must situate the messages in the appropriate context and consider the relationship involved to understand the correct intended meanings conveyed in the messages. When, where, and who broach the



communication subject all construe the meaning of the message (e.g., Yonashiro-Cho & Enguidanos, 2016). In contrast, low-context culture is more context-free, with vast information vested and supplied in explicit code such as words, sentences, and documents. In low-context culture, what is said is important, rather than how it is said and the environment in which it is said.

In Hofstede's cultural dimensions, Chinese culture has a high level of collectivism, a relatively low level of uncertainty avoidance, and relatively high levels of long-term orientation (Hofstede, 2001). In addition to preferring indirect and non-expressive communication, this collectivist orientation also reveals that Chinese people use more "other-oriented" communication strategies such as encouragement, credit-giving, or example-setting to exert influence (Li & Chi, 2004; Ma & Chuang, 2001). Therefore, Chinese communication is highly contextualized and cannot be reduced to a questionnaire investigation. To glean the essence of what is said and unsaid, one must probe into the collectivist nature of Chinese communication and situate oneself within the individual, relational, and familial context.

In exploring Asia-centric communication, Chen (2011) identified harmony as the foundation of the paradigmatic assumptions of Chinese communication, in parallel with the ideological goal of a harmonious society in Confucianism. To preserve social harmony, Chinese people have more reserved, modest, and other-oriented communication patterns, such as emphasizing listening over speaking, inhibiting aggression, and avoiding confrontation, adopting silence when dealing with authority, and avoiding emotionally upsetting communication (Chang, 2001; Yonashiro-Cho & Enguidanos, 2016). However, this importance of harmony does not idealize or beautify Chinese communication (Chen, 2013). When harmony cannot be upheld, Chinese people may show aggressive behavior and direct expression to protect their face, or harmony can be used as a superficial performance to mask turbulence and manipulation with minimal discord (Chang, 2001). For example, Chinese people can develop artfully crafted messages to



express competition or articulate messages with inconsistent external messages and internal emotional foundations, e.g., *zhi sang ma huai* (指桑罵槐)/ *xiao li cang dao* (笑里藏刀)/ *kou mi fu jian* (口蜜腹劍). Although face concern is universal, and it is likely for people in any country to express aggressiveness or issue impolite messages (Brown & Levinson, 1987), what distinguishes Chinese harmony is not only the intensity of such practices but also shared assumptions of social harmony which may, in fact, provide speakers more opportunities to challenge or hurt others without provoking social sanctions (Chang, 2001). Remediating such situations might be considered over-sensitive and inadvertently damaging to the relational network (Chang, 2001). Therefore, Chinese communication cannot be taken at face value. Even if family communication about dementia appears harmonious, efforts should be made to explore underlying hidden messages and why dementia communication is construed in a certain way.

2.2.3.2. Self, Culture, and Communication

Surface communication patterns reflect deeply structured worldviews shaped by cultures (Chen & Miike, 2006). To understand the cultural premises of communication, Gao (1998) argued that it is important first to examine how the Chinese self is constructed, as this self is defined by, and governs how, one speaks in everyday interactions. In essence, Gao (1998) noted that (1) “others,” (2) role relationship and hierarchical structure, and (3) ingroup/outgroup distinctions; altogether these shape the Chinese self and permeate interpersonal relationships. First, the other-oriented self-concept determines self is incomplete without others, and thus, one must be highly aware of one’s social relationships and be attentive to external expectations and social norms. Second, a Chinese person has prescribed roles in society with power embedded in seniority and authority, such as the elderly, rulers, parents, teachers, husbands. (Chen, 2013). People are expected to enact those roles accordingly to achieve social harmony rather than behaving based on “self” (Chen & Chen, 2002). Third, Chinese people make clear distinctions between insiders and outsiders, with family, friends, and established relationships being the



focus of social relationships and these being crucial to self-construal. The family, rather than the individual, is the most important unit of social organization in Chinese culture (Elliott et al., 1996).

The construction of the Chinese “self” determines the three functions of Chinese communication: (1) maintaining existing relationships, (2) reinforcing role and status differences, and (3) preserving ingroup harmony. It significantly differs from Western culture, where communication is regarded as an expression of individual “self” (Rosaldo, 1990). Therefore, while communicating about sensitive issues such as dementia, family communication may not necessarily be used to express personal needs and expectations; rather, communication may function to maintain existing family relationships without harming hierarchical roles. However, little is known about how such communication preference complicates the dementia journey, whether different generations hold similar perceptions of communication, and how family harmony is achieved while negotiating diverse communicative goals across generations.

Certain Chinese cultural beliefs and taboos about communication exist that render verbal and direct communication less important. For example, old sayings such as *huo cong kou chu* (禍從口出)/ *yan duo bi shi* (言多必失) indicate that speech should be exercised with caution and control or one must cope with the negative relational consequence; *yan bu jin yi* (言不盡意)/*yan wai zhi yi* (言外之意) demonstrates the inadequacy of spoken words in constructing meanings, and thus non-verbal expression and hidden messages are valued more. One must go around the circle to finally arrive at the communication target. Cultural taboos such as death and dying also necessitate indirect and depersonalized communication strategies while approaching EOL care issues that ideally minimize face-to-face and verbal communication pressure (Chi et al., 2018; Yonashiro-Cho & Enguidanos, 2016). The cultural emphasis on filial piety is therefore also sometimes utilized by healthcare professionals to pressure adult children to discuss end-of-life



arrangements (Yonashiro-Cho & Enguidanos, 2016). Similarly, dementia in Chinese culture is a sensitive topic that intertwines with the social, natural, and supernatural worlds (Elliott et al., 1996). Zeng et al. (2015) found that 76.6% of Chinese people have a personal fear of AD, and severe dementia is preferably labeled as “severe confusion” (老糊涂) by their family. Chinese people may attribute dementia and mental illness to sins of one’s ancestors or family misdeeds, misalignments of the physical environment (e.g., house, grave), or the afflicted person possesses evil spirits that opposed the flow of energy in the body (“yin” and “yang”). These cultural beliefs, combined with the ancient admonition that “bad deeds that happen in a family should not be disclosed to outsiders,” can trigger negative family responses toward dementia and often inhibit or delay formal help-seeking (Kleinman, 1986b; Lee, 1982; Phillips, 1993). Chinese people prefer hiding the afflicted elder and the dementia problem within the family for fear of shame and stigma (Elliott et al., 1996). These cultural beliefs and taboos may preclude people’s outward verbal expression but not their internal feelings. Recent research among the younger generation has started to explore the “taboo” subjects related to dementia, such as “I do not like my ill parent” and “I would rather it is death or other diseases” (Hall & Sikes, 2017). Although these thoughts and feelings are socially sanctioned, they are still legitimate, and thus intergenerational family members should be allowed to express their feelings about cultural beliefs and taboos and how they may affect the communication process.

2.2.3.3. Speaking Practices and the Immigrant Context

With the functions of, and beliefs about, Chinese communication in mind, Gao (1998) summarizes five major speaking practices in Chinese culture: (1) implicit communication (含蓄); (2) listening-centeredness (聽話); (3) politeness (客氣); (4) focus on insiders (自己人); and (5) face-directed communication strategies (面子). Although these speaking practices are neither examined in Chinese family communication nor health communication, they are nevertheless well evident in patient-doctor communication situated in the immigration context as this inevitably



requires cultural sensitivity. Many studies have examined how the Chinese American (C-A) subgroup communicates with foreign health professionals and how cultural barriers and family beliefs can restrict effective communication. For example, doctors are regarded as “outsiders,” and disclosing family health history (e.g., cancer, mental health) and “secret” information may be considered as disloyalty to the “insider” C-A family (Chen et al., 2016; Xu, 2014). Due to fear of losing face and desire to maintain social integrity, many Chinese immigrants do not openly disagree with doctors and may not ask questions even if they do not understand (Ho et al., 2012). Patients should not be aggressive or contradict their perceived roles when seeking help because doctors are busy and are seen as authorities in their fields (Chen et al., 2022). Communication strategies have been developed to effectively communicate with the C-A family, such as adding a longevity blessing while discussing EOL issues, associating adherence to a healthy diet with a long life to be enjoyed with family (Ho et al., 2012). Although this research mostly focuses on patient-doctor communication, evidence has also shown that many Chinese immigrants even avoid talking with their own family members about diseases that run in the family, such as dementia (Cheng et al., 2018; Sun et al., 2014). Exposing memory loss and communication failure can be seen as a face-threatening act. The prevalence of concealed mental illness is coined by Ross (1997) as a “silent epidemic” among older Asian American. However, there is scarce research into the family communication process regarding health issues and how these cultural beliefs and speaking rules are practiced within Chinese families.

In summary, Chinese communication is inherently different from the Western orientation. What Western scholars value as key elements of communication competence, *effectiveness*, and *persuasiveness* are not major cultural considerations in Chinese communication. Instead, the ability to negotiate relationships merged with the ability to interpret messages in a high-context situation, stood out from the literature as the most important aspects of Chinese communication competence (Yeh & Chen, 2004). Therefore, while studying



dementia communication in a Chinese context, one must focus on the inherent special cultural practices and extrapolate the underlying cultural assumptions that might be overlooked in the Western paradigm. The fact that Chinese people are nurtured with elaborate verbal exchanges to manage multiple layers of family as well as interpersonal relationships to compete, negotiate with, criticize, or even ridicule others requires a high level of theoretical abstraction in order to understand the Chinese interaction and communication (Chang, 2001). Despite this need, the concepts of low/high context, individualism/collectivism, and indigenous communication research are mostly applied to business and management studies, which is believed to be in response to the 1990s globalization trend when China opened to the global business world. This research helps navigate how to effectively communicate with people from different cultures and avoid unnecessary misunderstanding or cooperation failure. However, relatively little research is devoted to investigating intrafamilial communication with different generations of adults.

Therefore, this study will examine the impact of Chinese communication culture on adult family members and respond to the research gaps in dementia communication literature (initial decision to dementia diagnosis, diagnosis disclosure and family discussion, and communicating emotions). Although the study focuses on Chinese communication practices, it can shed light on the communication dynamics in other non-Western societies to enrich the cross-cultural understanding of family communication in the context of dementia care. The nature of understanding the communication process and family interaction is inherently different from other qualitative approaches, such as discourse analysis or phenomenology. In the next section, I will introduce the existing theoretical perspective on family communication and justify the necessity to broaden the theoretical understanding of the studied phenomena.



2.3. Theoretical Perspectives in Family Communication

Family communication is an important aspect of health communication (Zhang & Siminoff, 2003). Existing theories on dementia communication mostly focus on why family members find it difficult to communicate with PLwD and neglect the complexities of the family communication process (Ennis et al., 2019). There are several theoretical perspectives from other disciplines which explain the dynamics and difficulties of family communication, including the family system theory (Bowen, 1978), accounting for the family emotional patterns; the intergenerational ambivalence theory (Lüscher & Pillemer, 1988), accounting for why intergenerational relationship generate ambivalent feelings; and the communication accommodation theory (Giles, 1973), accounting for how and why family contexts change communication patterns. This section reviews the key focus of the three theories, including key variables and the definitions, hypothesized relationships between variables, the application of the theory, what the theory can predict, the application in the Chinese context, and critiques. However, none of these are sufficient to accurately predict family communication about dementia. This section concludes that it is necessary to conduct inductive reasoning rather than employ deductive hypothesis testing for the studied phenomenon.

2.3.1. Family System Theory

The family system theory (Bowen, 1978), originally developed in psychiatry and psychotherapy, is one of the first comprehensive theories on family system functioning. It recognizes the family as a complex social system and emphasizes the interdependence and interactions between multiple family members instead of dyadic relationships or a collection of individual relationships (Bowen, 1978). The focus is on the family emotional patterns developed and transmitted down generations and how family members seek to defuse chronic anxiety and maintain stability (Brown, 1999). Bowen (1978) regards chronic anxiety as a source of family dysfunction, and so the goal of Bowenian therapy is to recognize how the family emotion system functions and thereby increase the levels of differentiation



of self from the family of origin, resulting in less blaming and decreased reactivity (Brown, 1999).

There are eight interlocking concepts in Bowen's theory that describe the inevitable presence of chronic emotional anxiety in family relationships: (1) Emotional fusion and differentiation of self: the greater the tendency for the family to fuse, the less flexible the family is to adapt to stress; whereas "differentiation" is the individual capacity to function autonomously while remaining emotionally connected with the family system; (2) Triangles: triangling occurs when anxiety between a dyad is relieved by involving a vulnerable third party, who either take sides or provides a detour for the anxiety; (3) Nuclear family emotional system: this includes four basic emotional functioning areas: marital conflict, dysfunction in spouse, impairment in one or more children and emotional distance; (4) Family projection process: parents pass their emotional problems onto their children; (5) Emotional cutoff: this describes "breaking away" from the family rather than "growing away"; (6) Multigenerational transmission process: family patterns, themes, and positions are passed down from generation to generation; (7) Sibling positions: covering the different emotional roles of children in order of birth; (8) Societal regression: when families have higher levels of anxiety, this regresses to a lower level of social functioning.

Bowen's theory posits that the family and its emotion patterns exert influence on day-to-day well-being across life span. The emotional symptom of a patient is the reflection of the emotional symptom of the family, which is embedded in the patterns and behaviors of past generations (Haefner, 2014). The reciprocal functioning of all family members contributes to the patient's emotional intensity; thus, family therapy should consider more than one generation. The theory's focus on at least three generations and the application of the genogram is groundbreaking, as it gives insight into the hypothesis of family interaction patterns and behavior development (Brown, 1999). However, although the theory comprehensively



considered the emotional process across generations, it has been criticized for disregarding clients' requests for the shortest possible route to symptom relief or those who only want to address symptoms within nuclear families (Brown, 1999). Meanwhile, the gendered role in the family system theory, where males are assumed to have limited emotional engagement in family relationships, and women are over concerned and easily labeled as “fused” and “pathologized” in families, may be considered biased. In addition, Luepnitz (1988) pointed out that the family system theory lacks attention to feelings. The therapy focuses on rational and objective emotional patterns within the family and gives a low priority to the expressions of emotions during therapy.

Family is characterized by including multiple perspectives, with different family members giving different meanings to the same situation, some overlapping and others conflicting (Hammer, 1988). In the context of dementia, early research (e.g., Perry et al., 1996) has examined how families come to terms with dementia, how individuals make meaning of the new situation, and to what extent each family member is willing to accept the diagnosis and its consequences. Although applying the family systems theory can support family-centered dementia care and account for the impact of PLwD's communication difficulties with multiple family members, it is still a relatively understudied area, especially the lack of attention to the involvement of younger family members (Braithwaite Stuart et al., 2022; Kokorelias et al., 2019). Nonetheless, research in family communication about cancer has applied this theory and recognized that the cancer experience is not limited to a spouse, child, or patient; and thus, successful interventions must access the entire family unit (Harris et al., 2009). Although dementia research has explored how caregiving is experienced differently by spouses, adult children, and younger caregivers, how intergenerational family members manage communication challenges and discuss dementia-related issues remains unknown. With the guidance of the family system theory, this study will apply a three-generational lens and take a step further to triangulate family emotional processes and personal



feelings.

2.3.2. Theory of Intergenerational Ambivalence

Conceptualizing the complexities of family relationships in later life is challenging (Silverstein & Giarrusso, 2010). The most prominent paradigm, intergenerational solidarity, was proposed by Bengtson & Roberts (1991) to explain how personal feelings, such as affection, attraction, and warmth, serve to maintain cohesion in the family system. Another line of research has focused on the conflictual aspect of family relations, such as isolation, caregiving stress, and abuse (Marshall et al., 1993). In the late 1990s, Lüscher & Pillemer (1998) moved beyond the solidarity-conflict dichotomy, since family members may simultaneously have warm and antagonistic feelings for each other, proposing “ambivalence” to account for parent-child relations in adulthood. The concept of “ambivalence” has two dimensions: at the structural level, it means the contradiction among individuals’ positions in the social structure, such as family roles and generational norms; at the subjective or psychological level, it refers to the paradox of perceptions, psychological states, or emotions when faced with structural ambivalence (Bengtson et al., 2002). Since then, ambivalence has played an increasingly important role in the study of intergenerational relationships (Suitor et al., 2011).

Intergenerational ambivalence is defined as “contradictions in relationships between parents and adult offspring that cannot be reconciled” (Lüscher & Pillemer, 1988). The fundamental assumption of this theory is that intergenerational relations generate ambivalences (Lüscher & Pillemer, 1988). It is hypothesized that people must live with ambivalence, and they can cope with it in different ways (Lüscher, 2002). Lüscher (2000, 2004, 2005) theorized this concept by proposing four dimensions: the individual or subjective dimension (the vertical line) is divided into convergence (warmth and closeness) and divergence (coolness and indifference). On the structural or institutional dimension, it is divided into reproduction (keeping traditional family norms or structures) and innovation (changing family norms or



structures). This mechanism further produces four domains: emancipation, solidarity, captivation, and atomization to explain the source of ambivalence and attendant solutions. Under the emancipation domain, families keep close relationships and are willing to embrace changes, thus they can manage conflicts flexibly; in comparison, although families under the solidarity domain are supportive of each other, they are unwilling to change norms or structures, which may contain underlying ambivalent feelings. Under the captivation domain, one generation, usually the parents, tend to impose their will on others, and the other is forced to change reluctantly. Lastly, a family in the atomization domain does not emphasize cohesion and is open to change. A family may move from one to the another of the four types, indicating the possibility of changing family dynamics (Lüscher, 2005).

The intergenerational ambivalence theory derived from a sociological perspective was first considered to compete with the solidarity paradigm but later viewed as supplementary to conceptualizing the complex emotional ties in aging families (Bengtson et al., 2002). It challenged the long-standing assumptions that harmony exists among adult intergenerational family members (Silverstein & Giarrusso, 2010). Though originally considered pathological, ambivalence is currently viewed as part of the human condition and arises from the mixture of reciprocity, dependency, autonomy, tension, and concern among family relationships. Empirical evidence shows that spousal dementia caregivers either describe their caregiving experience as positive, ambivalent, or negative (Shim et al., 2012). Adult children have more ambivalent feelings towards older parents when providing care to them, indicating that parental dependence and role reversal causes mixed feelings (Fingerman et al., 2006). Ambivalence is also found among siblings. This can stem from current and past parental favoritism, particularly the mother's preference in childhood, which may also influence the sibling caregiving arrangement (Alm et al., 2014; Connidis, 2007). Feelings of ambivalence can also negatively affect the



psychological well-being and quality of life of older parents (Fingerman et al., 2006; Lowenstein, 2007).

From this theoretical perspective, a degree of ambivalent feelings is inherent to family relationships, and communication is both the ground for developing such feelings as well as the approach to managing them. However, although the theory of intergenerational ambivalence is extensively conceptualized, it has been criticized for its lack of operationalization in measuring ambivalence and the absence of a clear causal model to explain the real intergenerational relationship (Park, 2014). Only a few studies have conducted empirical research by applying this theory, but some still found it difficult due to the lack of appropriate measurements concerning the specific historical, cultural, and institutional context (Park, 2014). Therefore, intergenerational ambivalence can be utilized only as a sensitizing concept to understand the family communication process with dementia in the current study, since applying it directly into empirical research can be very challenging.

2.3.3. Communication Accommodation Theory

Communication accommodation theory (CAT) is rooted in social psychology, providing a causal explanation of how and why people shift their communication to or from those with whom they interact (Giles, 1973). It draws on the Social Identity Theory and posits that individuals' social identities, such as age, gender, and ethnicity, are reflected in their accommodative style (Gallois et al., 1995). Communication across generations is not only characterized by age segregation but also involves different values and beliefs about dialogue, different language codes as well as different social and existential agendas (Giles & Coupland, 1991). It provides a framework to view family communication through an intergroup lens, situates family relationships in the broader social and cultural context, and reveals the underlying socio-psychological processes (Harwood et al., 2006). Across contexts, intergenerational communication has been found to be unsatisfying and



even problematic (Ryan et al., 1986). Young and older people can often “communicatively miss” each other, such mutual negative stereotypes and misunderstandings (Giles & Gasiorek, 2011). Coupland et al. (1988) theorized four sociolinguistic encoding strategies in which family members adjust their communication to one another and the origin of problematic accommodative styles: (1) approximation strategies (i.e., convergence and divergence), (2) interpretability strategies, (3) discourse management strategies, and (4) interpersonal control.

First, approximation is employed to accommodate the interlocutor’s speech style, in which convergence refers to moving towards the other’s speech style, and divergence means moving away. It is hypothesized that children who strongly identify with their family and love their parents are more likely to adopt the family communication style than those who are alienated from their family. Similarly, spouses who are more psychologically engaged with their partner are more likely to converge their speech style. Thus, CAT predicts that increasing convergence is associated with higher levels of shared group identification.

Second, interpretability strategies refer to accommodating the interlocutor’s interpretive abilities, usually deficits. It is common for parents to attune their speech to children’s level or for young people adjust communication to compensate for an older adult’s physical or psychological deficits, such as hearing loss (Giles & Gasiorek, 2011). Nonetheless, such speech can go beyond what the interlocutor requires, which is termed *overaccommodation*. Overaccommodation has received the most attention in intergenerational communication, which is associated with negative age stereotypes (Caporael, 1981) and negative outcomes for older recipients (Ryan et al., 1986). Younger people tend to overaccommodate using simplified speech (e.g., syntax, grammar, content), patronizing speech (e.g., baby talk, depersonalizing language, third-party talk), or exaggerated intonation, pitch, and volume. However, within the family context, older people wish to be treated as individuals instead of being grouped simply as elderly people. Thus,



communication is crucial in negotiating relations and identity within a family. In addition to young-to-elder communication, elder-to-young communication can also be problematic: older people may show non-listening, disapproving/disrespecting youth, overprotective/parental talk, and criticism during the interaction, which often inhibits the younger generation's motive to express affect, maintain the conversation, and accommodate the older conversation partner (Giles & Gasiorek, 2011). Younger people also report being reluctant to accommodate older people due to a lack of knowledge of life history, feeling unsure of how to respond, and feeling discomfort in such interactions, which may lead to *underaccommodation* and *non-accommodation* (e.g., Barker, 2007; Williams & Giles, 1996).

Third, discourse management strategies focus on the other person's conversational needs, such as topic selection. It is appropriate for young people to talk about the past with the old, and older people may open the conversation by talking about the children's school life. However, intergenerational communication can be problematic when it involves painful self-disclosure by older people, such as poor health, depression, loneliness, and bereavement, which is negatively associated with relational solidarity and reciprocated communication (Soliz & Lin, 2014).

Fourth, interpersonal control refers to an explicit attempt to control and manipulate the recipients, especially older people who are dependent and powerless in institutional settings. Older people are patronized due to their inferior social group status hierarchies.

These four accommodation strategies are influenced by cultural variability (Parke, 1994). For example, accommodation within the family carries more weight in collectivist cultures as they view communication as a shared responsibility to maintain relationships (Chuang, 1998; Gao, 1996). The CAT is also useful for understanding cultural norms through communicative practices. For example, what is perceived as overaccommodation by Anglo-American students was not considered so when translated and rated by Hong Kong students (Giles et al., 1998).



Similarly, young Chinese respondents did not downgrade an overaccommodative speaker as was done in California. It is highly possible that in cultures where role relations and hierarchy are heightened, such as China, accommodation strategies are practiced and reacted to differently, but this field has not been empirically studied (Giles & Gasiorek, 2011). CAT has also been criticized for its reductionist nature since communication is very complicated and can be conveyed by nonverbal language and so cannot be broken down by words alone. In addition, people can shift their communication patterns within the same conversation, making it difficult to capture subtle changes within abovementioned strategies. Furthermore, CAT assumes people communicate in a rational manner, ignoring the fact that people can be irrational and unreasonable during conflict, and may say what they do not mean. Therefore, CAT does not address the reductionist problem and irrational aspect of interpersonal communication.

In the context of dementia, as family members usually have long-established communication patterns before the onset of dementia, they may not know how to change their communication behavior effectively to accommodate PLwD, which may result in under or over-accommodation (Small & Ann, 2012). Family members may have conflicting views as to what degree to accommodate PLwD's communicative needs and how to elicit views from PLwD. Given the complexity of cognitive-communication problems, it is also tricky to determine what constitutes appropriate accommodation, particularly for the younger generation. Negative elder-to-young communication may also be exacerbated by dementia and behavioral and psychological symptoms of dementia (BPSD). Even under this theoretical framework, little is known about how intergenerational family members accommodate each other's communication needs bi-directionally and negotiate potential conflicts.

As summarized in **Table 2**, the three theoretical perspectives provide an insightful lens on intricate family emotional patterns, ambivalent feelings across generations,



and difficulties in accommodating communication needs. However, none of these are sufficient to predict or guide the process that family members use to communicate about dementia diagnosis and emotions. Although assessment instruments can also be used to understand dementia communication, Kim & Liu (2023) argued that existing instruments are still in early stage development and yet to be validated among the dementia population and thus are unable to support hypothesis testing on whether families communicate in a certain pattern. Therefore, the current study requires inductive reasoning to understand the phenomenon rather than deducing and testing hypotheses from existing theory. Thus, it is appropriate to apply the Grounded Theory approach and extend the theoretical foundations of how intergenerational family members communicate about dementia-related issues in the Chinese context. In the next chapter, I will justify the selection of the Grounded Theory methodology, its implementation, limitations, and implications.

Table 2. Summary of Existing Theoretical Perspectives on Family Communication

	Family system theory	Theory of intergenerational ambivalence	Communication accommodation theory
Key focus	Family system functioning	Complexities of family relationships	Accommodative style and social identity
Key variables	(1) Emotional fusion and differentiation of self; (2) Triangles; (3) Nuclear family emotional system; (4) Family projection process; (5) Emotional cutoff; (6) Multigenerational transmission process; (7) Sibling positions;	(1) Structural ambivalence: reproduction and innovation; (2) Psychological ambivalence: convergence and divergence	Four accommodation strategies: (1) approximation strategies; (2) interpretability strategies; (3) discourse management strategies; (4) interpersonal control strategies



(8) Societal
regression

Hypothesized relationships between variables	The emotional symptom of a patient is the reflection of the emotional symptom of the family, which is embedded in the patterns and behaviors of past generations.	Four variables generate four family types: (1) emancipation; (2) solidarity; (3) captivation; (4) atomization.	The four strategies generate three problematic accommodation behavior: (1) overaccommodation; (2) under accommodation; (3) non-accommodation
What to explain	How the family emotional patterns developed and transmitted down generations (Brown, 1978)	Why intergenerational relations generate ambivalence and how to cope with it (Lüscher & Pillemer, 1988)	How and why people shift their communication to or from those with whom they interact (Giles, 1973)
What to predict	The emergence of chronic anxiety in the family	The emergence of ambivalent feelings	The emergence of accommodative communication behaviors
Critique	(1) Disregarding the shortest possible solutions towards family therapy; (2) Gender bias; (3) Lack attention to feelings or expression of emotions	(1) Lack of operationalization in measuring ambivalence; (2) Absence of a clear causal model; (3) Lack of contextualized measurements	(1) Reductionist; (2) Difficult to capture shifting communication strategies; (3) Assuming communication as rational and ignoring conflicts



Chapter 3: Methodology

Carter & Little (2007) argue that high quality qualitative research should demonstrate internal consistency among three fundamental facets of research: epistemology, methodology, and method. Beliefs about reality (ontology) determine what we construe as legitimate knowledge (epistemology), how we obtain it (methodology), and the principles of scientific investigation and research techniques (method) (Slevitch, 2011). In this chapter, I first state my ontological and epistemological position. Then I put forward my research questions and justify the selection of methodology and methods. The Constructivist Grounded Theory approach will be used to guide data collection and analysis. I introduce the inclusion and exclusion criteria, recruitment strategies, ethics concerns, distress protocol, and how I ensure the scientific rigor with the TACT framework. This chapter concludes by presenting the research activities and participants' profiles.

3.1. Ontological and Epistemological Position

Ontological positions refer to the nature of physical and social reality (Blackburn, 1993; Bruner, 1990). Realism and relativism are the two polarized orientations on the continuum describing objective reality at one end to multiple realities on the other (Andrews, 2012). While quantitative research postulates objectivism in that the ultimate truth exists and there is only one truth, the qualitative tradition is based on interpretivism and constructivism, asserting multiple possible realities (Slevitch, 2011). Thus, the belief in relativism forces focus on the process of knowing, which is coined *epistemology*. Epistemology concerns how we know what we know, what is truth, and the relationship between the investigator and investigated (Slevitch, 2011). Social constructionism, introduced by Berger & Luckmann (1991 [1966]), holds that knowledge is created by the interactions of individuals within society, and it is the product of specific cultural, political, and historical contexts (Schwandt, 2003). Although social constructionism accepts objective reality in the natural world, it primarily focuses on the subjective experience of everyday life, how



subjective reality is socially defined, and how the world is understood (Berger & Luckmann, 1991). The world can only be known in relation to people's experience of it (Andrews, 2012). Therefore, subjective reality is shaped, modified, and reconstructed by socialization, conversation, and interaction with others (process), among which conversation is the most important means (Berger & Luckmann, 1991).

The key concepts in the current study, family, dementia, and communication, are consistent with social constructionism. First, Burgess (1926), the pioneer of family sociology, proposed that the "family" is essentially a process, an interactional system influenced by each family member rather than merely a structure. Second, the experience and behavior of PLwD and family caregivers are socially constructed through the interplay of conversational and social practices (Adams & Gardiner, 2005). Third, the inferential nature of communication can be understood as "constructing understandings, identities, relationships and even dementia itself" (Forbat, 2003, p. 71). When PLwD and family caregivers communicate, they make various inferences about meaning, and it is valuable to learn about their disease experience through the lens of family communication (Hutchby & Wooffitt, 1998; Potter, 1996). Therefore, this paper's research enquiries are in line with social constructivism. The next sections will state the research questions and how to investigate what can be known (methodology) and strategies to be used in the scientific inquiry (method).

3.2. Research Questions

In summary, the literature review identifies three research gaps. Firstly, although the family is the most important communicative partner in the "long haul" of the disease journey, little is known about how families communicate about dementia beyond spouses and adult children and how adult grandchildren are involved in the communication process. As grandchildren play a prominent role in Chinese families, the current study uses grandchildren as the entry point to examine the family



communication process. Secondly, three family communication processes remain unknown, specifically the initial decision to undergo dementia diagnosis, how families inform others about the diagnosis and discuss the uncertainties, and how they communicate during emotional arousal. Thirdly, research on family communication about dementia among Chinese families is scarce, and little is known about how subtle and intrinsic Chinese communication culture shapes the family communication process. The overarching research question is “How do intergenerational family members communicate about dementia-related issues?” with three sub-research questions below:

1. Initial communication: Who (PLwD, adult children, grandchildren) initiates the first medical screening for probable dementia and why? What is the relationship between this person and PLwD?

2. Post-diagnosis communication: Who and how did the family member disclose the dementia diagnosis to the patient, family members from different generations, relatives, and friends? How did they respond to the news?

3. Emotional communication: How did family members, including PLwD, communicate with each other under negative (frustrated), positive (happy), and normal emotions? [The reflected episode should be interactions of more than 30 minutes]

3.3. Methodology: Constructivist Grounded Theory

Theories offer accounts for what happens, how an event ensues, and may also aim to account for why an event happened (Charmaz, 2014, p. 228). Grounded theory is an abductive method involving reasoning about an experience, making theoretical conjectures and inferences, and checking them through further empirical data (p. 201). It emphasizes analyzing social and social psychological processes within a social setting or a particular experience (p. 253). What distinguishes grounded theory from other qualitative methods (e.g., phenomenology) is that grounded theorists analyze actions and processes instead of themes and structure. Rather than



describing and summarizing the studied phenomena with extant theories, the grounded theory approach pursues analyzing data, developing categories, and constructing theory (p. 15). Therefore, grounded theory is well suited for learning what is happening and how it happened in family communication about dementia.

There are positive and interpretive definitions of theory, with differences surfacing in objectivist and constructivist grounded theory (p. 234). While the positive tradition seeks to increase validity and reliability to maximize generalization, interpretivists seek to learn contextualized meanings and actions (**Table 3**). Constructivist grounded theory has its roots in symbolic interactionism, which derives from the pragmatist philosophical tradition that assumes a theory's value rests on its effective practical application. Pragmatism views reality as fluid, somewhat indeterminate, and open to multiple interpretations (p. 263). Symbolic interactionism is a dynamic theoretical perspective that views human actions as constructing self, situation, and society. It assumes that language and symbols are crucial in forming and sharing meanings and actions. Our ability to communicate with ourselves and others allows us to rethink, recast, and redirect our views and actions (p. 270). The tentative research question is consistent with pragmatism and symbolic interactionism, since family communication about dementia is a form of interaction, a symbolic process that depends on shared language and meanings and is open to interpretation. Therefore, this study follows social constructivism and adopts the methodologies of constructivist grounded theory (Charmaz, 2006; 2014).

Constructivist grounded theory assumes that any theoretical rendering offers an interpretive portrayal of the studied world, not an exact picture of it (Charmaz, 1995b, 2000a; Guba & Lincoln, 1994; Schwandt, 1994). Research participants' implicit meanings, experiential views, and the resultant finished grounded theories are constructions of reality.



Table 3. Objectivist and Constructivist Grounded Theory

	Objectivist/Classical Grounded Theory (Glaser & Strauss, 1967)	Constructivist Grounded Theory (Charmaz, 2006; 2014)
Ontology	Realism	Relativism
Epistemology	Objectivism	Social constructivism and pragmatism
Theoretical perspective	Positivism	Interpretivism and symbolic interactionism
Foundational assumptions	- Assumes discovery of data - Assumes conceptualization emerges from data analysis - Views representation of data as unproblematic - Assumes the neutrality, passivity, and authority of the observer	- Assumes mutual construction of data through interaction - Assumes researcher constructs categories - Views representation of data as problematic, relativistic, situational, and partial - Assumes the observer's value, priorities, positions, and actions affect views
Objectives	- Aims to achieve context-free generalizations - Aims for parsimonious, abstract conceptualizations that transcend historical and situational locations - Aims to create theory that fits, works, has relevance, and is modifiable	- Views generalizations as partial, conditional, and situated in time, space, positions, action, and interactions - Aims for interpretive understanding of historically situated data - Specifies range of variation - Aims to create theory that has credibility, originality, resonance, and usefulness
Implications for data analysis	- Views data analysis as an objective process - Sees emergent categories as forming the analysis - Sees reflexivity as one possible data source - Gives priority to researchers' analytic categories and voice	- Acknowledges subjectivities throughout data analysis - Views co-constructed data as beginning the analytic direction - Engages in reflexivity throughout the research process - Seeks and represents participants' views and voices as integral to the analysis

Adapted and expanded from Kathy Charmaz (2007).



Previous dementia communication research uses different methodologies, such as observing naturally occurring conversations between PLwD and family (Kindell et al., 2017). These analytical methods usually contain audio/video recordings of conversations, exploring sequential interactions, and observing verbal and non-verbal cues directly from the data rather than predefined hypotheses (Kindell et al., 2017). However, the nature of observing naturally occurring conversations is different from other qualitative work since it pursues descriptive and specific interaction rather than exploring meanings, views, and understandings. A retrospective approach can allow time for interviewees to reflect and digest the experience and construct meaning with deeper understanding through in-depth interviews with the researcher. A constructivist approach prioritizes the studied phenomenon and sees both data and analysis as created from shared experiences between the researcher and research participants. Researchers are part of the world they study, the data they collect, and the analyses they produce through interaction with people, perspectives, and research practices (p. 17). Constructivists study *how* and *why* participants construct meanings and actions in specific situations, and this further extends to learning how, when, and to what extent the studied experience is embedded in the larger, but often hidden, social relationships, situations, networks, and structures (p. 239). Through constructivist grounded theory analysis, variations between different experiences can become visible, as well as power hierarchies, communication strategies, and the opportunities that maintain and perpetuate such distinctions (p. 240).

Grounded theorists can produce two kinds of theories: substantive or formal. This study will produce a substantive theory addressing delimited problems in a specific substantive area: family communication about early stage dementia. Although grounded theory can reach across substantive areas and move into the realm of formal theory, the current study is not capable of addressing problems across multiple substantive areas. Despite this formal theory development should be the long-term goal. Glaser and Strauss (2017), who originally developed grounded



theory, suggest that a finished substantive/formal grounded theory should explain the studied process in a new theoretical term, explicate the properties of the categories, and demonstrate the causes and conditions under which the process emerges and varies, as well as delineate its consequences (p. 10). By entering participants' worlds, I will locate their meanings and actions in larger social structures, ideologies, conventions, discourses, and power relationships of which they may be unaware (p. 241). Constructivist grounded theory enables me to transcend situated actions and make relationships between implicit processes and structures visible.

3.4. Method

3.4.1. Data Collection Procedures and Analysis

Grounded theory is a way of conducting inquiries that shapes data collection and emphasizes analysis (p. 17). In this section, I will delineate key grounded theory steps (**Figure 2**) and show how I plan to increase the conceptual precision of emerging ideas through systematic data-gathering and analytical strategies.

In grounded theory, the general research topic informs initial methodological choices for data collection (p. 320). *First-round* interviews will be conducted through purposive and convenience sampling to explore the general research idea (**Appendix I**). The primary goal of the first-round interview is to understand the research participants' perspectives, meanings, and experiences and elicit their own interpretation. I will delve into intriguing inquiries, hints, implicit views, and action accounts during the interview and interpret what the participants take for granted or do not state (p. 34, p. 56). Then, I will analyze the initial interviews by separating, sorting, and synthesizing data through qualitative coding. *Coding* means attaching labels to segments of data to depict what each segment is about and make comparisons with other segments. While coding and making comparisons, I will write preliminary analytic notes, called *memos*, about my ideas and reflections. Through studying data, comparing them, and writing memos, I will define the ideas



that best fit and interpret the data as tentative analytic categories. The analytic categories should be emergent and constructed from data, not dictated by preconceived logically deduced hypotheses or forced into the data.

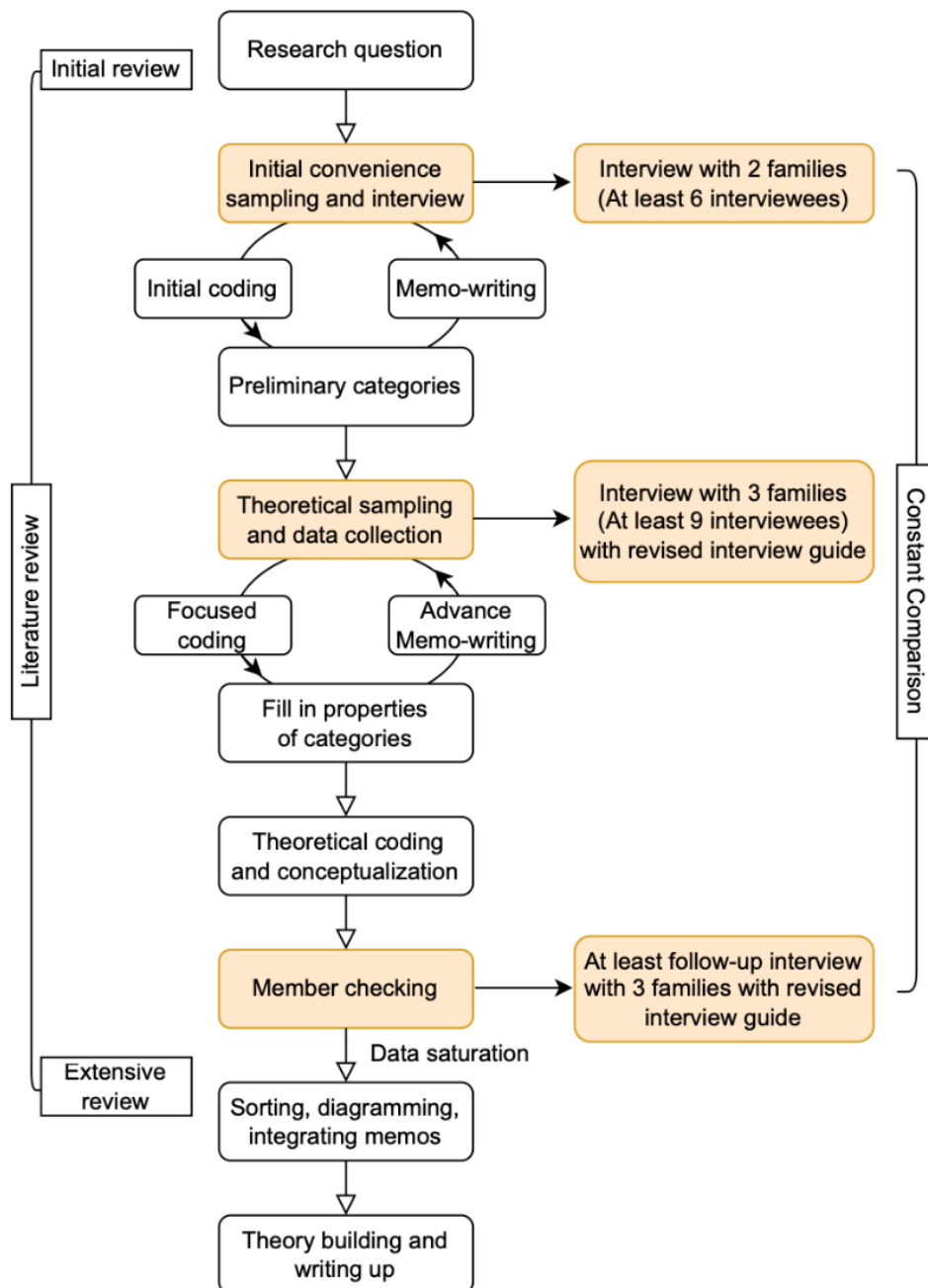


Figure 2. A visual representation of the key grounded theory steps

Through the first-round interview, I will learn what the research participants view as problematic during the family communication process and treat this analytically.

These preliminary categories will point to areas to explore during the *second-round* data collection, called *theoretical sampling*. Theoretical sampling aims towards theory construction by elaborating on the properties of a category, demonstrating links among categories, and strengthening the theoretical power of categories, but not for population representativeness (p. 8, p. 205). The initial interview questions are subject to change since the emerging categories allow me to delve into problematic topics that emerge in the field or pursue new areas that had not yet occurred to me. Using the *constant comparison approach*, I will compare the events and experiences that initial participants talk about with codes and memos from subsequent interviews. The material for comparison is not limited to interviews but also includes other material provided by the interviewees, such as family conversation records (e.g., in WeChat), conversations with health professionals, and other relevant documents. The relationships drawn from these categories will provide a conceptual handle on the studied experience and allows me to build levels of abstraction from the empirical data.

Finally, I will gather *third-round* additional data to check and refine the emerging analytic categories by following up with key interviewees, which is also called member checking. By taking ideas back to research participants for confirmation, earlier open-ended and participant-centered interviews may shift into a mutual conversation about the theoretical categories, which is particularly important under the subtle, indirect, and assumed Chinese communication culture (p. 19). For example, Cheryl and Dan Albas (2004) explained the major categories to certain participants and asked to what extent these categories fit their experience. If their agreement is bland, they conclude that the categories have not penetrated the core of experience and thus engage in follow-up discussions with interviewees to generate new categories (p. 210). Although new data will always provide new insight, the grounded theory approach suggests stopping data gathering when the properties of the theoretical categories are “saturated” with data (p. 213). Theoretical category saturation does not mean the witnessing repetitive events,



stories, statements, or patterns; rather, it occurs when, and indicates that, new data will not add to the core category and conceptualization model. Therefore, when data is saturated through the three rounds of data gathering, my work will culminate in a middle-range grounded theory, or an abstract theoretical understanding of the studied experience (p. 4).

In summary, data analysis consists of 1) informal analysis and memoing during the interviewing and transcription stage, *initial coding* (open to all possible theoretical directions indicated by data) of the **first-round** interviews; 2) theoretical sampling, advanced memoing, and *focused coding* (most salient codes) of the **second-round** interviews to enrich categories; 3) *theoretical coding*, theoretical memoing, diagramming, and conceptualization in conjunction with the **third-round** interviews. All interviews will be transcribed verbatim. NVIVO, a computer-assisted qualitative data analysis software program, will assist in coding and analyzing.

3.4.2. Inclusion/Exclusion Criteria

Families with a patient who, at the time of the interview, had already been diagnosed from the memory clinic in a general hospital, or outpatient and inpatient neurology, psychiatric and geriatric departments will be eligible for this study since formal diagnosis is only available from these hospital departments in China (Zhao et al., 2022). Although the focus of this study is people with early stage dementia, family members (spouses, adult children, adult grandchildren) of patients with middle- to late-stage dementia were also considered. Seeking a diagnosis is generally delayed for 2 years after first recognizing cognitive symptoms of dementia in China, and some types of dementia can worsen very quickly from early to middle and late stages (Zhao et al., 2016). Therefore, involving families dealing with middle- to late-stage dementia increased the study feasibility and family members could provide more in-depth reflection when recalling their early stage experiences compared with later stages.



Table 4. Inclusion and Exclusion Criteria of the Study Sample

	Inclusion criteria	Exclusion criteria
People with early stage dementia	(1) People diagnosed with dementia by physicians (≥ 60 years); (2) people who have received formal diagnosis from a memory clinic in a general hospital, or outpatient and inpatient neurology, psychiatric and geriatric departments in mainland China; (3) people possessing the communication capacity to take part in an interview; (4) people able to provide informed consent or assent; and (5) people living in the community	(1) Severe impairment in communication; (2) inability to give informed consent; and (3) severe BPSD symptoms
Spouse of PLwD	(1) Spouse of a person living with dementia; (2) no cognitive impairment; (3) no communication difficulties; and (4) able to provide informed consent.	
Adult children (Depending on whether the PLwD is paternal or maternal grandparent)	(1) Adult children of a person living with dementia; (2) aged 18 or older; (3) no cognitive impairment and (4) no communication difficulties; and (5) able to provide informed consent.	
Adult grandchildren	(1) Grandchildren or younger family members of a person with living dementia; (2) aged 18 or older; (3) no cognitive impairment; (4) no communication difficulties; and (5) able to provide informed consent.	

PLwD = People living with dementia

PLwD with severe BPSD, as reported by family members, will be excluded since involving them in in-depth interviews may exacerbate the practical difficulties (Abbato, 2015). In addition, since the grandparent generation may have multiple



children, the recruitment of the adult parent generation will depend on whether the patient is a paternal or maternal grandparent. For example, if the grandmother from the grandchildren's mother's side is diagnosed with dementia, then the mother will be recruited. Only if the grandchildren's mother or father is unavailable, will other siblings be considered in the interview. The word "caregiver" will be deliberately avoided in participant recruitment since communication is not confined to caregiving relationships but is an integral part of every relationship within the family (Purves & Phinney, 2012). All the interviews will be conducted individually online or face to face. Before each interview, the interviewee's general information will be collected, such as age, gender, education level, living status, marital status, and geographic area. The interview guide for each generation is attached in **Appendix I**.

Ideally, at least one family member needs to agree to be interviewed from each of the three generations (adult grandchildren, adult children, partner of PLwD, grandparents with early stage dementia). However, as previous family research scholars have pointed out, this sample strategy can be difficult and costly since one refusal means losing a whole family unit (Hagestad, 2006). There are also "contamination" concerns since family members may discuss the interview questions and reorganize answers privately (Hagestad, 2006). Due to the sensitive nature of discussing dementia especially in a Chinese context, carers may be protective of PLwD and worry that they might become upset and refuse to participate (Connell et al., 2001). Nonetheless, while taking a three-generational perspective on family research, there are two different but equally effective approaches: one is to gather data from grandparents, parents, and grandchildren, one member from each generation; the other is to obtain data from only one generation while occasionally considering the other two generations' viewpoints (Hagestad, 2006). Therefore, this study will primarily take a three-generational approach by trying to reach at least one family member from each generation. It is acceptable if PLwD wish caregivers to accompany them to interviews so as to



provide prompts and clarifications. However, since discussing emotional issues is rather personal, the presence of others may inhibit authentic expression and compromise autonomy, and thus interview companions shall only be accepted if the PLwD insists on being accompanied. If the conditions are undesirable, it is also feasible to recruit three generations from different families (Jiang et al., 2007). These inclusion and exclusion criteria are summarized in **Table 4**.

3.4.3. Ethics Concerns and Distress Protocol

The ethics approval of this study was obtained from the Human Research Ethics Committee (HREC), The University of Hong Kong, on August 7, 2023. The HREC reference number is EA230303, and the award letter is attached in **Appendix II**.

The involvement of PLwD in qualitative research, particularly involving in-depth interviews, poses ethical and methodological issues that require critical examination (Novek & Wilkinson, 2019). Although all research involving human participants has inherent risks, these risks are heightened in dementia research due to the sensitive nature of the disease, the impaired cognitive-communicative abilities of the PLwD, and the attendant social stigma (Hellstrom et al., 2007; Pesonen et al., 2011; Garand et al., 2009). However, although interpreting PLwD's words may not be straightforward due to their cognitive-communication problems, previous studies have demonstrated that information provided by PLwD is genuinely authentic (Digby et al., 2016). The participant's ability to communicate is also influenced by the listening and speaking skills of the interviewer (Hubbard et al., 2003). Therefore, based on the previous works and recommendations, nine principles will be followed to interact safely and ethically with PLwD (Novek & Wilkinson, 2019; Digby et al., 2016; Cridland et al., 2016).

(1) Informed consent: The process of signing informed consent pertains to an individual's ability to understand the research protocol and decide whether they voluntarily participate in the research (Black et al., 2008). Although dementia



compromises decision-making abilities, a formal diagnosis or even a low cognitive test score does not necessarily indicate inability to give informed consent (Cridland et al., 2016; McKeown et al., 2010; Woods & Pratt, 2005). Contrary to traditional competence-based informed consent, Dewing (2007) has argued for a more person-centered and situation-specific approach to consent. Previous research has proved that those in early to moderate stages of dementia have the ability to provide informed consent and participate in decision-making (Fetherstonhaugh et al., 2013; Karlawish, 2008). Even people with moderate to severe dementia suffering advanced impaired capacity may still be able to make a choice and show some understanding (Hegde & Ellajosyula, 2016). Therefore, it is a good practice to consider PLwD being competent of providing informed consent unless indicated otherwise by professionals (Agronin, 2014). When the researcher explains the concept of informed consent, PLwD will be deemed incapable of making informed consent if she/he cannot fully understand different choices, their implications, and the associated risks and benefits (Karel et al., 2010; Woods & Pratt, 2005). In this case, proxy consent (e.g., by family caregivers) together with PLwD's assent will be employed as recommended by the Alzheimer's Association (2004). Consent is a written agreement to participate in research, while assent is a verbal agreement based upon less than full understanding (Beuscher & Grando, 2009). In both scenarios, a process consent approach will be adopted, including getting to know the person, seeking their initial consent/assent, and checking their consent/assent during the interview. This method is regarded as the most appropriate when interviewing PLwD by Dewing (2007). PLwD's nonverbal behavior will also be closely monitored for signs of inconvenience, refusal or objection, or any indication of a desire to withdraw from the study (Mazaheri et al., 2013; Meulenbroek et al., 2010).

(2) Time and location: The researcher should ask about PLwD's preferred time and location before the interview. It is common to conduct interviews in their home to create a sense of familiarity and maximize comfort instead of a healthcare setting



with an underlying connotation of disease. The interview appointment should be outside of late afternoons when PLwD may be tired and find difficulty concentrating. The researcher should periodically evaluate the needs of PLwD and when they may need a break during the interview and adjust the interview pace according to their preference. Allowing choice and flexibility for PLwD can give them control and empowerment over the interview process which in turn can facilitate more authentic responses.

(3) Rapport between researcher and interviewee: Researchers should have genuine engagement with PLwD before the interview. Previous studies recommend strategies such as having informal conversations with PLwD before and after the interview and conducting interviews over several time points. However, since this study conducts all interviews online, it would be difficult to physically connect with the research participants. As the younger generation is more familiar and comfortable using the Internet, it would be easier for the researcher to build trust and rapport with the adult grandchildren first and let them introduce the researcher and study to the PLwD to create a sense of familiarity.

(4) Clarification of the role of the researcher: The researcher should take steps to help PLwD understand the role of the researcher and may need to regularly reiterate it to avoid role confusion during the interview. PLwD may forget who the researcher is or the purpose of the interview due to cognitive problems. If role and purpose are not repeatedly clarified, PLwD, particularly those with unmet needs, may seek advice about their condition, treatment options, and available resources (e.g., Pesonen et al., 2011; Pratt, 2002). This may raise ethical difficulties in addition to wasted time and confusion.

(5) Communication strategies: It is inevitable for PLwD output to feature digressions, repetitions, and tangential conversations (Digby et al., 2016). The researcher must balance between allowing PLwD to discuss what matters to them



and bringing the topic back tactfully. Tangential conversations can be a protective strategy for PLwD to avoid discussing sensitive issues or subjects they do not remember (Digby et al., 2016). PLwD may also have unpredictable thoughts and utterances far from the researcher's anticipations, requiring the researcher's flexibility to respond to and manage the conversation flow.

(6) Withdrawing from the interview: Some participants may feel a sense of loss when the interview and research relationship ends (Hewitt, 2007). Therefore, it is important for researchers not to leave the interview hastily after obtaining all information. The researcher should continue having casual conversations, show interest in the PLwD, and leave them with a feeling of positive contribution before the interview ends (Hellström et al. 2007).

(7) Feedback and knowledge dissemination: The researcher should discuss with PLwD their expectations of the interview and whether and how they would like to keep in touch and learn about the research result in the future. It is important to share with them if this is what they want, or they may feel used and devalued.

(8) The use of the term “dementia”. Through the pilot interviews, I noticed that some families did not disclose the diagnosis to PLwD since they did not want to make him/her feel overwhelmed or stressed. This situation also happens in other disease research. Pratt (2002) noticed that some participants may be unaware of their diagnosis at the time of research. Even healthcare professionals may use the term “memory problems” while disclosing dementia (Bartlett & Martin, 2002). Given the shame and stigma associated with dementia, some PLwD may reject the disease label (Garand et al., 2009). Therefore, to ensure safe and ethical research practice, Novek & Wilkinson (2019) suggest developing a plan to communicate sensitively with participants who are unaware of their diagnosis. For example, Steeman et al. (2007) noticed that only 9 out of 20 families with dementia in their study informed PLwD of the diagnosis, while others were told they had “memory



problems.” Thus, researchers use this term in their research introduction and interviews rather than “dementia” (Steeman et al., 2007). In the current study, I will first consult family caregivers on whether the PLwD is aware of the diagnosis and receptive to talking about it. If not, I will change all the dementia-related terms in the consent form and interview guide into “memory loss” or “memory problems” and show them to the caregivers in advance. If they deem any question risks revealing the diagnosis or causing discomfort, I will further revise or remove the question. Therefore, considering the sensitive nature of dementia and the importance of involving PLwD in social research, a certain degree of deception is justifiable when explaining the study purpose and conducting the interview by shifting the focus from “dementia” to “memory loss.” An alternative version of the interview guide designed for this group is shown in **Appendix I**.

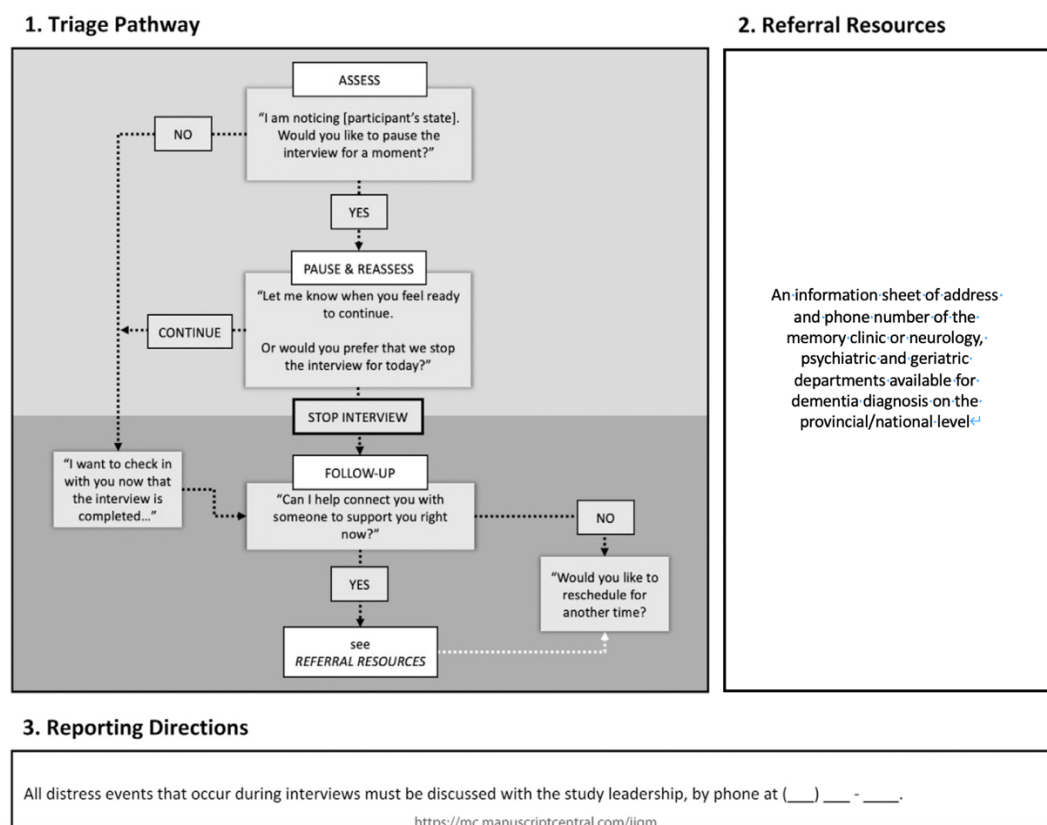


Figure 3. Distress protocol

(9) Distress protocol for all interviewees. Reflection and communication about dementia can be stressful and emotionally demanding for grandchildren, adult children, and people with dementia. Particularly the interview question in this study involves reflecting on frustrating communication with family members. For PLwD, misunderstanding the researcher's role can also cause anxiety, and difficulty communicating and recalling can lead to distress or agitation (Hellstrom et al., 2007; Pesonen et al., 2011). Therefore, the current study adapts Whitney & Evered's (2022) distress protocols to deal with participant distress. Distress protocols (see **Figure 3**) are a comprehensive and systematic method for assessing and responding to participant distress safely and consistently, instead of through researcher countertransference (Whitney & Evered, 2022). This method offers structured and standardized language to respond to unforeseen situations that may emerge from and during the interview process. First, when the interviewee shows signs of distress, researchers will check if, when, and how participants would like to proceed. Sometimes researchers may prematurely stop an interview due to inability to cope with emotional situations, however, this may be viewed as rejecting the participant's feelings (Morse et al., 2008). Interviewees usually want to continue the interview even if it triggers difficult emotions (Pratt, 2002; Moore & Hollett, 2003; Pesonen et al., 2011). Therefore, offering breaks and letting the interviewee talk through difficult issues would be more helpful. Ending an interview prematurely may limit the transferability of the study result and unfairly overburden other participants. Secondly, these protocols provide scripts for available resources that participants can assess if needed. As such the researcher will distribute an information sheet detailing all memory clinics or neurology, psychiatric and geriatric departments available for dementia diagnosis on the provincial/national level. If the interviewee decides to stop, the researcher should engage in informal conversations until he/she no longer appears distressed and end the interview by validating their strength and support. Third, all participant distress and referral requests will be notified to the supervisor to determine the next step. Through applying distress protocol approach, participants are expected to experience both



decreased stress and a beneficial therapeutic effect by participating in the research (Carter et al., 2020; Lockwood et al., 2018).

3.5. Scientific Rigor: The TACT Framework

The “Trustworthiness, Auditability, Credibility, and Transferability (TACT)” framework proposed by Daniel (2019) will be adopted to achieve rigor in this qualitative study. First, trustworthiness refers to quality in the research process and outcome. It requires the researchers’ reflexivity to acknowledge the experience they bring to guide their understanding of data, assumptions of the phenomenon they studied, and personal bias. A systematic process will be employed to show how the researcher arrived at a conclusion. Second, auditability refers to a high degree of transference in qualitative research. It requires the researcher to keep records of all the decisions made during the research process and all the files they used to document their thought processes. The transparent data collection and analysis process can facilitate verifying the research outcome by other researchers or the research participants. Third, credibility in qualitative research requires that findings are dependable, relevant, congruent, and reflect the intended reality that is obtained from the interviewee’s experience. I will bring back my result to research participants for verification (member checking). I will also invite other professionals in this field to explore the data independently and get a more comprehensive understanding of the phenomenon (investigator triangulation). And finally, transferability means findings gained from a particular context that can offer insights into other similar contexts. In other words, the research findings need to be meaningful to those not involved. Therefore, I will delineate the particular context of this study and recruit participants who most likely represent the general experience. With the TACT framework, I can provide adequate justification for the research method, maintain the research integrity, and increase the authenticity of research outcomes.



3.6. Overview of Research Activities

Methods for conducting sampling in grounded theory vary according to the specific study (p. 205). In this current study, the first-round interviews consisted of two recruitment approaches spanning October 2023 to January 2024, as summarized in **Table 5**.

Table 5. Timeline of Major Research Activities

Time Period	Major Research Activity
April 2023	Pilot interview
July–August 2023	Application for ethical approval
October–December 2023	Data collection: first-round interview through social media
December 2023–mid-January 2024	Data collection: first-round interview through fieldwork in Shanghai, China
January–April 2024	Data analysis (writing fieldnotes, coding, writing memos)
April 2024	Data collection: second-round interview (theoretical sampling)
May 2024	Data collection: third-round interview (member checking)

From October to December 2023, I recruited families with dementia by looking for potential interviewees, the adult grandchildren of PLwD, through Chinese social media platforms (e.g., Sina Weibo and Xiaohongshu). Weibo and Xiaohongshu are currently the most popular integrated social media platforms in mainland China (Pemarathna, 2019). In 2020, 83.31% of active users of Xiaohongshu were between 18 and 34 years old, similar to Weibo users, of whom 75% are between 18 and 30 years old (Guo, 2022). Therefore, both platforms are suitable for recruiting adult grandchildren, whose age range likely makes them among the majority of active users. I reached out to those who shared relevant experiences online. The key search terms were “Alzheimer’s disease,” “dementia,” and “grandparents.” I sent messages to them, explained the research details, and asked for consent, including whether their parents and grandparents with dementia were willing to be interviewed. Through this approach, I successfully recruited five adult grandchildren and one adult child caregiver and had online in-depth interviews.

Although the interviewees showed interest in inviting their grandparent or parent with dementia for an interview, it turned out to be unfeasible because some were reluctant to be interviewed by a stranger or felt uncomfortable talking online. Because many adult grandchildren do not live with their grandparents, they also lacked the momentum to persuade their parents to arrange such interviews. I also encountered difficulty locating early stage PLwD online.

Therefore, from December 2023 to mid-January 2024, I volunteered at two daycare centers caring for PLwD in Shanghai, where I closely interacted with both PLwD and their family caregivers and built rapport. In the first 2 weeks, I intensively interacted with PLwD by organizing group activities, visiting their homes, and arranging community outreach health talks. I further learned about the potential participants' disease experience from social workers to obtain the accurate time of and severity at medical diagnosis and gain a deeper understanding of their family background. I also checked with social workers about whether each PLwD would be comfortable if I used the term "dementia." In the remaining 4 weeks, I started interviewing eligible early stage PLwD and family caregivers, mostly recruited in patient-child or patient-partner dyads. Although my target was early stage PLwD, I was surprised to find that some PLwD in the middle and late stages could communicate normally and had the desire to communicate in their good moments. Thus, a small number of middle- to late-stage PLwD were also recruited. Formal interviews were conducted either at the daycare center or at their homes when they reported being comfortable and not tired. I clarified my role as a researcher with the participants, because their initial impression of me was a volunteer. I also had interviews with volunteers and social workers who at the same time, were dementia caregivers.

After the data collection was completed in January and preliminary categories were generated, I went back to my participants to check my ideas against the empirical world through second-round interviews in April 2024. I reinterviewed five



participants with more focused questions, including three adult grandchildren and two adult children. Finally, after finishing the theoretical sampling and conceptualization, I returned to the five participants for member-checking in early May. I examined whether and to what extent these categories fit each participant's experience. My research activity stopped when no new properties of the theoretical categories emerged or in other words, I achieved data saturation.

3.7. Demographic Profiles of Participants

In total, twenty-eight people participated in this study, including six adult grandchildren, five adult children, seven partners of PLwD, nine PLwD, and one sister-in-law. Among these interviewees, one adult child cared for two PLwD, her mother-in-law and grandmother; and one spousal caregiver cared for two PLwD, his mother (deceased) and wife. The sister-in-law, who cared for her brother-in-law (deceased), happened to be the social worker at the NGO where I volunteered. The interviewees also included five dyads: two child–parent dyads (Child 2 and PLwD 4, Child 5 and PLwD 7) and three spousal dyads (Partner 5 and PLwD 9, Partner 6 and PLwD 8, Partner 7 and PLwD 1). Therefore, this study covered the experience of 25 unique families with dementia. The demographic information of the participants is listed from **Table 6** to **Table 10**, with each assigned a unique case ID. The average length of the interviews was 55 minutes, ranging from 20 to 139 minutes. Data analysis was conducted using NVivo. In total, 3,286 codes were generated with 4,867 references.

Grounded theory is a decidedly emergent process of learning and interpreting research participants' views of their experiences (Charmaz, 2014, p. 106). Because the constructivist grounded theory approach does not seek generalizability or representativeness but rather a detailed understanding of a phenomenon, sampling adequacy is more crucial than sample size when determining category saturation (Bowen, 2008, p. 214). Therefore, in the current study, the sample size of 28 participants was adequate to address the research questions and generate a detailed



understanding of the studied phenomenon.

Of the 28 participants, eight were male and 20 were female. The mean age of the adult grandchildren interviewees was 26.3 years old, and all had earned a bachelor's degree or beyond. The mean age of the adult children interviewees was 51.6 years old, with an education background from middle school to a master's degree. The mean age of the spousal caregivers was 78.9 years old, with an education background from secondary vocational school to a bachelor's degree. The mean age of the PLwD was 80.1 years old, with an education background from uneducated to a bachelor's degree. The diagnostic year of dementia ranged from 1990 to 2023, with an average of 2018. In terms of dementia types, 16 PLwD were diagnosed with Alzheimer's disease, three were diagnosed with vascular dementia, two were diagnosed with mixed type (vascular dementia and AD), one was diagnosed with Parkinson's disease with dementia, and the remaining five were diagnosed with general dementia. Three PLwD did not receive a formal diagnosis or were diagnosed with brain atrophy. Among all PLwD, 14 were in the early stage, one was in the early to middle stage, four were in the middle stage, eight were in the middle to late stage, and two were in the late stage of dementia. Three PLwD resided in rural areas, and the remaining 25 resided in urban cities.



Table 6 Characteristics of Participants (Adult Grandchildren)

Case ID	Gender	Age	Education	PLWD is	PLWD's age	Diagnostic year	Dementia type	Dementia stage	Residence
Grandchild 1	F	25	Master's degree	Father's mother	82	2022	AD	Early to middle	Rural
Grandchild 2	F	27	Master's degree	Mother's father	80	2018	AD	Middle to late	Urban
Grandchild 3	M	27	Master's degree	Mother's father	86	2022	AD	Early	Urban
Grandchild 4	F	32	Bachelor's degree	Mother's mother	75	2020	AD	Middle to late	Urban
Grandchild 5	F	20	Bachelor's degree	Mother's father	85	2023	Dementia	Early	Rural
Grandchild 6	F	27	Master's degree	Father's mother	84	2018	AD	Middle to late	Urban

Table 7. Characteristics of Participants (Adult Children)

Case ID	Gender	Age	Education	PLWD is	PLWD's age	Diagnostic year	Dementia type	Dementia stage	Residence
Child 1	F	38	Associate degree	Mother	65	2023	AD	Early	Urban
Child 2	F	66	Middle school	Mother	86	2023	Dementia	Early	Urban
Child 3	F	52	Master's degree	Mother-in-law	70	2022	Parkinson dementia	+ Early	Rural
				Mother's mother	97	No formal diagnosis		/	Urban
Child 4	F	46	Bachelor's degree	Mother	84	2019	AD	Middle to late	Urban
Child 5	M	56	Bachelor's degree	Mother	80	2021	AD	Middle	Urban

Table 8. Characteristics of Participants (PLwD's Spouse)

Case ID	Gender	Age	Education	PLWD is	PLWD's age	Diagnostic year	Dementia type	Dementia stage	Residence
Partner 1	F	78	Secondary vocational school	Husband	78	2017	Vascular dementia	Middle to late	Urban
Partner 2	M	69	Secondary vocational school	Wife	71	2022	AD	Early	Urban
Partner 3	M	86	Bachelor's degree	Mother	90 (deceased)	1990	Brain atrophy	Late	Urban
				Wife	79	2021	AD	Middle to late	Urban
Partner 4	F	80	Bachelor's degree	Husband	88	2021	Dementia	Early	Urban
Partner 5	F	76	Secondary vocational school	Husband	80	2011	Vascular dementia + AD	Middle to late	Urban
Partner 6	M	85	Bachelor's degree	Husband	85	2016-2017	Dementia	Early	Urban
Partner 7	M	78	Middle school	Wife	79	2019	AD	Early	Urban

Table 9. Characteristics of Participants (Sister-in-law)

Case ID	Gender	Age	Education	PLWD is	PLWD's age	Diagnostic year	Dementia type	Dementia stage	Residence
Sister-in-law	F	65	High school	Brother-in-law	63 (deceased)	2013	Vascular dementia	Late	Urban



Table 10. Characteristics of Participants (People Living with Dementia)

Case ID	Gender	Age	Education	PLWD is	PLWD's age	Diagnostic year	Dementia type	Dementia stage	Residence
PLwD 1	F	79	Middle school	/	/	2019	AD	Early	Urban
PLwD 2	F	83	Bachelor's degree	/	/	2023	AD	Middle	Urban
PLwD 3	M	77	Secondary vocational school	/	/	No formal diagnosis		Early	Urban
PLwD 4	F	86	Not educated	/	/	2023	AD	Early	Urban
PLwD 5	F	73	Middle school	/	/	2021	AD	Middle	Urban
PLwD 6	F	78	High school	/	/	2023	Vascular dementia	Early	Urban
PLwD 7	F	80	Primary school	/	/	2021	AD	Middle	Urban
PLwD 8	F	85	Middle school	/	/	2016-2017	Dementia	Early	Urban
PLwD 9	M	80	Secondary vocational school	/	/	2011	Vascular dementia + AD	Middle to late	Urban

Note. AD = Alzheimer's disease



Chapter 4: Findings

In this chapter, I answer the three key research questions and explore the abstract understanding of intergenerational family communication about dementia. Guided by constructive grounded theory, I abstracted four core sequential categories from the initial coding—unseeable, unspeakable, unhearable, and unmovable—to explain why communication become problematic in some families but not others. Whereas unseeable and unspeakable are more related to those who initiate the communication (message sender), unhearable and unmovable relate to those who receive and react to the communication (message receiver). Using a metaphor to illustrate, in families with problematic communication, family members put invisibility cloaks on each other or PLwD that impede them from seeing each other, speaking to each other, hearing each other, and moving on to actions. In the following sections, I present the properties and codes under each category, with detailed explanations of the relationships among these concepts. I also spell out the detailed process subsumed by each category and code to understand what they communicate about, how they communicate, and why they communicate in certain ways, supported by direct quotes from the interviewees.

4.1. Unseeable 看而不見: Not Seeing Personhood

To begin, family members need to see each other, including PLwD, to enable the following three steps (speakable, hearable, movable). Seeable here does not simply refer to seeing the person on the surface level, but empathically seeing the core of their personhood, involving their rights, needs, preferences, autonomy, and so on. Thus, acts that put the invisibility clock on PLwD, whether by PLwD or family members and whether intentional or not, are the starting point of invisible communication. Two properties contribute to the process of not seeing personhood—namely, rejecting illness claims and dismissing self-consciousness, as shown in **Figure 4**. In other words, the personhood of PLwD is left unseen because the family either rejects the illness claims or dismisses the PLwD's self-consciousness. For each property, I delineate the generic process using codes in the following section.



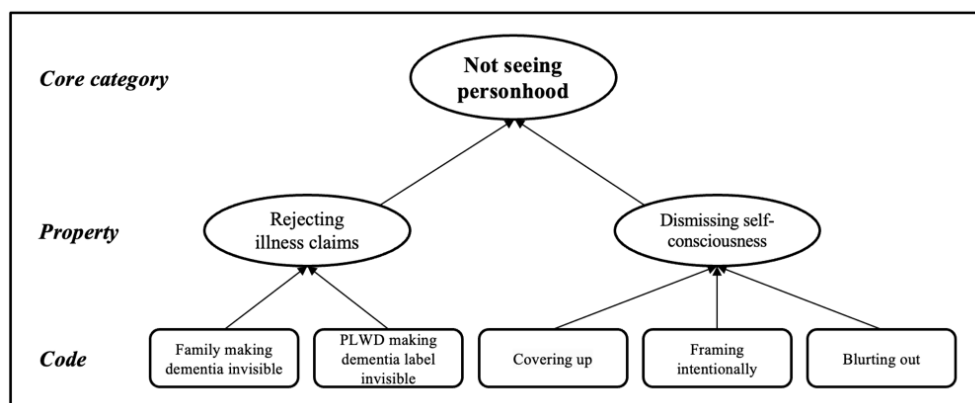


Figure 4. Illustration of the codes and properties under the core category of “not seeing personhood”

4.1.1. Rejecting Illness Claims

Illness claims can be either rejected by family members or PLwD, and I use the codes “family making dementia invisible” and “PLwD making dementia label invisible” to illustrate the two circumstances covered by this property.

4.1.1.1. Family Making Dementia Invisible

Family members can put the invisibility cloak of cognitive health, pathology, and treatment on PLwD and treat their symptoms lightly, holding the belief of fatalism.

- 1) Claiming no cognitive problems (making cognitive health invisible): Some family members may claim that cognitive problems are unseeable, as commonly occurs in early stage dementia. The cognitive symptoms are very subtle, and only family members who have lived with the PLwD for a long time are attentive to the emerging symptoms and might have certain dementia knowledge to detect the problem. Among our participants, this person was a grandchild, child, or partner who had intimate relations with the PLwD. However, when they try to raise this issue with other family members, they may evaluate the situation wrong due to the lack of knowledge and/or interaction with PLwD. Because early stage PLwD usually have normal functional and physical health and can even take care of others,

family members might deem it fine as long as they can maintain their functional and physical health and there is no need to worry about cognitive health. Thus, they put the invisibility cloak on the PLwD's cognitive health and reject the illness claims because they cannot see any issues.

They don't seem to take it seriously. They think that Grandma can walk, eat, dress on her own, she can take care of herself and even Grandpa. She doesn't seem like she's sick. (Grandchild 1, 25)

- 2) Memory decline as normal aging (making pathology invisible): Although cognitive problems are visible in some families, in line with the fatalism belief of dementia, many of them regard memory decline as normal aging rather than disease. In response to illness claims, they may use their own health as an example, indicating that everyone will decline while getting old. By making the disease invisible, family members accept what they view as their fate rather than fight against something unchangeable.

Don't overthink it. Your grandmother is getting older, and her body is definitely not as strong as before. We have to accept this reality. My and your mother's health are the same, and it will happen to everyone. (Grandchild 1, 25, sharing a conversation record with her father)

It's just normal aging combined with taking medication on time. (Grandchild 3, 27)

- 3) Folk explanation of dementia (making pathology invisible): Even if cognitive problems are visible in some families, they might adopt superstitious beliefs rather than accept an illness claim, a phenomenon that occurs mostly in rural areas. They put the invisibility cloak on the disease and believe a fortune teller can cure it rather than doctors.



Even my aunt, she's superstitious. She always wonders if something bad has happened to my grandpa. She insisted on taking my grandpa to fortune tellers, thinking that maybe something has possessed him. Their [aunt and uncle] mindset is extremely ignorant, and they can't understand that dementia is a disease. (Grandchild 5, 20)

- 4) Uncurable and expensive disease (making treatment invisible): Although some family members acknowledge dementia as a disease, they regard it as uncurable and thus, not worth further medical attention, putting the invisibility cloak back on PLwD after seeing them momentarily. Some also think medical services are too expensive and simply accept the dementia diagnosis without seeking further support.

And then they [parents] said, "Isn't it just because she's getting old?" And they also say, "Even if it is this illness, there's no way to cure it. Can you say what we can do?" (Grandchild 1, 25)

They [uncles] think that since she's already old, she should just be treated as having dementia. They question the necessity of seeking medical treatment because, in their opinion, undergoing an expensive MRI scan is unnecessary. They always have doubts and suspicions. (Grandchild 5, 20)

- 5) Avoiding medication (making treatment invisible): There is also a folk belief that old people should avoid taking medication because every medicine has its side effects. Family members also equate dementia checkups and treatment as taking medication only, without awareness of nonpharmaceutical interventions. Therefore, although family members can see the PLwD's cognitive problems and somewhat the need for formal treatment, they still decide to refrain from medication, considering its harmful effects on old people, and see no other ways to treat dementia; thus, putting the invisibility cloak back on PLwD.



Don't worry too much, my dear. As long as your grandma's physical health is good, that's enough. She's in her 80s, so she should avoid taking unnecessary medication. Her brain is just starting to shrink, but there's no major problem. (Grandchild 1, 25, sharing a conversation record with her father)

However, this code does not infer that family members seeing suspected dementia symptoms is a prerequisite for PLwD to get a medical checkup. In this study sample, though very limited, some early stage PLwD self-initiated a medical checkup after sensing their symptoms, without the involvement of other family members until getting a formal diagnosis. They either asked social workers for help or learned about memory problems online, indicating their proximity to formal helpers and relatively higher education level. Nonetheless, most older people suspected of dementia usually lack such immediate formal help or heightened awareness of dementia. Thus, it is usually intimate family members who first become aware of the possibility and initiate medical care.

Although the invisibility cloak of cognitive health, pathology, and treatment can be put on PLwD by any family member, it is often most devastating when put on by the family decision maker who exert influence over this subject matter. Even if the initial symptoms are detected and brought to the decision maker in hope of spurring proactive action by taking the PLwD for medical care, these illness claims might be rejected due to the decision maker's beliefs and concerns. Consequently, the invisibility cloak renders both dementia and personhood unseeable even before the diagnosis, resulting in delayed or no checking and prognosis. In this study sample, almost all concerns related to this code were raised by grandchildren, who were the first to notice dementia symptoms but failed to bring them to the attention of the parental generation decision makers. According to a recording provided by Grandchild 1 of a consultation with a geriatrics doctor, the doctor also indicated more grandchildren were taking their grandparents to checkups for suspected dementia, because their parents are often very busy. Thus, in view of their greater involvement starting from an initial stage, these



members of the younger generation should be empowered to learn effective persuasion strategies to enable early detection and intervention.

To be honest, nowadays, I see more people of your age bringing their grandparents to seek medical care, mainly your generation. I understand that the family members might be busy, it seems like they are busy. (Grandchild 1, 25, sharing a recording of a geriatrics doctor)

4.1.1.2. PLwD Making Dementia Invisible

Some PLwD see their memory problems deep inside, but they view mentioning them even to close family members as a threat. Therefore, they put the invisibility cloak on the (suspected) dementia label by rejecting medical care, denying the diagnosis, and reducing social interactions.

- 1) Rejecting medical checkups: In China, a dementia diagnosis can be obtained in either the neurology department or memory clinic of a general hospital or a mental health center. Compared to checking at a general hospital, there is more severe stigma to get diagnosed at a mental health center. Some early stage PLwD retain the negative impression of mental health problems and reject it because they fear people will regard them as having psychosis. They put on the invisibility cloak to hide their suspected mental health problems. As a result, family caregivers must adjust their communication strategies and lie to the PLwD to persuade them to get the checkup. Possible excuses include checking for diseases other than dementia or getting the diagnosis in return for a subsidy.

We have talked about it for a long time, and actually, I wanted to take her to the doctor much earlier. However, when she heard it was a mental health center, she didn't want to go, fearing that people would think she was crazy. She still had consciousness, so it was difficult for me to bring it up at that time. It was challenging to lie and convince her to go. (Child 4, 46)



- 2) Denying the diagnosis: After completing a medical checkup and getting a formal diagnosis of dementia, some PLwD will put on the invisibility cloak to hide their diagnosis from both insiders and outsiders due to severe stigma. Some are averse to the term “dementia” when others bring it up and will leave the room to avoid the illness claim. Some will aggressively argue by claiming others have the disease rather than them. Some will deny the label by claiming their independence and autonomy in managing daily life activities. Some patients also have severe suspicion; although this is a major symptom of dementia, many caregivers find that PLwD constantly worry that their family caregiver talks about them behind their back, especially to others about their dementia.

Initially, he didn't admit it [the diagnosis]. Whenever we mentioned the term “dementia,” he strongly disliked it and didn't want to stay in that environment. (Partner 1, 78)

He doesn't admit it [the diagnosis]. He even thinks that others have something wrong with their brains, not him [laughs]. He doesn't acknowledge his own condition or even realize there's something wrong. (Partner 5, 76)

On that day, my grandfather got angry. He said, “Why do you insist on saying I have dementia? I am a normal person.” Then he mentioned how he can still manage his own life and take care of himself. (Grandchild 5, 20)

Whenever I'm talking to others, she becomes extremely suspicious and worries that I'm speaking ill of her. It's not about saying anything bad, but even mentioning her dementia. (Child 5, 56)

As a result, family members do not dare to broach the topic of dementia both in and outside the family because it is considered negative stimulation, nor will PLwD broach

the topic. Family members and PLwD thus develop a silent pact, even though they are both aware of the existence of dementia.

- 3) Reducing social interaction: To make the dementia label invisible to outsiders, some PLwD intentionally reduce their social interactions to lower the chance of making mistakes. The pursuit for collective interdependence brings them worries of being labeled by society as “foolish” or “confused” and losing face, as shown by Dooley et al. (2015). Some PLwD feel it is shameful to socialize with others, whereas staying at home makes them feel safe because they have this silent pact with family members not to mention dementia, thus leaving no room for the dementia label to grow among outsiders and bring harmful social consequences.

It's most evident that she definitely refuses to go outside. Her social interactions have significantly decreased. ... It's like autism. (Partner 2, 69)

She thought having this illness [dementia] makes it inappropriate for her to socialize with others. (Sister-in-law, 65, discussing PLwD 1)

Through rejecting medical checkups, denying the diagnosis, and reducing social interactions, family members and PLwD make a joint effort to ensure the dementia label stay invisible to reduce concerns about losing face and dealing with stigma. Although PLwD seemingly protect themselves against social norms, they also relinquish their right to scrutinize and reveal their needs and preferences during the disease journey. Family members also have to prematurely exclude them from family communication, losing the perspective of PLwD and rendering the personhood of PLwD gradually unseeable. Thus, PLwD should be empowered to learn about their rights in navigating the disease and not let dementia overpower their personhood.

In summary, on one hand, family members, especially the family decision maker, often do not see the importance of cognitive health and pathology or the feasibility of



treatment, thus making the disease and the afflicted person invisible. On the other hand, PLwD might only see the negative dementia label newly attached to them and not the personhood underneath the label that consistently shapes who they are and what they need. These two codes altogether explain why illness claims might be rejected and personhood might remain unseen among early stage PLwD. Grandchildren and PLwD should be empowered in social work practice to reverse this trend, as discussed in Chapter 5 on the implications of this research.

4.1.2. Dismissing Self-Consciousness

Contrary to rejecting illness claims, families with this property of dismissing self-consciousness see the dementia diagnosis, but make distinct decisions regarding whether and how to disclose the formal diagnosis to the PLwD, according to how they perceive the PLwD's current level of consciousness. After obtaining the dementia diagnosis, they either chose to cover up, intentionally frame, or blurt out the diagnosis. Only families who formally tell the PLwD about the diagnosis and medication actualize the conditions for seeing their personhood, by fully recognizing the diagnosis and the PLwD's consciousness and paving the way for further communication involving the PLwD.

4.1.2.1. Covering Up

Like how most Chinese doctors and families deal with a cancer diagnosis and end-of-life issues, family members mostly cover up a dementia diagnosis and the patient's feelings. They may feel it is unnecessary to tell the patient because it not something they should reveal. Telling them is also seen as useless due to its incurable nature, which is consistent with nihilist and fatalist beliefs regarding a dementia diagnosis. Some family members also think it is too complicated for PLwD to understand the diagnosis, dismissing their comprehension ability or perceiving it as compromised due to dementia.

It's not something that needs to be openly discussed. ... People with dementia have

a strong sense of self-esteem. (Partner 2, 69)

This [dementia diagnosis] is very complicated for her. She may not fully understand it. (Children 3, 52)

As mentioned in the previous code, PLwD can make the dementia label invisible due to concerns about losing face and stigma, this fear can also be perceived by family members on the patient's behalf, no matter whether it reflects the reality. For example, some family members deliberately hide the dementia diagnosis from the PLwD, because they perceive that the PLwD may face stigma or feel inferior at hearing the diagnosis. Although they seemingly protect the PLwD's self-esteem by making the diagnosis invisible, they fail to recognize their self-consciousness in deciding what is in the best interest of the PLwD, prematurely excluding them from family communication. This is strikingly different from the Western tradition that prioritizes the patient's right to learn about their disease.

We haven't told her about her dementia because I think she has a sense of shame. (Child 3, 52)

If you tell him, he may have a sense of inferiority. It's better not to tell him. It's quite common for older people to get dementia, right? (Partner 4, 80)

4.1.2.2. Intentional Framing

Family members usually break the diagnosis intentionally, such as framing it as memory loss or poor concentration to protect the PLwD from worry or unhappiness. Despite literature that has revealed that disclosing the diagnosis to patients does not instigate an intense emotional reaction and is in fact beneficial to both patients and caregivers because it relieves the psychological burden if delivered in a sensitive manner, family caregivers often expect the emotional feedback of the PLwD at hearing this news to be negative and deem it necessary to protect the PLwD from negative



emotions. Although done in the name of love, such behavior dismisses the patient's right to know their condition, denying them the ability to accept the medical diagnosis regardless of stigma and communicate and digest negative emotions. After putting a partial invisibility cloak on the diagnosis, family caregivers might perceive that the PLwD is aware of their dementia, has no awareness, or hovers between the two. The ambiguity brought by an intentionally framed discussion of the diagnosis often results in family caregivers' hesitancy to initiate communication about dementia, because they have set the tone that memory loss and poor concentration are normal parts of aging and should not be worried about. They also neglect the temporality of happiness or worry they brought on for the patients. After being excluded from knowing their diagnosis directly from the doctor, the PLwD receives information that is filtered using these euphemisms, rendering the diagnosis unclear and uncertain. However, in interdependent relationships, PLwD are able to take the perspective of their caregivers, and they know any reaction that reflects their unhappiness and worry may violate the family caregivers' good intention to protect them. Thus, PLwD are also restrained from talking about their questions and concerns, if any, with family caregivers, reaching another silent pact that makes dementia-related problems invisible.

In our previous conversations, we just told him [the PLwD] that, "You are getting older now. No need to get worried." (Grandchild 5, 20)

I just tell him, "You're a bit sick." ... I'm just afraid that he might be unhappy. (Sister-in-law, 65)

4.1.2.3. Blurting Out

The process of disclosing a dementia diagnosis is delicate. Although covering up or intentionally framing dementia is not desirable, to some extent these approaches reflect family members' genuine effort to think from the perspective of PLwD, though they are not aware it is problematic and wrong. However, some family members dismiss the PLwD's consciousness and never consider how the PLwD will react to the diagnosis.



They carelessly blurt out the diagnosis directly to the PLwD in front of other family members and ridicule their poor memory, using the term “old confusion” with humiliating connotations. The invisibility cloak is readily available, but they suddenly strip it off and expose the situation to everyone, dismissing other family members’ deliberate effort to prepare a careful explanation at the right time and occasion. In the high-context culture, where and how something like this is said can be more important than what is said. The communication context in this instance becomes extremely undesirable, violating the Chinese norms related to breaking bad news. Therefore, it can be even more harmful to the PLwD, because in this inappropriate context, blurting out requires the PLwD not only to receive the news but also react to the malice immediately, leaving almost no room for remedy.

I can't believe she [aunt] actually talked about that in front of Grandma, saying, "Don't you know your grandma has dementia?" And she laughed. It made me extremely mad. (Grandchild 1, 25)

And then my aunt just went ahead and told her [grandmother] directly, "You have dementia." (Grandchild 5, 20)

Are early stage PLwD devoid of consciousness, including awareness of and feelings about dementia? Not necessarily. It may be true that it becomes impossible to share the diagnosis with PLwD in the middle to late stage of dementia because they are unable to understand, but early stage PLwD do have the ability to recognize their memory problems, react to how people treat them, express their emotions, describe strategies to manage their life and illnesses, and retain the ability to feel even though they might be unable to express themselves clearly.

I prefer to die soon. It would be best to die soon because today my memory is completely gone. (PLwD 1, 79)



If no one talks to me anymore, I won't say a word. What should I do? They [children] won't talk to me either. (PLwD 9, 80)

If you treat him badly or scold him loudly, he knows. (Partner 5, 76)

Although some PLwD acknowledged this during their interview, family members may never try to explore their consciousness or never think about this question, naturally dismiss their self-consciousness, and take control of the diagnostic disclosure or following care arrangement. Some even reject the existence of the PLwD, regarding their opinions as unnecessary or irrelevant.

He [PLwD] won't be aware of what he's thinking about. (Grandchild 2, 27)

Now it seems like he [PLwD] is completely unconscious. ... It feels like he doesn't even consider his own situation anymore. (Grandchild 5, 20)

Ideally, families accept the illness and recognize the PLwD's consciousness—disclosing the diagnosis to the PLwD formally, either by doctors or family members, and feel at ease in talking about dementia-related issues. They see the personhood of the PLwD and acknowledge their independence and autonomy at the very beginning of the disease journey. These acts fulfil the prerequisite to engage the PLwD in the prognosis discussion, or the second phase of visible communication: speakable.

During the treatment process, my mom took him [PLwD] to the hospital, and he learned about it [the diagnosis] directly. He also knows the treatment effects of the medications he is taking. (Grandchild 2, 27)

In summary, families who find communication problematic do not see the personhood of the PLwD by rejecting illness claims and dismissing their self-consciousness. They fail in the first step of actualizing visible communication and consequently, they are



unable to speak openly about dementia, thus further excluding PLwD from discussing their prognosis, care arrangements, and preferences. However, even with the ability to see the PLwD's personhood, this does not necessarily ensure they can speak easily about this topic and be heard and moved by the information receiver. In the next section, I delineate the conditions under which issues are seeable but not speakable.

4.2. Unspeakable 見而不說: Censoring Speakability

Glaser viewed grounded theory as the study of how people account for and resolve a problem in their lives. On the basis of seeing the personhood of PLwD, families are able to see problems that need to be resolved with other family members, such as initiating medical diagnosis, optimizing caregiving approaches, balancing caregiving tasks among multiple caregivers, moving to nursing home, and so on. However, rather than directly communicating their thoughts as usually practiced in an independent view of self, the interdependent self conducts self-censorship to determine the weight carried by their words and what can be said to whom by examining the internalized hierarchy and designated role in the family. Therefore, in consistent with Communication Accommodation Theory, they may adjust their communication strategies on a continuum from speakable to unspeakable, gradually lowering the level of certainty and putting the invisibility cloak on themselves (communicating directly, communicating informally, communicating indirectly, communicating matter-of-factly rather than emotionally, and keeping to oneself), as shown in **Figure 5**. In the following section, I examine how family members censor the speakability of dementia-related issues and how communication can become unsatisfactory and unhearable.



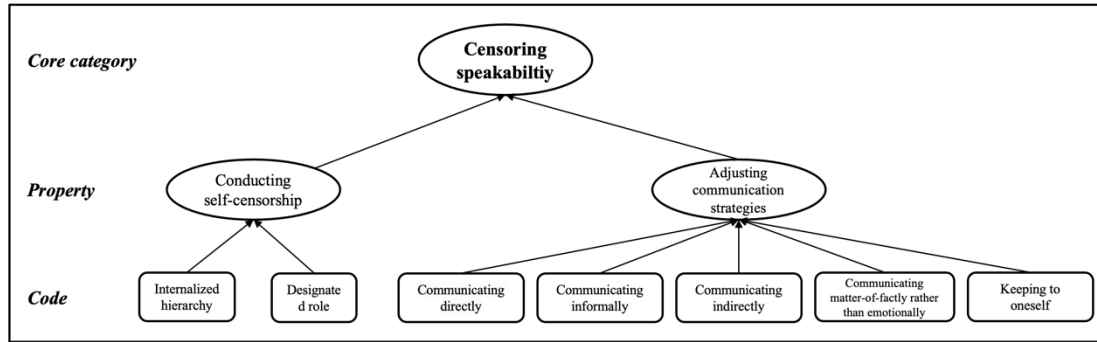


Figure 5. Illustration of the codes and properties under the core category of “censoring speakability”

4.2.1. Conducting Self-Censorship

With something in mind to communicate or persuade about, people will usually first scrutinize to what extent it can be useful and make a real difference. If the communication is deemed to be useless, they will refrain from discussing the issue or let it hang in limbo. If it has the potential be useful, they may further consider the social context, including whether they are in an appropriate position to initiate a discussion of the subject matter, the consequence of saying something that goes against their prescribed roles, and strategies can be used to articulate message without destroying harmony. This illustrates the psychological process of how people conduct self-censorship after identifying a problem and before initiating communication, as reflected in two codes: internalized hierarchy and designated role.

4.2.1.1. Internalized Hierarchy

Hierarchy, or the relative positioning of family members, has been deeply internalized in Chinese families and determines how much weight their communication carries. What is said by family members in less powerful positions, such as grandchildren (relative to parents), younger brothers and sisters (relative to older brothers and sisters), adult children (relative to parents), girls and women (relative to boys and men), may be rendered as less reliable and important, thus discouraging them from communicating. Although those in relatively more powerful positions carry more weight in what they say, they may be unwilling to communicate to those in less powerful positions to uphold

the hierarchy, preventing the free flow of information and perspectives.

She [older sister] always regards me as a little sister, an immature little sister. Whatever I said lacked reliability and trustworthiness. (Child 4, 46)

In her [PLwD's] eyes, you're just an immature little girl, so she won't discuss such [dementia-related] matters or talk about her thoughts with you. (Child 4, 46)

Gradually, hierarchical communication patterns lacking equality and respect form and are internalized, lowering the willingness of those in higher and lower positions to communicate, which further reinforces the stability of the hierarchical communication or noncommunication patterns. Thus, as shown in the first quote below, family members situated in such communication environment may censor themselves and feel reluctant to initiate communication. They put the invisibility cloak on themselves because they regard communication as either useless or unnecessary. Nonetheless, as illustrated in the second quote below, the relative positioning in some families is more equal and collaborative. They conduct less self-censorship before communication and regard each other as capable communicators, even with PLwD. In this situation, nobody puts on the invisibility cloak during communication.

When you grow up with pushy parents, once they established their authority, you no longer have the desire to communicate with them. I feel uncomfortable communicating with them, that kind of feeling. (Grandchild 3, 27)

For any major family events, my grandfather will take my grandmother [PLwD] with him. Whether it's making decisions about family issues, having family meetings, celebrating Chinese New Year, eating together, or attending a grandchild's wedding, my grandfather always brings my grandmother. (Grandchild 4, 32)



4.2.1.2. Designated Role

Whether the communication pattern is hierarchical, family members will examine their role in relation to other family members and locate themselves appropriately in the caregiving context before initiating communication. In line with the literature (Brodaty & Donkin, 2009), in the current study, spouses were mostly care providers, adult children and sometimes spouses tended to be care managers, and grandchildren provided emotional and additional support. The role delegated to each generation reflects the family's expectations and social norms of what each member should do, what they can say, and how should they accommodate others in communication. Although the division of the caregiving role is often not problematic, when it is embedded in the hierarchical relationship, any act that crosses the line can go against the normative requirements and yield significant communication consequences.

- 1) Naturally assuming caregiving responsibility: In some families, the caregiving role naturally falls on the PLwD's partner, without communication about this issue with their children. As discussed in the section on intergenerational value differences, the spousal caregiver (G1) is aware of their children's (G2) low availability due to work and family commitments (Williams et al., 2018) and their designated role to care for their partner as required by marital responsibility. Thus, spousal caregivers naturally shoulder the caregiving tasks and accommodate their children's needs and communication. With their role as spouse, caregiver, and parent, they typically feel satisfied if their children can occasionally give a hand or drop by to visit them. They often censor what they communicate to their children because they do not want to add to their burden. Consequently, they may feel lonely because there are fewer people alive from their generation (G1) with whom they can talk with, and they feel reluctant to communicate with their children (G2) because they decided to shoulder the caregiving responsibility by themselves. They seldom ask for their support, either instrumental or emotional.

I: Did the family discuss who should take care of her [PLwD]?

P: No, never. Anyway, I take care of her. ... It's natural to be me. (Partner 3, 86)

We also don't want to disturb him [son]. ... He doesn't come unless there are major problems. (Partner 7, 78)

- 2) PLwD's partner or parent generation as decision maker: In most families, it is either the spouse or an adult child who becomes the key decision maker. Some of these decision makers are overly dominant and dismiss the opinions of others, whereas others are more considerate of the perspectives of other family members and organize care in a more collaborative way. Nonetheless, both types of decision makers conduct less self-censorship in their communication, tending to communicate directly without any invisibility cloak and let others accommodate their thoughts and styles.

Whatever he says is important. Whatever I say, he feels is not important, not what he cares about. It's a just dismissive attitude. (Grandchild 6, 27)

Because I still have some influence at home, they [parents] feel that I am rather reliable in handling these things. They trust me, so they accept what I say. (Child 3, 52)

- 3) Grandchild as emotional caregiver: Although grandchildren are relatively powerless in decision making, they are in a desired role to provide emotional support to the PLwD and the PLwD's partner and accommodate their communication needs. Although the literature has pointed out the positive role of grandchildren in attending to the emotional and social needs of PLwD (Blanton, 2013), this role is in line with cultural expectations in the Chinese context because they do not provide the majority of care, are not the family decision maker who can exert influence on the disease journey, and are not powerful enough to persuade other family members to take certain actions. Thus, the only pathway for them to



actualize their care or filial piety is through providing emotional support. Despite how other family members treat dementia and the PLwD, they usually call their grandparents regularly and try their best to accompany them during holidays. It is not only something they want to do and can do very well, but also the only thing they can do due to their relative powerlessness in the hierarchical family structure. They also feel a psychological burden when trying to broach serious topics because it makes it seem like they want to be the key care provider or decision maker, violating their designated role. It can also be inferred that like the supplementary role of grandchildren in dementia care provision, emotional care is also regarded as secondary to instrumental support and somehow inconsequential to other family members.

I: If you can't get a definite diagnosis, what do you think you can do at this stage?

P: I just call her [PLwD] more often when I'm free. (Grandchild 1, 25)

When I talk about filial piety, it means spending time with them [grandparents]. For example, when I have a vacation, even though I only go back for a short time every year, many friends want to go out with me. I cancel on them all because I want to be with my family. However, when it comes to the illness, I don't really discuss it much because, as I said before, I feel a bit of difficulty expressing those things. (Grandchild 3, 27)

As for my eldest cousin, he doesn't pay much attention to his own family matters. But when he heard about Grandpa's dementia, he got so worried. Although he's too busy to come back, every day, he calls our grandparents when he's free to chat with them. (Grandchild 5, 20)

Despite neofamilism leading to a more predominate role of grandchildren in the family, this does not go beyond the realm of their own issues. When they try to exert influence in the broader family context, what they do should not challenge the role of other family

members nor interfere with the normal progression of dementia management. Thus, the role of emotional caregiver is considered ideal for them. Therefore, although neofamilism has not elevated their relative positioning in the family, it has weakened the negative consequences if they do not exercise self-censorship in communication and initiate communication topics that are traditionally prohibited. With increasing levels of self-expression and lower self-suppression, the younger generation also expects family members to express themselves more openly and actively, and they are less likely to tolerate ambiguity in other people's communication. Therefore, in this study sample, members of the grandchild generation were more willing to break through cultural norms and jump out of their designated role to bring what they deem to be important to the attention of family decision makers, rather than adopting silence when dealing with authority. This also implies a heightened awareness of the independent self in the younger generation, despite the hierarchical relationship remaining unchallenged, which will be further discussed in next chapter.

In summary, some may put an invisibility cloak on themselves by suppressing what they want to say in view of power differences to maintain ingroup harmony. Among those who still opt for communication, such as grandchildren, the next property, adjusting communication strategies, illustrates how they censor information in conversation and adjust their communication strategies while acknowledging role differences and avoiding direct confrontation.

4.2.2. Adjusting Communication Strategies

After conducting self-censorship, family members will often adjust their communication strategies in response to their interpretation of the internalized hierarchy and their designated role. This is consistent with the symbolic interactionism view that people will fit their actions to the actions of others. This highly accommodative nature also reflects the foundation of Chinese communication culture as maintaining ingroup harmony instead of engaging in assertive self-expression as in Western culture. The five codes linked to this property—communicating directly,



communicating informally, communicating indirectly, communicating matter-of-factly rather than emotionally, and keeping to oneself—demonstrate how family members censor the speakability of dementia-related issues on the continuum from speakable to unspeakable.

4.2.2.1. Communicating Directly

Direct communication appears in families with dehierarchized communication patterns, with family members holding more equal positions. It is also commonly seen among family members occupying the role of decision maker, who often communicate directly and instruct what others should do without any invisibility cloak. For example, older brother can directly instruct their younger sisters and brothers how to divide care, older sisters can directly suggest that younger brothers visit more often, and spousal caregivers can directly tell others how to arrange care in special circumstances. In these situations, the communication initiator conducts little censorship, because the dehierarchized communication patterns and their designated role make them feel more certain and confident that what they are saying is influential and acceptable. They are more likely to communicate regardless of the context because what they say already carries significant weight, so it is more important than where and how it is said. Receivers, thus, can easily understand the message and proceed to implementation.

No, everyone discusses and consults each other because we are all rational about these issues. We all know that it's for the interest of Grandma [PLwD], and our thoughts align deep inside. Therefore, we don't have any arguments. In some families, there might be disagreements, but in our case, we resolve everything through consultation and negotiation. (Grandchild 2, 27)

My older brother told each of us to take care of Mom for a year. Because they said that, we will follow it. (Partner 3, 86)

Later, I called his [PLwD, brother-in-law] younger brother and said, "Your brother



is developing dementia, so you must provide my sister-in-law with some emotional support. Even if you visit him once, it will make my sister-in-law happy.” His younger brother then came to see her. (Sister-in-law, 65)

However, some people may wrongly perceive their relative position as equal with other family members or that their designated role in the family allows them to cross boundaries. They confidently communicate with other family members and not unexpectedly, what they said is left unheard or the communication receivers are not moved. As a result, they often reevaluate their position and role in the family and adjust their communication strategies accordingly, with a lower level of certainty.

4.2.2.2. Communicating Informally

Although communicating directly reflects the initiator’s high level of certainty and confidence, families with lower level of certainty may have reservations about directness; thus, they prefer to communicate informally in casual settings and language. It is not the Chinese style to hold family meeting or directly share information, formulate care plans, and arrange task divisions. On one hand, such formal communication often implies something serious has happened, with the possibility of triggering direct confrontation during an intense verbal exchange, which would violate the Chinese communication goal of pursuing harmony. Therefore, Chinese families feel more comfortable communicating important issues when the whole family is eating or watching TV. The context lowers the seriousness of the communication topics, enabling family members to feel more relaxed and able to express themselves. It also allows them to redirect attention if the communication becomes conflictual or goes nowhere, winding up the conversation smoothly and allowing everyone to save face.

While we are at the restaurant, we discuss and confirm this issue while having a meal. (Grandchild 4, 32)

On the other hand, although family meetings enable effectiveness in solving problems,



they limit the ability to read the minds of others among individuals holding an interdependent view of self. Chinese family members often believe in mutual understanding and accommodating through daily interaction and observation rather than direct requests. The cultural belief of *yan bu jin yi* (言不盡意) not only indicates the inadequacy of spoken words, but also implies that the meanings obtained and attached through observation tell more of a person's needs than what they say directly. As such, families can successfully solve problems through informal discussion during daily care interactions. For example, when deciding how to allocate caregiving tasks among a PLwD's spouse, uncle, mother, and grandchildren, these family members might not have a formal discussion. Instead, they observe one another's work and acknowledge what they are good at. Through trial and error and consistently adjusting to one another's needs, they find the most suitable care routine that fits everyone and achieves work-care balance.

There's nothing specific, just daily communication among my grandpa, mom, and uncle. There's nothing special to trigger the communication. (Grandchild 4, 32)

For Chinese people, communicating informally is not seen as inferior to communicating directly. Although it might lower the perceived importance of discussion topics and compromise effectiveness, it can sustain what “home” means to Chinese people—that is, harmonious and nonconfrontational. Thus, it can be even more desirable than direct communication. Nonetheless, such informal communication requires all family members involved to have higher-level consensus on the definition of dementia, how to manage dementia, and mutual agreement to share caregiving tasks and do whatever is best for the PLwD. If family members hold divergent views, they may first take informal communication for granted, mistakenly. When they fail to solve problems through informal communication, they might feel disappointed, adjust their expectations, and adopt alternative strategies, which is further explained in the section on “keeping to oneself.”



4.2.2.3. Communicating Indirectly

Compared to communicating informally, family members with lower levels of certainty will often adopt the indirect approach. They are in a relatively powerless position to initiate communication, because their direct expression can be unheard and unmovable or the hierarchical family communication pattern has socialized them to communicate in a roundabout way. For example, when a brother and younger sister discussed whether they should send their mother with dementia to a nursing home, the younger sister did not express her supportive attitude directly, because it would obviously violate the cultural expectation of filial piety. Instead, she said if her mother-in-law had dementia, her husband would definitely send her to a nursing home. In this hypothetical scenario, no matter what her husband might actually do, she imposed her viewpoint on him and showed her support for this so-called choice by her husband. Though her communication was indirect, her brother could easily grasp her meaning beyond the words, or as the Chinese term it: *yan wai zhi yi* (言外之意). Like she imaginarily supported her husband, her brother should do the same by supporting her choice without any sanctions or blame.

She [younger sister] didn't explicitly say it, but the meaning was quite clear. My sister said if her mother-in-law had dementia, her husband would definitely send her to a nursing home. It clearly implies that sending [the PLwD, their mother] to a nursing home is the solution, right? (Child 5, 56)

Some family members are also socialized to communicate indirectly due to a long-standing hierarchy without respect and equality in the family. For example, when one grandson tried to point out that the home environment was too quiet to stimulate PLwD's brain and encouraged his parents to make some improvements, he did not mention the consequences and what he expected them to do explicitly. Instead, he just pointed out, "It is too quiet, everyone will fall sleep," without providing further explanation. He attentively observed his parents' nonreaction and inferred that they dismissed what he said, thus further internalizing the hierarchy of communication in



the family. He was aware of the necessity to communicate, but he could not do so fully because his certainty and confidence were restricted by his role in the family. Indirectness was the best he could do to respect the role difference and deliver the message he wanted. However, he was not aware that before sending out the message, he put on an invisibility cloak, thereby reducing the richness of the information contained under the cloak. Thus, the message might not even have been sent out or only at a very low volume, causing it to go unreceived by the communication receiver. This issue is further elaborated in the next core category, unhearable.

According to my understanding, our home is too quiet, which is extremely uncondusive to providing regular stimulation for the brain. I have mentioned it [to my parents] at home, but I haven't pointed out that it is not conducive to brain stimulation. I remember saying this to my parents, that everyone will fall asleep in such a quiet environment, but they didn't pay much attention to it, I guess. (Grandchild 3, 27)

I: Do you think you will try to communicate with your parents about these things in the future? Will you try again?

P: I will definitely try. For example, when you finish your thesis, I will definitely share it with them then, right? Maybe because this is the way I usually do things, this is my role in the family, I don't know how to change it either. (Grandchild 3, 27)

Alternatively, senders can use more indirect approaches without speaking to deliver their message. For example, they may send articles related to dementia to other family members, hoping they pay more attention to these issues. Or they may take photos of their grandparents to show their love, because direct communication can be difficult for them. Although they have the desire to communicate, they are highly aware that direct communication will not yield the desirable result; thus, they naturally lower their expectations and put on the invisibility cloak before communication to save face and



protect their self-esteem.

4.2.2.4. Communicating Matter-of-Factly Rather Than Emotionally

Whereas some family members feel uncertain in general and communicate indirectly, some might feel particularly uncertain about communicating feelings and emotions. Thus, they communicate selectively by only talking about issues matter-of-factly, such as the disease, the PLwD's behavior, and treatment. As I argued relative to the code of "designated role," family members avoid talking about feelings because they can be viewed as secondary to factual issues and thus, not deserving of discussion. Another explanation is that revealing feelings, to some extent, is equal to exposing vulnerabilities and revealing one's inner world, which is not routinely practiced in most Chinese families. In a high-context culture, personal feelings are strongly controlled (Hall, 1976). It is something subject to the reading of interdependent others rather than self-revelation. Communicating feelings is especially uncommon in families that are not emotionally close and whose members view it as threatening to their authority and seniority. For example, at hearing his grandfather's diagnostic result from his parents, one grandson said he instinctively did not show any emotion in responses. It is not their family tradition to talk about anything deep or personal, even apart from dementia, and the consequences of such communication are unpredictable.

I: Wouldn't you talk to them [parents] about your feelings and thoughts?

P: Based on my understanding, I don't think we will have much to say. It's just about exchanging ideas specific to my grandfather's behavior and symptoms. (Grandchild 3, 27)

Our family is very introverted. We don't open our hearts to each other and aren't talkative. (Grandchild 6, 27)

In another case, an early stage PLwD described a very detailed psychological process related to his understanding of dementia, how he sought information, his struggles when



he found the medicine was not effective, why he deemed his condition to be incurable and not worthy of medical diagnosis and treatment, and his views about life and death. However, he did not tell his family about these issues in the way he told me. He only talked about factual matters with family members, including factual aspects of dementia and his decision to stop seeking medical support, rather than the mental process and feelings beneath these emotionless facts.

I: Will you talk to your wife about it [thoughts about dementia]?

P: I will talk about it if necessary, but I will say more fundamental issues. (PLwD 3, 77, early stage)

Although we cannot assume what is in someone's mind and need to be extremely careful when applying the language of intention and motivation, it is nonetheless safe to suggest that some family members deliberately put on the invisibility cloak to avoid feelings in their communication, even though they have abundant feelings, especially negative ones, associated with the diagnosis and prognosis. They feel uncertain about the consequence of revealing their feelings and intentionally avoid it as something less important, difficult to express, or subject to the interpretations of others in Chinese culture.

4.2.2.5. Keeping to Oneself

On one end of the continuum are family members who decide to keep what they want to say to themselves and not initiate communication. They might be unwilling to communicate because they take mutual understanding for granted, they have experienced failed communication and deem their thoughts as unheard or unmovable, or they are reserving communication for future crises.

- 1) Taking mutual understanding as granted: As argued relative to the code of “communicating informally,” caregivers may misconceive that they will be understood by their family members and feel it is not necessary to communicate



verbally. For example, when an older brother wanted his younger sister to shift her caregiving day from Friday to Saturday, she questioned why he needed to do so. The brother viewed the reason as extremely apparent: He felt very tired after caring for their mother with dementia on both Saturday and Sunday, for the full day without any daycare or respite service. He was certain that his difficulties were clearly evident and would be easily understood by sister without any explanation. Therefore, he was irritated when his sister asked him such a careless question, creating conflict. Eventually, the brother decided to let it go and adjusted his behavior by lowering his expectations. He apologized first for losing his temper to maintain their relational harmony. Although he felt he could expect very little from his sister, she was still the most immediate source of help he had in times of need.

There's no need to explain this [switching caregiving day]. Explaining it would be unnecessary and would make me unhappy, right? (Child 5, 56)

- 2) Communication being unheard or unmovable: Alternatively, family members may feel certain that communication is useless, mostly after experiencing communication failure; thus, they decide to keep their thoughts to themselves. For example, when a spousal caregiver wanted to describe his caregiving burden and difficulties to his daughter, instead of receiving the consolation he expected, his daughter blamed him for not sending her mother with dementia to a nursing home, implying his suffering was deserved as a consequence of his choice. She refused to provide emotional support for something she deemed as unworthy. Gradually, the spousal caregiver said he felt it was better not to communicate because he would certainly receive something negative and even hurtful in response.

You cannot talk to anyone about these things [caregiving burden]. There's nowhere to talk about them. (Partner 3, 86)

What's the point of talking about it? My daughter, if you talk to her [about the



caregiving burden], says, “I have told you to send her [the PLwD] to a nursing home, but you refused.” What’s the point of talking about it? I knew it. (Partner 3, 86)

Sometimes it’s better not to say anything. (Partner 5, 76)

- 3) Reserving communication for other crises: When communication is deemed as unhearable or unmovable, some decide to give up, whereas others decide to reserve communication for when another crisis arises in future, when they believe what they say will be better heard and accepted. For example, when a granddaughter failed to persuade her parents to take the PLwD for further medical checks, she decided to broach the topic again when the situation worsened and the weight of her suggestion would be more significant. In another case, the PLwD had some thoughts on his care preferences and how to manage finances after his dementia worsens, but he never broached these topics because his children treated him very well. Although he acknowledged the necessity of communicating, he could not hold a family meeting and seriously talk about these ideas, which may have been misunderstood as dissatisfaction with the current care he received from his children and inadvertently hurt them.

Yes, yes, if the situation worsens, I will talk about it [with my parents]. (Grandchild 1, 25)

My son treats me well, so I can’t say, “Sit down, let’s have a talk.” If they treated me badly, then we would definitely sit down and talk about how I treat you and how you are supposed to treat me. But for now, this problem doesn’t exist. (PLwD 3, 77)

In these three scenarios, the first speaker is certain that he will be understood and thus, put the invisibility cloak on his thoughts, but unexpectedly experience a decreased level of certainty. The second is certain she cannot be heard or nothing can be changed, so

put on the invisibility cloak, indicating the lowest level of certainty. The last speaker is certain that what he says will be heard or lead to action in the future but not now, so he put on the invisibility cloak temporarily, indicating an increasing level of certainty as the disease progresses. In this moment, all three developed a silent pact with other family members to not broach the topic.

In summary, when family members see the personhood of the PLwD and want to communicate with others, occasionally they can directly communicate with a higher level of certainty and confidence. However, most often they feel uncertain and conduct self-censorship by examining their relative position and designated role in the family. Although the family's internalized hierarchy can deter some from communicating, members of the younger generation tend to conduct less censorship and break through their designated role to bring salient issues to the attention of the family, with less harmful consequences as a generation characteristic. After censoring speakability, based on their level of certainty and confidence that they can make a difference in their position and role, family members decide on the strategy to pursue: communicating directly, communicating informally, communicating indirectly, communicating matter-of-factly rather than emotionally, or keeping to oneself. Nonetheless, the selection of strategies is not static. Although some may succeed in their first choice, others will move across strategies with a lower level of certainty when their initial strategy is unheard or unmovable. In the next core category, I delineate conditions that occur when issues are seeable and speakable but remain unheard by the receiver.

4.3. Unhearable 說而不聽

The communication process involves both message senders and receivers. As illustrated in the first two core categories, although senders might make considerable efforts to see the PLwD's personhood and vocalize problems, the message they send out can remain unheard by the receivers. Chinese communication is centered on the receiver. The meaning is subject to the interpretation, listening skills, and sensitivity of the receiver rather than the sender. The implicit nature of communication also renders it as an



infinite process of interpretation (Yum, 1988). Therefore, communication can be unhearable because the receiver does not see the personhood of the PLwD; they put an invisibility cloak on the PLwD and thus, are not sensitive to what is said about the PLwD. In addition, although their personhood is seen by the sender and the subject matter is delivered, it becomes unhearable because the receiver did not receive the message. As shown in **Figure 6**, this could occur because the message is too implicit, a situation in which the sender puts on the invisibility cloak on themselves. Alternatively, the receiver might not try to interpret the meaning contained in the implicit message, thereby putting the invisibility cloak on the sender. Finally, although the message might be properly delivered, the receiver could ignore the message by putting on the invisibility cloak on themselves, possibly because they have a negative family history or they want to shed the caregiving responsibility.

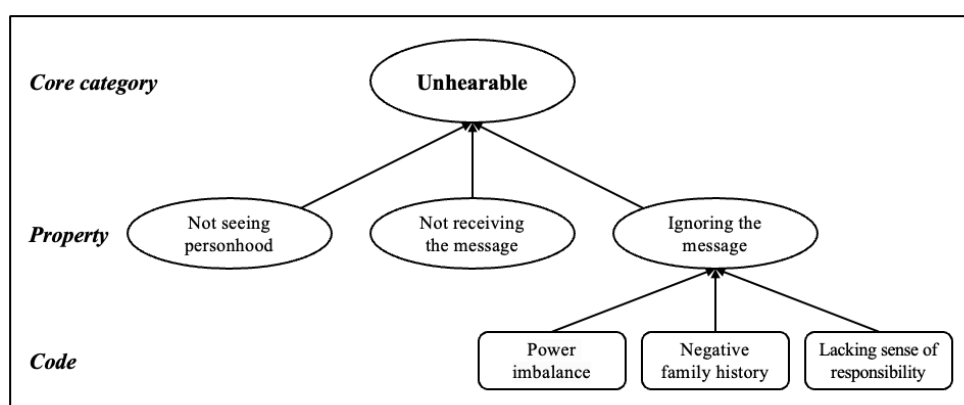


Figure 6. Illustration of the codes and properties under the core category of “unhearable”

4.3.1. Not Seeing Personhood

In Section 8.1, I argued that family members might not see the personhood of the PLwD because they either reject the illness claim or dismiss their consciousness, thus prematurely excluding the PLwD from family communication about dementia and even making dementia-related issues become unspeakable in the family. This argument is more related to the message sender’s side, but it also applies after the message is sent but remains unhearable by the receiver. They do not see the personhood of the PLwD,

thus any message concerning the PLwD is seen as noise. For example, when a spousal caregiver communicated her caregiving burden to her son and daughter-in-law, they only expressed annoyance and would not listen to what she said, which is consistent with what Braithwaite Stuart et al. (2022) found. They regarded the PLwD's existence as unnecessary, putting the invisibility cloak on both the PLwD and the spousal caregiver.

When I talked to them [son and daughter-in-law], they found me annoying. “Who asked you to care for him at home?” And then they would complain that he [PLwD] is annoying. I won't bother arguing with them. Sometimes when I nag, they find me annoying, too. So, I just stopped talking. (Partner 5, 76)

They [son and daughter-in-law] just can't tolerate him. They wonder why a person like him is still alive. It feels like his existence is meaningless. (Partner 5, 76)

4.3.2. Not Receiving the Message

As argued regarding the previous category, family members who are uncertain about whether what they said will be heard or lead to action can communicate indirectly as a strategy to mitigate any negative effects. However, by putting on the invisibility cloak and expressing themselves implicitly, they also risk not sending the message or having it fail to reach the receiver. Thus, the receiver may have nothing to interpret or they may find the message difficult to interpret and give up. Another possibility is that although the message can be implicit, some receivers do not try to interpret its meaning, which is also integral to Chinese communication. Whereas a message that is too implicit indicates the receiver is paying attention to the message and does not intend to put the invisibility cloak on the sender, those who do not try to interpret a message deliberately put the invisibility cloak on the sender and leave the message hanging, which could even be more disheartening for the sender.

P: I just sent them [dementia-related articles] via WeChat. ... Anyway, I do these



things indirectly.

I: If you send these articles to your parents, what would they say?

P: They wouldn't say anything. (Grandchild 3, 27)

I: Do you think they actively seek knowledge on how to take care of people living with dementia?

P: No, I don't think so. If they did, they might send some messages or articles in the [family chat] group to let me know. (Grandchild 3, 27)

However, it can be extremely difficult for the sender to distinguish whether the receiver does not understand the implicit meaning or is not trying to interpret the message. They can only observe their reactions to determine their attitudes toward the message. For example, a grandson felt uncomfortable talking about dementia issues directly with his parents, so he sent them online articles related to dementia as an indirect way of communicating. He hoped these articles would trigger his parents to pay attention to dementia and cope with it proactively, but he never expressed such expectations directly. He said that if his parents received his message, they would reply and make the conversation flow naturally. He also expected his parents to communicate with him in the same way, such as when they read good articles about dementia, they would send them to him and let him know they truly care about dementia. However, they did not respond in their online conversation, nor did they reciprocate with online articles. Because the sender did not see any improvement in how his parents dealt with dementia, he naturally believed that his parents did not understand the message he sent and still did not take dementia seriously. But was it because his message was too implicit, or were his parents are not trying to interpret the message? Maybe both—but again, we have to be extremely careful when applying the language of intention. Due to the sensitive nature of the communication in such families, this study did not involve outreach to the message receivers to triangulate the findings regarding their communication process, which could be explored by future researchers. As a result, both sender and receiver fell into a loop of infinite interpretation, adding to the



ambivalence of intergenerational communication, and the message eventually went unheard by the receiver.

4.3.3. Ignoring the Message

In comparison, when some family members have successfully received a message, they might choose to ignore it due to power differences, the influence of relationship history, or an unwillingness to shoulder the caregiving responsibility. They intentionally put on an invisibility cloak to block out the message and stay away from the people it concerns.

4.3.3.1. Power Imbalance

First, a message can be ignored due to power imbalance. In Section 4.2.1, I discussed the internalized hierarchy of families and designated role of each family member, causing some family members to undertake the role of dominating decision maker. This can occur on the message sender's side and indicate they have more certainty in initiating communication and thus, seldomly conduct self-censorship before sending out the message. Similarly, the rationale can be applied when the decision maker is the message receiver. In view of the power imbalance between them and less powerful family members, such as grandchildren, they can easily ignore their message and regard what they say as noise that is as inferior as their role in the family. However, younger generations are more likely to express themselves and less likely to tolerate ambiguity in communication and noncommunication. Thus, some grandchildren will start an argument with their parents to seek their attention and force them to listen to what they want to say. It is an act that challenges the Chinese pursuit of harmony and even family hierarchy, but as the only child, they are less likely to suffer from negative consequences compared with previous generations regarding such deviant behaviors.

That is, when I have a big argument with them [parents], they are finally willing to listen to me. For example, our arguments have pushed me to the point where I might even start throwing chairs, and that's when they finally start to listen to what I want to say. (Grandchild 3, 27)



4.3.3.2. Negative Family History

In situations with a negative relationship history, some family caregivers may be reluctant to receive messages about the PLwD and ignore any such messages by wearing an invisibility clock. Negative relationship histories mentioned by these interviewees included parental favoritism, in which the PLwD favored one child than others and particularly, gave one child more assets, making the less favored child less willing to provide support in adulthood. Similarly, in families with a preference for sons, daughters may be less likely to undertake the caregiver role or obey what they are told to do by their brothers. Adult children who have suffered from childhood violence on the part of the PLwD might be averse to handling their parents' issues in adulthood. Although some people can overcome these psychological barriers and discard their resentment to execute filial piety, most family members with such experiences only undertake a small portion of caregiving to an extent that will not trigger their traumatic memories or even avoid any involvement to protect themselves. Therefore, they put on a partial to full invisibility cloak according to the severity of previous conflicts and filter the messages they want to receive.

Because my mother [PLwD] has a strong son preference, even though she has dementia, she still likes me the most and doesn't like my sister. (Child 5, 56)

My mom has a bad temper, yes, extremely bad. She even hit my sister before, so my sister doesn't really have [love] for her. ... It's just about responsibility, having certain responsibilities is good enough. But there's a boundary to it; she won't go too far. (Child 4, 46)

Consequently, they tend to shoulder less caregiving responsibilities compared to those who are emotionally close with the PLwD. For example, when a younger sister got MRI results and sent them to her older sister, who had a conflictual relationship with the PLwD, her sister immediately said to never send her such things again and that she did



not need to know, blocking the message. She hid under the invisibility cloak, not because she was unfilial or lacked a sense of responsibility, but because she could not tolerate such a psychological burden due to childhood trauma. Her younger sister understood this and decided to make her responsible for picking up medication and other emotionless tasks without direct interaction with the PLwD.

I took a photo [of the MRI results] and sent it to my sister and the doctor, with an explanation. Then my sister told me, “Don’t bother me, I don’t want it. Don’t send me these kinds of things in the future. I don’t need to know.” It’s just that situation. She prefers less responsibility and doesn’t want too much of it. It’s actually a burden for her. Yes, I can understand. (Child 4, 46)

These negative family conflicts mostly happen between grandparents (G1) and parents (G2) or among siblings (G2); they seldomly involve the younger generation (G1). They are intentionally excluded from these conflicts as a way of protecting them. No matter how conflictual G1 and G2 get, they tend to love G3 unconditionally, because it is the collective goal for the multigenerational extended family to ensure the well-being and success of the only grandchild (Yan, 2018). Meanwhile, there is a tendency for Chinese parents to raise their children together with their grandparents or even leave the childrearing to the grandparents. Therefore, the relationship between G1 and G3 tends to be intimate at a very young age and is usually not influenced by family relationship history. Thus, grandchildren can be more attentive to their grandparent’s dementia than relatives who have a longer but more negative or conflictual relationship history with G1. For example, during her interview, a granddaughter mentioned she and her cousins lack knowledge of any negative family history, and she often unconditionally take the side of her grandparents in conflicts. After witnessing changing family relationships due to dementia, she asked her mother about previous family issues and deepened her understanding of the rationale for these conflicts, difficulties each family members faced, and long-established grievances. Gradually, she developed a more objective perspective when contemplating family relations, a personal development for the



interviewee.

But they [grandparents] definitely dote on all the grandchildren. We are not involved in those matters [conflicts]. We all look so adorable to them. They [cousins] probably don't even know—like my younger brother has no idea about the past family conflicts at all. My mother told me all these things. Sometimes I don't understand, so I ask her why, and she tells me. (Grandchild 5, 20)

4.3.3.3. Lacking a Sense of Responsibility

Whereas some receivers ignore messages due to a negative relationship history, some simply ignore messages because they are unwilling to shoulder the caregiving responsibility despite there being no previous conflicts. For example, when a daughter wanted her father to be more involved in her mother's dementia care, such as taking her for a walk or help her manage their finances, he would make excuses to avoid these tasks, such as saying there is no need to accompany her or that he cannot manage everything for her every day. In the interviewee's words, her father had been a "hands-off boss" for a lifetime, holding the attitude of male superiority and dismissing the importance of family matters. With an invisibility cloak, he hid from the responsibility he should have shouldered, and it was difficult to change him no matter how hard she tried. Thus, despite her many suggestions and her father clearly receiving her messages, he still chose to ignore them due to the lack of responsibility, which is fundamentally different from those who have a negative family history with the PLwD.

I: If you tell him [father] to take Mom for a walk, what was his reaction? Did he say anything?

P: He said, "Can't she go for a walk on her own?" It's a bit of male chauvinism, I think. It's that old-fashioned kind of male chauvinism, like, "If you want to take a walk, go ahead. Why do you need me to follow you? Do you need me to watch over you?" Something like that. (Child 1, 38)



In summary, although the message was spoken by the sender, it may not be heard by the receiver. They either regard any message related to the PLwD as noise because they do not see their personhood, they fail to receive the message because it is too implicit or they do not try to interpret its meaning, or they receive the message but ignore it due to a power imbalance, negative family history, or lack of a sense of responsibility. This makes it unlikely that the message will move the receiver to act. Findings show that previous family relationships, family organization, and family communication patterns all shape intergenerational family dynamics and influence the communication process (Esandi et al., 2021). Even though the receiver hears the message, it does not necessarily mean they will act. In the next section, I explicate the conditions when communication is seeable and speakable from the sender's side and hearable but unmovable from the receiver's side.

4.4. Unmovable 聽而不動

After going on a long arduous journey from seeing the PLwD's personhood, speaking out about the subject matter, and being heard by the communication receiver, the sender expects the receiver to be moveable. In this category, "move" implies two layers of meaning. One refers to what is seen, said, and heard touching them deep inside so they feel moved and faithfully acknowledge the necessity for action. The second refers to the receiver being moved or driven to act. However, as shown in **Figure 7**, families may be unmovable because they are pro-child rather than pro-older adults; thus, they put an invisibility cloak on the PLwD. Alternatively, although the receiver feels moved by the message, they might not be moved to action because it is difficult to challenge their deeply rooted ideologies, so they are unwilling or unable to make any changes. Another reason that prevents the receiver from moving to action is that there is no traumatic event. Although they are aware of the rapid progression of dementia and potential devastating consequences for the entire family unit, nothing significant has likely happened in early stage dementia. Therefore, they put a temporary invisibility cloak on the PLwD and will only move to action after something traumatic has happened and the invisibility cloak can no longer hide the PLwD's problems from the



family's attention.

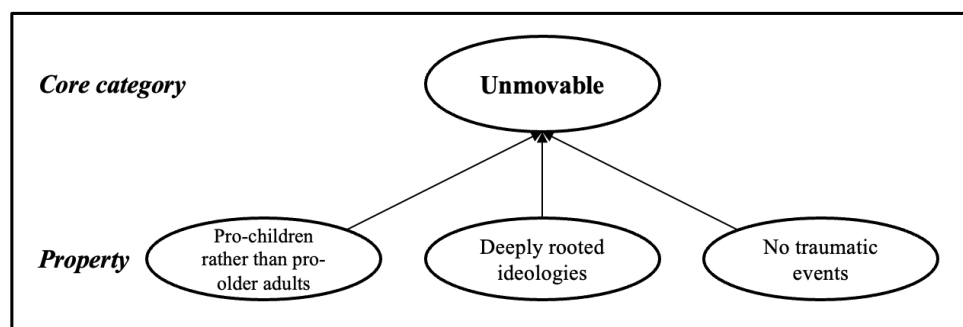


Figure 7. Illustration of the codes and properties under the core category of “unmovable”

4.4.1. Pro-Child Rather Than Pro-Older Adults

The parent generation may not be moved to action because they believe the grandparent generation's issues are not any more important than those of the grandchildren. For example, when a granddaughter tried to suggest that her parents take her grandmother for a medical checkup, her parents received her message and felt moved by her filial piety to care for her grandmother. However, they suggested that she leave it alone and that the best way to repay her grandparents is through effort and achievements in her studies and work, explaining that her grandparents would be glad to see these achievements rather than see her worrying about their health. They overrode the grandparents' interests with that of the grandchild. Although seemingly ironic in a society deeply influenced by Confucianism, this approach is well accepted by the parent (G2) and grandparent (G1) generation, in line with the neofamilism concept of pro-child rather than pro-older adult and different generational value orientations.

You working hard and achieving is the greatest reward for your grandparents. Tonight, you went back home with your younger brother, and I video-called your aunt later. They are very happy. (Grandchild 1, 25, sharing a conversation record with her father)

However, although neofamilism has heightened the importance of grandchildren in the family, leading them to receive more resources and attention, this narrative also leaves them no room to take part in other family issues beyond their personal realm. The most important issue for every family member has been predetermined—that is, the achievement and progression of the grandchildren. Whenever grandchildren want to meddle other subject matters, such as a grandparent’s dementia, it can be used as an excuse to exclude their voice and involvement. Thus, on the surface, it seems like this granddaughter’s parents put an invisibility cloak on the PLwD because they are no longer the focus of family and thus, they do not need do anything. At the same time, they gently put an invisibility cloak on the grandchild to freeze her plan of action for fear of interrupting their plan of inaction.

4.4.2. Deeply Rooted Ideologies

The older generation may be unmoved because of difficulties overcoming deeply rooted ideologies formed across the life span and accepting the opposing views of other family members. For example, some spousal caregivers resist hiring a maid or moving the PLwD to a nursing home because they perceive themselves as capable of providing care, despite having heard multiple times from other family members that home care would no longer maximize the patient’s well-being. Some spousal caregivers are too old to understand the pathology of dementia and why they should not engage in arguments with the PLwD. Although family members feel distressed at their resistance to change their ideologies or behavior, they often do not keep making these suggestions because it goes against the core belief of maintaining harmony and avoiding emotional upheaval.

My father-in-law said, “I can do everything. What’s the point [of hiring a helper]? I can do whatever he does.” He thinks he’s perfectly capable. (Child 3, 52)

My father is too old, and he cannot listen to me. (Child 5, 56)



Nonetheless, though difficult, some spousal caregivers are willing to integrate their interests with those of other family members when considering care arrangements. For example, although a spousal caregiver refused to send the PLwD to a nursing home and rejected his daughter's suggestion to do so many times at first, he eventually agreed because he was aware that if he passed away, his daughter could not afford to take care of the PLwD by herself due to her multiple commitments. It was necessary to place the PLwD somewhere to mitigate the burden for his daughter in the long run. Thus, although he was initially unsure about the quality of nursing homes, he was willing to take the perspective of his daughter and act. He visited and evaluated many nursing homes with his daughter, and eventually they decided on the best one. It turns out to be beneficial to the PLwD, spousal caregiver, and adult child.

I can't only focus on making her [PLwD] suffer less for my own sake. If something happens to me and I pass away, then it's my daughter's turn to handle this matter. How will she handle her [PLwD], right? She [PLwD] needs somewhere to be settled, and then my daughter can take care of her, that would be acceptable. (Partner 3, 86)

4.4.3. No Traumatic Events

Similar to the code of “keeping to oneself” explaining why communication senders reserve what they want to say until a crisis, family members often do not act because currently there are no traumatic events. Thus, they put the invisibility cloak on PLwD temporarily and activate in times of crisis when the cloak can no longer cover the problems. The symptoms of early stage dementia can be very light and even negligible if not closely observed. Therefore, prematurely bringing up something that is yet to happen, especially with negative connotations such as more severe memory loss and BPSD, can be considered undesirable or even as cursing older people in Chinese families, rendering early prevention and intervention unnecessary and inappropriate. For example, although family members are aware of the need to provide psychosocial and emotional support to the PLwD in addition to instrumental support, they did not



implement this idea until the PLwD shows violent behavior that can no longer be ignored.

Initially, we had very little understanding of the symptoms of Alzheimer's disease. Based on what we have observed so far, my grandfather is physically healthy. When we talk to him, he doesn't exhibit any strange behaviors. He also listens when we talk. So, we didn't realize that the condition could progress so quickly. (Grandchild 2, 27)

I was thinking about providing care in daily life, but it seems inadequate in terms of emotional and mental support. It was not until she [PLwD] exhibited some psychological and irritable behaviors that I realized that I should do something. (Grandchild 6, 27)

In summary, although problems can be seen and voiced by the sender and heard by the receiver, sometimes the receiver is not touched inside and does not act in families with early stage dementia. It is more culturally and psychologically acceptable to react to traumatic events rather than proactively trigger them.

In conclusion, in this chapter, I presented the key findings regarding the phenomenon of family communication about dementia becoming problematic between the message sender and receiver. Although some families had not encountered major problems in communication, most interviewees experienced communication failure, conflicts, or no communication related to dementia. As summarized in **Table 11**, the sender can first face challenges in empathetically seeing the personhood of the PLwD, because some family members reject illness claims and dismiss the PLwD's consciousness, which prematurely excludes the PLwD from family communication, and even makes such communication unfeasible among other family members. Although personhood and the problems arising from the disease journey can be seen, they are not always speakable. Family members sometimes censor their concerns in view of the hierarchical



relationship and their designated role in the family. After conducting such censorship, they might adjust their communication strategies according to the level of certainty that their message can be heard by and move the receiver. On the receiver's side, despite the message being delivered by the sender, they might not hear it due to perceiving the message as irrelevant noise, not properly receiving the message, or choosing to ignore the message. Last, even though the message is seen, spoken, and heard, the receiver may not act because their family is not pro-older adults, they hold unchangeable ideologies, or no traumatic events are occurring at the moment. As a result, the link between sender and receiver become disconnected. In the next chapter, I will provide contextual explanations on why family communication during early stage dementia can become problematic and theorize the studied phenomenon.



Table 11. Summary of the Core Categories, Properties, and Codes

Core categories				Properties	Codes
Message sender		Message receiver			
<u>Unseeable</u>	Unspeakable	Unhearable	Unmovable	Rejecting illness claims	Family making dementia invisible PLwD making dementia label invisible
				Dismissing self-consciousness	Covering up Framing intentionally Blurting out
Seeable	<u>Unspeakable</u>	Unhearable	Unmovable	Conducting self-censorship	Internalized hierarchy Designated role
				Adjusting communication strategies	Communicating directly Communicating informally Communicating indirectly Communicating matter of fact rather than feelings Keeping to oneself
Seeable	Speakable	<u>Unhearable</u>	Unmovable	Not seeing personhood	--
				Not receiving the message	--
				Ignoring the message	Power imbalance Negative family history Lacking sense of responsibility
Seeable	Speakable	Hearable	<u>Unmovable</u>	Pro-child rather than pro-older adult	--
				Deeply rooted ideologies	--
				No traumatic events	--



Chapter 5: Conclusion: Discussion and Implications

In this chapter, I first discuss the contextual factors that shaped intergenerational family communication about dementia and theorize the phenomenon with abstracted relationships among the four core categories, properties, and codes. Then, I present relevant implications for social work practice, reflect on the scientific rigor and the research topics. Finally, I discuss the strengths and limitations of my research and conclude the study.

5.1. Discussion

5.1.1. Cultural, Social, and Historical Context

To understand intergenerational communication about dementia, we must situate it into its appropriate context. First, we need to understand the cultural representation of self and others that gives context to how Chinese people communicate and interpret messages in particular social relationships. In the face of a disease full of uncertainty, we also need to figure out how Chinese people understand their relationship with the supernatural power of fate to understand their views of dementia, establishing the boundaries of the communication context. Second, family communication about dementia cannot be detached from the social context, especially how the medical system deals with dementia and treat PLwD, which gives context to what necessities communication and who is involved in the communication. Finally, intergenerational communication involves different value orientations across generational cohorts. Thus, family communication across three generations must be situated in their respective historical environment, indicated by economic, political, and cultural changes over time. In this section, I elucidate the cultural, social, and historical contexts that shape intergenerational family communication, with quotes and examples provided from the empirical data.

5.1.1.1. Cultural Context: Self, Others, and Fate

Interdependent Self



How people naturally or effortlessly perceive and understand the world is rooted in their self-perceptions and self-understanding, which are constrained by the given culture (Markus & Kitayama, 1991). Therefore, to understand the underlying factors that influence family communication, we must first figure out how the Chinese self is construed and how it shapes individual behavior. Self-construal refers to how individuals define and make meaning of the self in relation to others (Cross et al., 2009). Markus and Kitayama (1991) first proposed two distinct cognitive presentations of the self: independent and interdependent self. Whereas independent self-construal defines the self without connection to others and values unique internal attributes (e.g., traits, attitudes), interdependent self-construal views close relationships, social roles, and group memberships as central to the sense of self and incorporates others' views into self-identity (Cheek & Cheek, 2020; Giacomin & Jordan, 2020). In other words, for interdependent individuals, internal attributes are secondary and must be constantly controlled and regulated to realize the primary task of interdependence, which is also a sign of maturity (Markus & Kitayama, 1991). The interdependent self can be further divided into relational interdependence (based on close and dyadic relationships) and collective interdependence (based on group memberships). Although individuals can have both independent and interdependent views of the self, cultural contexts typically promote one or the other more strongly, with interdependent self-construal being more prominent in collectivist cultures like China (Cross et al., 2009). In addition to delineating the relationship between self and others, self-construal also shapes individual behavior, primarily in three aspects: cognition and information process, motivation and self-regulation, and interpersonal relationships.

Cognition and Information Processing. First, maintaining connections with others means constantly taking the perspectives of others and attending to the context of interaction, resulting in more other-oriented and context-sensitive self-representation. In decision making, interdependent individuals give more weight to others' views and fulfill others' goals. This requires the ability to feel and think about what others are feeling and thinking without being told—in other words, the ability to understand the



other's mind. It is strikingly different from the independent view of self in which one must say what is on one's mind if one expects to be attended or understood. On the one hand, inhibiting the "I" perspective and processing from the "thou" perspective can be beneficial to interdependent relationships, such as when family members cooperatively discuss family issues by giving attention to each other's thoughts and feelings rather than self-assertion. However, it may also result in a failure to determine what represents the best interest for others. For example, when it comes to a dementia diagnosis, family members might consider its meaning from the perspective of the PLwD and decide whether they should hide it and protect them from potential stigma, even if this means benevolently depriving the patient's right to know their situation.

In addition, the understanding self and others is usually embedded in a specific social context. When responding to communication, the first tendency often is to consider the particular social context defined by the current interaction, such as parent-child or younger-elder, then gauge the range of appropriate responses (Markus & Kitayama, 1991). For example, when hearing about his grandfather's dementia diagnosis from his parents, Grandchild 3 said he "did not instinctively show much of a reaction, because it was my parents who told me". His evaluation of the communication context with his parents is likely hierarchical and unequal, prohibiting his natural expression of feelings. He may find it easier to communicate with friends in an equal position about their thoughts and feelings. Such an interdependent view of self can influence how people process and react to information about the world around them (Giacomin & Jordan, 2020). As a result, selfness in an interdependent culture is a fluid concept. What one says and does can differ in different situations and based on the people involved in the interactions.

Motivation and Self-Regulation. Second, self-construal has implications for the manifestation of motivation and self-regulation. Interdependent individuals are motivated to be a part of social groups and maintain harmonious relationships, rather than pursuing individual autonomy. Even in stressful situations, interdependence



individuals are more likely to alter their feelings and regulate their emotional responses to accommodate the objective environment to maintain a sense of group cohesion (secondary control), rather than change the environment to fit their personal needs (primary control; Weisz et al., 1984). For example, in dealing with irresponsible brothers and sisters in dementia care provision, instead of engaging in direct confrontation to solve disputes, Child 4 and 5 lowered their expectations, adjusted the caregiving load in a way that can be accepted by others. They might even apologize first, to maintain harmony and preserve the source of help. Even if they hold conflicting views of dementia care, there is no point to solving the problem at the expense of harming sibling relationships. After all, for interdependent self-construal, maintaining the relationship is the goal in itself rather than a means to achieve individual goals. Changing the self according to the nature of the relationship and even subordinating personal goals to benefit the group is intrinsically rewarding, because it helps achieve harmony without confrontation or dominating behavior.

Later, I thought about it and decided to let it [the conflict] go. There's no need to make the relationship so tense. Later, I also talked to my sister about it. After all, it's just the two of us, me and my sister, there's no need to be unhappy about this matter. After I talked to her, she also understood me, and she talked to me, too. Anyway, I apologized and she apologized. In the end, things returned to normal. (Child 5, 56)

I can't make her [older sister] not come [to help]. That would be troublesome, right? I would be even more disadvantaged, right? At least she is still a pillar, so I need to balance it well. She will come [to provide care] when she is able to come, so I'll make sure the level of care is appropriate [for her to handle]. (Child 4, 46)

In another case, when Grandchild 1 tried to persuade her parents to take grandma for a medical checkup, her parents rejected and claimed getting the diagnosis would bring her no benefit and might even make her feel like a sick person. While originally holding



a firm belief of the need to get a medical diagnosis, she suddenly felt it made sense since they should not “burden her with a heavy psychological load”. Therefore, instead of trying to persuade them, she adjusted, convinced herself of her parents’ logic, and conformed to her parents’ decision, inadvertently justifying her replacement of her personal goal with that of her parents. In such situations, conformity does not necessarily reflect the inability to resist social pressure and stick to personal attitudes and beliefs. Rather, conforming and not being assertive toward her parents, with whom she is interdependent, can be highly valued as a sign of maturity in Chinese culture (Markus & Kitayama, 1991).

The act of voluntary adjustment can lead to growing self-control over time. There are two basic self-regulatory foci: a promotion focus that motivates people to pursue desired end goals, and a prevention focus characterized by an avoidance of undesired states (Higgins, 1996, 1997). For interdependent individuals, compared to failure to pursue desired goals, the failure to live up to one’s obligations or external expectations can bring more significant negative relationship consequences. Thus, family members are expected to behave based on those roles rather than selfishness to maintain harmony (Chen & Chen, 2020). Interdependent people are highly conscious of where they belong in relation to others and assume a receptive stance toward others’ designated roles. In summary, the interdependent self is motivated to maintain harmony rather than autonomy, and these people actualize this motivation through voluntary self-regulation, such as adjusting their behavior and trying to meet the expectations of others.

Interpersonal Relationship. Third, people with an interdependent self-construal focus on fostering their relationships and group memberships. Although PLwD suffering from cognitive declines may face increasing difficulties in social interaction, they still strive to maintain connectedness with society to actualize their collective interdependence. When PLwD 3 was asked why he keeps joining daycare activities, he said if he stays at home, “no one cares about me, and I mean that people in society do not care about me”, implying that the interdependent self is not only construed through



close or dyadic relationships with family members, but also inseparably linked with recognition from other members of society. Nonetheless, he understood that people in society cannot care for him, because they do not know about him. Therefore, he proactively joined social activities and created opportunities for social workers, other PLwD, and volunteers to learn about and care for him, eventually fostering his sense of collective interdependence. Fitting in the social group, thus, is an important basis of self-esteem.

Although the pursuit for collective interdependence drives some PLwD to be more socially active and maintain group memberships, it also moves them to another social group that is highly stigmatized. Thus, in a society primarily holding an interdependent view of self, a dementia diagnosis is not only a medical process but also a social act that can implicate outsiders, thus having significant social implications for PLwD's interpersonal relationship and decision-making rights. Whereas for insiders, getting a diagnosis at a mental health center involves learning more about their condition, outsiders may label PLwD and avoid them, thus reducing their social network. In line with previous research (e.g., Portacolone et al., 2014), PLwD have been found to be prematurely deprived of the right to manage their financial accounts and make other decisions to avoid inconveniencing other people in society, thereby harming their social life. Thus, families have to exercise caution when disclosing the diagnosis to outsiders and even insiders who are not very close, to avoid potentially devastating social consequences. But still, some PLwD voluntarily reduce their social interactions due to the fear of making mistakes and losing face in front of others.

But it [dementia] has a significant social aspect. ... Sociality (She hui xing 社會性) means that your social life will be affected. Many people don't want others to know that they have been to a psychiatric hospital. It's a social phenomenon. They will avoid such things. What are you avoiding? People will distance themselves from you—that's sociality, right? Similarly, in social interactions, if you discover your conditions early, for you, it's self-understanding, but when others come to know



about it, they will label you, especially those who are unfamiliar with this condition. They will label you because you are a stranger to them. (Child 4, 46)

In addition to keeping connections with society, interdependent self-construal also shapes interpersonal communication behavior, including how individuals express themselves and interpret messages. People with an interdependent self-construal are highly sensitive to communication contexts, such as where and when they should communicate; they are highly attentive to and accommodating of others' feelings, perspectives, and goals, even at the expense of their own; and they readily adopt indirect communication strategies to avoid confrontation. In summary, although in an individualist culture, the view of an autonomous self that pursues personal needs and goals and resists conformity is a sign of maturity, interdependent self-construal regards changing personal behavior in response to different contexts and regulating emotional expression to maintain group harmony as signs of maturity (Cross et al., 2009). For Chinese people, self-construal is fundamentally connected to other people; thus, they strongly focus on their relationships with others. However, this does not imply that all Chinese people predominantly hold the interdependent view of self. Rather, independent behaviors exhibited by a person in an interdependent culture may carry different significance and signify cultural change, as discussed in Chapter 4.

Fate and Fatalism

Whereas the previous discussion of the interdependent self highlighted the Chinese understanding of self and others, this section illustrates how Chinese people understand the relationship between self and fate, a supernatural power beyond human control. Chinese people often attribute the cause of dementia to fate, or a normal course of life. Chinese philosophy provides a mechanism to discriminate what can be changed (subject to human agency) and cannot be changed (predetermined by fate; Raphals, 2003). This is an incentive to encourage people to do all that is in their power to advance as much as possible within the boundaries established by fate. As such, although the occurrence of dementia may be fated, free will still reigns amid its boundaries. The



belief in fate positively affected families with dementia in this study in three aspects: coming to terms with the dementia diagnosis and caregiving role, becoming free from worry, and coming to terms with dementia symptoms.

First, the belief in fate helped these families to go through the painful initial stage and come to terms with the dementia diagnosis. Although latent research has explicated the risk factors of dementia, such as lower education level and physical inactivity, this does not always work when trying to explain the cause of dementia to a family. In this study, some PLwD had obtained a master's degree, worked in the high-tech industry, and worked out every day before being diagnosed with dementia, but still they became afflicted with dementia and even doctors cannot provide a reasonable explanation. Who, how, and when dementia occurs, as described by a social worker, is like opening a "black box." When fate opens this black box, nobody knows who the next PLwD will be. Therefore, family members and social workers can only use fate to explain a dementia diagnosis to PLwD, evading its inexplicability and unpredictability. They also often reorient the PLwD's attention to what they can control, such as proactive prognosis and intervention.

According to Western beliefs, illnesses are the arrangement of God. But we Chinese people don't believe in that. We see illness as a natural occurrence, not because someone wants to harm you intentionally. (PLwD 3, 77, early stage)

Similarly, family members often question why PLwD has been chosen for dementia and why the family has been chosen for insurmountable suffering, and they eventually find no answers except fate. Although dementia is a misfortune, they understand it as the arrangement of fate that goes beyond human control; thus, there is no need to bargain with an impersonal force, only to accept and manage their new reality. Such belief also helps family caregivers accept their changing role and associated burden, because fate has chosen them to undertake the caregiving role rather than others. For example, Child 5 mentioned that when he was unemployed several years prior, it



happened to coincide with his mother's initial dementia symptoms, like fate had chosen him to be her caregiver, "as if it was planned that way". Thereafter, he did not get back to work and started to care for the PLwD full time, following the arrangement of fate. The explanation of fate is also widely adopted when multiple brothers and sisters are qualified for the role of primary caregiver, but it falls on one particular person.

Interviewer (I): Do you feel like you've had particularly high psychological and caregiving stress over these years?

Participant (P): There's no other way. It's your turn. It's fate, what can you do?
(Partner 3, 86)

Second, adopting the belief of fate can free PLwD and family members from constant worrying. Although a dementia diagnosis is a life-changing event understandably associated with feelings of fear, both PLwD and family members often feel there is no need to worry because this is their fate and it is beyond their control. Instead, what is left to be controlled is their mental state and attitudes, which should be their focus across the disease journey. As pointed out by PLwD 3, he had already accepted that dementia would take away a few years of life from him. If he constantly worried, his life might only become shorter, bringing him no benefit. Therefore, he opened his mind and fully embraced what he saw as a predestined diagnosis. He remained positive about what can be changed, such as making his attitude toward dementia more positive and social participation more frequent to ensure good quality of life.

That means once you come around [to acceptance of the diagnosis], maybe my life will be a bit longer. When you have this condition [dementia], originally your life will be shorter. But if you keep worrying and constantly thinking, "What should I do?" I'm sorry, but your life becomes even shorter [laughs]. (PLwD 3, 77, early stage)

I: Does Grandpa [spousal caregiver] think you need to worry about Grandma's

dementia?

P: He doesn't think so. He just thinks it's really unlucky that it happened to Grandma, but since it happened, then just let it happen. (Grandchild 4, 32)

Third, in addition to helping with acceptance of the dementia diagnosis, the belief in fate also helps family members come to terms with dementia-related behaviors, especially BPSD. In general, there are two types of PLwD: One has negative attitudes toward accepting care and seems depressed, and another resists care with attacking behaviors, which is a manifestation of severe BPSD and described by family members as one of the most difficult aspects to tolerate and accept. In this fieldwork, Chinese family members and social workers often used the metaphor of “Wen and Wu” to distinguish the two categories. Traditionally, Wen and Wu are used to describe Chinese political culture, indicating the opposing but complementary civil and military realms of government. When it comes to an individual, this describes an ideal person with both academic and military achievement in ancient China, which is also the cultural and family expectation of an accomplished man. In the context of dementia, family members applied the two concepts to the seemingly distinct but also sometimes coexisting symptoms. In the eyes of family members, whether the PLwD's dementia symptoms are Wen or Wu is also a black box, because it not always determined by their personality or educational level. According to some family members, they noticed that their previously gentle PLwD can gravitate toward the Wu subtype, be high or low in both Wen and Wu, or change their levels of Wen and Wu as the dementia progresses. This can only be explained by fate; they cannot choose the level of severity. Although to some extent, the Wen type is more acceptable compared to Wu because these PLwD are quieter and easy to take care of, they are also indifferent and have little interest in anything. Thus, it is difficult to connect them with society and provide intervention. The Wu type, in comparison, although it involves more BPSD symptoms, also involves more social participation. Thus, Wen and Wu are no more fortunate than the other, and family members can come to terms with both types and develop coping strategies accordingly.



Nonetheless, in unfavorable situations, the belief in fate may slide toward fatalism and even nihilism. Fatalism is the belief that all aspects of life are predetermined and unchangeable by human agency; thus, there is no need for distinguishing or worrying about what can be altered (Raphals, 2003). Fatalism has been widely adopted in cancer research because cancer fatalism is a common occurrence in both Western and Eastern cultures (He & Li, 2021). It is defined as a belief that hinders people from seeking cancer information, prevention, management, and intervention (He & Li, 2021). The conceptualization of fatalism not only encompasses predestination, but also includes pessimism and attribution of cancer to bad luck, rendering it a barrier to cancer control. Not until recent years have researchers discovered that Chinese beliefs regarding dementia are deeply influenced by fatalism, too (Liu et al., 2022; Sun et al., 2023). In addition to viewing dementia as predetermined by fate, some also believe the outcome of dementia is not affected by human behavior—thus, developing an escape–avoidance coping response to dementia attribution, disease management, and long-term preparation. A Taoist way of “non-doing”¹ (*Wu wei* 無為) is thus adopted for self-control.

First, people might normalize the occurrence of dementia and regard it as normal part of aging, which is inherently different from the view of dementia as a normal course of life mentioned previously. This belief in dementia as a normal course of life regards whatever fortune or misfortune as fixed; thus, dementia simply needs to be accepted and managed like previous misfortune without the need to worry much. This reflects the Buddhist wisdom of accepting whatever happening with a calm and composed mindset in the natural flow of life. Instead, the belief in dementia as normal aging mistakenly attributes dementia to the unchangeable aspects of fate in old age. Alternatively, people may demonize dementia and use folk explanations (e.g., seeking

¹ *Wu wei* (無為) is a key concept in Taoism, or “The Way” philosophy, that translates to “non-doing” or “effortless action”. It is not about giving up or being lazy, but rather about achieving the natural flow and developments of life events.

a fortuneteller to cure dementia) to understand its cause rather than adopting the biomedical model, which is also consistent with Chinese beliefs and taboos regarding dementia. Second, because family members might only see the incurable side of dementia, they often delay or reject dementia screening, diagnosis, and treatment and lack the initiative to seek what is controllable, such as nonpharmaceutical interventions. Third, family members may hold a lazy attitude or lack of planning because they feel they have no control over an uncertain future. Although they are somewhat aware of the severe consequences of dementia, they are determined to live for today and save communication for the “right” time. If they communicate earlier but the dementia turns out to be not a big deal, it would be a waste of effort to address uncertainties in advance. However, if the dementia turns out to be devastating, no matter whether they communicate, they cannot change destiny; thus, they would rather not communicate now. Thus, with a positive outlook, they proceed without communicating about their plans and decide how to act only when a crisis arises.

I would rather believe that my mother won't be in that [severe] state within the next 3 to 5 years, so I want to make the most of each day now. That's my mindset. (Child 1, 38)

We haven't talked about it yet. We just proceed without a plan and decide how to do things later. (Partner 5, 76)

In summary, predestination plays a pivotal role in Chinese culture. The belief in fate empowers both PLwD and family caregivers in the Chinese context by helping them come to terms with dementia and ease their psychological burden; thus, family members hold positive attitudes toward open communication about dementia. Instead, fatalism makes family members feel powerless and thus, avoid informational seeking, disease management, and prognosis discussions, which further prevents them from dementia communication.



5.1.1.2. Social Context: Medical System and Patient Autonomy

Medical System

As argued in Chapter 1, dementia by nature is highly complex and insidious and thus, necessitates family communication to address its uncertainties. Nonetheless, the complex nature of dementia is further complicated and exacerbated by the inadequate health care system in China (Jia et al., 2016), imbuing dementia communication with uncertainty and ambiguity. In the current study sample, participants rarely received a dementia diagnosis at their first medical checkup after noticing symptoms. They often obtained a diagnosis after visiting various hospitals, departments, and professionals, spanning years to decades after first noticing the symptoms. Child 3 in Chengdu mentioned that very few doctors specialize in dementia care and it is very rare that symptoms are identified as signs of dementia at the first medical checkup. This is in line with the findings of Gauthier et al. (2021), who showed that it can be very challenging to distinguish early stage dementia from normal cognitive aging. Child 4 in Shanghai went to a mental health center and received a dementia diagnosis after the doctor listened to her description without any further checking. Child 1 in Beijing had a second medical checkup in a neurology department, but despite the patient's severe forgetfulness, the doctors "think it wasn't Alzheimer's disease" because "memory loss is common in old age". They also "suggested getting the thyroid checked because thyroid issues can also cause forgetfulness".

The doctor was quite funny. He didn't do anything, but said it was Alzheimer's disease just after hearing our description. I didn't think it was very reasonable because he didn't take any tests, examine the brain, or do an MRI. He just listened to our narrative and concluded that it was Alzheimer's disease. I thought it was too arbitrary. (Child 4, 46, Shanghai)

After the examination, to be honest, he [doctor] couldn't tell much [about my condition]. (PLwD 3, 77, early stage)



Even after providing a dementia diagnosis, some doctors may not inform the patient or family about which stage it is, some are unwilling to provide MRI images to a family caregiver, and they frequently not discuss the prognosis. For example, Child 2 mentioned that she felt anxious and distressed after getting the results, but the doctors “didn’t say anything, they don’t tell you anything”. She said “the doctors today are not as concerned as we are, they have seen so many cases”. In line with previous research (Brodaty et al., 1990; Stokes et al., 2014), the doctors did not say anything about the prognosis despite the family members desiring a realistic discussion about the implications of the diagnosis. In another case, when Child 5 in Shanghai wanted to retrieve a formal medical diagnosis from the hospital to obtain a government subsidy, he was informed that the medical system had just upgraded and thus, all its previous data had been lost. Therefore, he had to take the PLwD for another formal diagnosis.

Although these are only barriers faced by the interviewees in this study and do not fully represent the deficits in the medical system, these findings provide insight regarding the difficulties and barriers caregivers face in trying to get a formal diagnosis and accurate prognosis information. It is also notable that these deficits were mostly noted by participants residing in Beijing, Shanghai, and Chengdu, the most developed cities in China with the most advanced medical expertise. Given the pronounced urban–rural divide, it is not hard to imagine the difficulties experienced by people from less-developed cities and rural areas. As family members gradually lose confidence in the medical system, this has significant communication consequences for the family. Caregivers are often in a state of uncertainty, and they are unable to assert themselves persuasively and face increasing difficulty in broaching sensitive topics. For example, when a caregiver failed to obtain a formal diagnosis in two hospitals, she said it was difficult to persuade her family members to go for another medical checkup, because they had lost confidence in anything she said. Therefore, an efficient and considerate medical system would empower caregivers to become more assertive and powerful communicators, thus facilitating family communication about serious issues like dementia.



Patient Autonomy

Along with complicating the process of acquiring a dementia diagnosis and lowering the level of certainty for family members to communicate about dementia, problems embedded in the medical systems also systematically perpetuate the lack of patient rights. The unbalanced triadic therapeutic relationship among patients, doctors, and family caregivers has excluded PLwD from health communication. In the last decade, the view of truthful dementia disclosure to PLwD has shifted from whether to tell them to how and when to tell them, reflecting the increasing recognition of PWLD's autonomy in managing their conditions and the benefits of full disclosure. Although patients' rights and autonomy are widely accepted in Western countries, in China, doctors place greater emphasis on informing the patient's family, even if the patient still has the capacity for decision making. Doctors usually let the family members decide whether they will disclose a health condition to the patient (Lou et al., 2023). In subsequent health care decisions, the family's decision can overrule the patient's wishes, resulting in so-called "family-informed consent." In a study investigating Chinese young doctors' attitudes toward patients' informed consent, only 5.4% said informing the patient alone is sufficient when it comes to serious conditions, and the percentage dropped to 3.8% when it came to older patients (Xu & Yuan, 2024). Most doctors said they would primarily ensure the family members were informed, because they are believed to sufficiently represent the best interests of the patient (Xu & Yuan, 2024). In this prior study, 79.6% of doctors said they would cooperate with family members if they wanted to conceal a condition from older patients to protect their optimism and psychological well-being (Xu & Yuan, 2024). This is strikingly different from practices in Western countries, where it is hard to imagine families raising such a request and doctors complying. Even in some countries, professional ethics explicitly prohibit doctors from withholding information from patients, but this is neither guaranteed nor protected by Chinese law (Lou et al., 2023).

In Chinese culture, which emphasizes harmonious interdependence, individuals are part



of family units and individual autonomy requires family determination (Tse et al., 2003). Thus, self-determination is less recognized than family determination (shared decision making, with the family as the proxy regardless of the agreement of the patient), especially in aging families. Nonetheless, in recent years, a reverse trend is emerging, especially in highly educated groups in urban China. For example, Zou et al. (2017) found that most well-educated individuals in Shanghai favored disclosure when they or their family members were diagnosed with Alzheimer's disease. Because family determination is culturally ingrained and can take a long time to be replaced with self-determination, this study unveiled something that more severely deprives patients of their rights that necessitates more urgent intervention in medical settings. When asked whether their doctor told the patient about their diagnosis in addition to family members, many caregivers (e.g., Partner 4, Child 1) said they picked up the diagnostic results without the patient; thus, these patients were automatically precluded from knowing their condition. In this way, the power of disclosing naturally fell into the hands of the caregivers. PLwD, without any clue to their situation, may find it difficult to initiate such communication by asking their children about their medical results.

Our son also went [to the hospital], and the doctor explained the diagnosis results to both of us. (Partner 4, 80)

I took her [PLwD] for the examination, but I went by myself to collect the results. (Child 1, 38)

After the diagnosis, PLwD are often prescribed medication every month from the hospital to prevent the misuse of psychopharmaceuticals. Surprisingly, in the current study, almost no PLwD went to hospital alone to pick up the medication. Instead, their caregivers picked it up on their behalf, communicated with doctors about their recent condition, and adjusted the dose accordingly. As the quote below indicates, even after years of diagnosis, some PLwD still did not know what medication they were taking, and some didn't even know they were taking dementia medication. They had no



opportunities to discuss their situation with the doctors and adjust medication, even at the early stage of dementia, because this task is handled by their children.

P: He takes medication anyway, usually for high blood pressure. He also takes medication for the prostate and heart. So, I put the dementia medication together for him to take.

I: Does he know specifically what he's taking?

P: He doesn't know [whispering]. (Partner 4, 80)

Most surprisingly, in this study, Grandchild 1 used her name to book an appointment with a geriatrics doctor to get a dementia diagnosis without her grandmother. Although this family member faced practical difficulties in bringing the PLwD to the hospital, such as transportation difficulties and her parents' dismissal, it is indeed surprising that the doctor eventually provided the diagnosis after listening to her description and offered suggestions on what medication and doses her grandmother should take.

Because it's under your name, I can't prescribe medication for you. I would suggest considering buying this medication for his condition. Take 5 milligrams of this medication once a day in the evening, and if there is no dizziness or discomfort after a month, you can increase it to 10 milligrams every night. (Grandchild 1, 25, sharing a recording of a geriatrics doctor)

There are two underlying reasons for the deliberate exclusion of patients from diagnosis and medication discussions. First, in a caregiving context deeply influenced by Confucianism, family caregivers, especially adult children, regard taking over these “trivial issues” as an act of love and filial piety. Aging Chinese parents are often pleased to see their children's concern and involvement in their disease journey (Xu & Yuan, 2024). Second, the older population in China has a lower education level on average. Only 13.9% of the population aged 60 or older has a high school education or beyond, which was confirmed in the current sample, in which grandchildren mostly had a higher



education level than their parents and especially grandparents (Seventh National Population Census of China, 2020). As such, having less education may significantly limit their ability to fully understand their medical conditions, especially a complex condition like dementia. Therefore, informing family members can improve understandability and efficiency in busy medical settings.

Although not all patients want to know their diagnosis and the right not to know has to be respected, recent research has shown that most older patients in China want to be informed by doctors about their diagnosis and prognosis (Stocklassa et al., 2022). However, the common practice for family members to pick up diagnostic results, monthly medication, and even get the diagnosis without the presence of patients in the name of love has indiscriminately deprived patients of their right to build a patient–doctor relationship, leaving many PLwD with no room to bargain because the caregivers bear no maleficence. Therefore, although the current literature mostly focuses on whether doctors should disclose a dementia diagnosis and discuss the prognosis directly with PLwD, the current study showed that doctors may not even experience such concerns because the patients never show up. It is also a common practice for doctors to accept family caregivers as a proxy, without knowing the preference of the patient. This is a severe violation of the principle of person-centered care and the common practice of patient autonomy and independence. Although the traditional biomedical model and authority of professionals over dementia care have been increasingly challenged and taken over by relationship-centered care that puts PLwD and family caregivers at the center, the current practice in the Chinese medical system generally empowers family caregivers rather than patients. This strongly paternalistic approach cannot be justified by the belief in interdependence. As a result, such routine practices unintentionally make PLwD invisible in the disease journey and thus, excluded from family communication about dementia. Therefore, practically speaking, before empowering doctors to adopt individual informed consent from patients, the current medical system must ensure the presence of the patients at all medical stages, else the diagnosis, prescription, and treatment should not be enacted.



5.1.1.3. Historical Context: Intergenerational Value Differences

A generational cohort is a group of people born and living at the same time who have experienced similar macrolevel social, political, and economic events during their preadult years (Mannheim, 1952). Because a person's fundamental worldview is determined by their experience of youth, different historical settings result in a generational identity and form distinct generational value orientations that remain relatively stable throughout the lifetime (Fornas, 1995; Inglehart, 1997; Strauss & Howe, 1991). Therefore, younger and older generations will express different values and attitudes even toward the same life event, thus engendering less communication. For example, during an interview, PLwD 3 mentioned he has less communication with his grandson. He emphasized that it is not only his personal experience but “a social phenomenon”, because they have totally different worldviews and attitudes toward society and thus, they are unable to communicate well. These distinct values, beliefs, expectations, and behaviors are also important for understanding the evaluation of cultural change. With China's significant socioeconomic changes over time, this study divided the three generations of family members by integrating key historical events that affected the older generation (G1), middle generation (G2), and younger generation (G3; Liu, 2024).

G1, the “pre-reform generation,” was born between the early 1930s and early 1950s. This generation experienced political transitions and intensive turmoil such as the establishment of the People's Republic of China, the Great Leap Forward, and the Cultural Revolution. At the same time, Chinese people suffered from poor living standards and material shortages. Thus, the social upheaval and resource scarcity engendered modernist survival values, such as rationality, materialism, conformity, and respect for authority (Egri et al., 2004). Among all three generations, G1 members raised during the Cultural Revolution generally attribute the greatest importance to the value of power, because the extreme societal instability they experienced strengthened their survival values and pursuit of power and status. In addition, education received by



G1 mostly focused on the promotion of the Communist ideology that emphasized contributions to the nation and downplayed the importance of self. Thus, G1 members are less likely to express their thoughts and feelings. As mentioned by Child 4 (G2), her mother (G1), like most parents of that generation, does not know how to express love and affection to her children. Although she can understand, it still affects her caregiving decisions because she cannot reciprocate with love. Her care is purely provided out of responsibility, and anything beyond the boundaries of responsibility is not considered. Nonetheless, despite the lack of affectionate expression, G1 members have more time to attend to the family care needs of both older parents and their children, because the planned economy has guaranteed job security for a lifetime.

I actually don't feel love towards my mother in our relationship, it's more responsibility. In my mother's generation, parents wouldn't express love, and she lacks such knowledge. Considering her family of origin, she also lacks that. So, I can understand this state of mind. (Child 4, 46)

G2, the “reform generation,” was born between the early 1960s and early 1970s. These individuals experienced a transition period toward economic reform and modernization, from turmoil to order, from planned to market economy. Although the unprecedented opportunities enabled wealth accumulation and self-achievement, the housing reform and job mobility engender G2 members to delay the activation of support until their parent's need levels are perceived as high (Ikels, 2004). They are less likely to devote substantial amount of time to their extended family without suffering financial consequences, which is the same for G3 (Liu, 2024). Thus, compared to G1, G2 and G3 have reduced their instrumental support provided to the family on a daily basis as a generational characteristic. Meanwhile, the implementation of the one-child policy since 1979 has dramatically increased the value of G2's only child in the family, tilting the power balance across generations. G1 and G2 make every effort to ensure the success of their only child in the competitive market, and G1 also avoids burdening G2 with their problems or diseases so that they can make money and spend time supporting



G3's development (Liu, 2024). As a result, as the family priority, G3 members are less likely to surrender to parental authority. Research has also found that younger Chinese parents in G2 have embraced more warm and open communication patterns with their children to respect their autonomy and independence. Nonetheless, the tilted power balance has not resulted in the neglect of the older generation because the ideology of filial piety remains strong. G2, thus, has become the “sandwich generation” that bears the most complex and bidirectional family responsibility.

G3, the “post-reform generation,” was born between the mid-1980s and 2000s. These people experienced more rapid economic, societal, and cultural development and a stronger shift toward the focus on human development, which all converge to create greater individual autonomy and self-expression (Tang et al., 2017). Together with the improved living standards and economic security, G3 members also have enjoyed the rapid growth of information technology, which exposed them to information from within and outside China; thus, they are more open to the Westernized lifestyle and individualist values. The system of nine-year compulsory education further equipped them with richer knowledge and opportunities to pursue higher education. In the current study, the grandchildren interviewees also perceived themselves as having more advanced knowledge of dementia, such as being more likely to know dementia is a disease rather than a normal part of aging. They are also more attentive to psychological intervention and brain stimulation rather than only considering traditional pharmaceutical treatment. In sum, G3 is increasingly open to change, more secular, more likely to regard self-development as the most important thing rather than making contributions to society, and more individualist (Sun & Wang, 2010; Tang et al., 2017). Such a significant shift toward new value priorities also indicates a more open and inclusive culture, with G3 increasingly embodying an independent view of self compared to G2 and G1.

Despite these differences, there are also value continuities across generations, such as strong crisis-induced intergenerational solidarity (e.g., hospitalization and terminal care)



among G1 and G2, which is highly likely to extend to G3 (Liu, 2024). In summary, these Chinese generational cohort differences and similarities in value orientation have strong implications for understanding generational views and conflicts in dealing with dementia. Their different views toward power and hierarchy, knowledge gaps regarding dementia, and different levels of self-expression shaped by the historical context during their formative years certainly make family communication more complex and even conflictual. Nonetheless, because Chinese culture prioritizes achieving harmony, it is also interesting to explore how the three generations negotiate their divergent value orientations through communication.

In conclusion, this section delineated the cultural, social, and historical contexts that shape the intergenerational family communication context about dementia. The understanding of self, others, and fate affect communication in specific interpersonal relationships. Dementia involves more than the primary caregiver, affecting the entire family unit in which the patient is embedded, involving actions and decisions across generations. The current shortcomings of the medical system and neglect of patient autonomy regulate the certainty of communication and who is allowed to communicate. The generational cohorts further reveal underlying differences in value orientations, thus situating intergenerational family communication in their respective historical contexts. In next section, I further analyze the core categories, properties, and codes on a higher level and seek theoretical explanations regarding the intergenerational family communication process about dementia.

5.1.2. A Theory of Connected and Disconnected Communication

With a more in-depth understanding of the cultural, social, historical contexts, in this section, I abstract these concepts by theorizing about the studied phenomenon, providing key assumptions, and illustrating their meanings and consequences.

Grounded theory is a methodology used to study how people resolve a main concern. It explicates the problem-solving process and reveals its complex variations.



Constructive grounded theory also aims to study how and why people construct meanings and actions in specific situations. In this study, based on the overarching research question—How do intergenerational family members communicate about dementia-related issues?—I have examined three key communication processes: detecting symptoms and disclosing diagnosis, arranging caregiving tasks among family members, and managing negative emotions. Although a few families in this study had no problems with such communication, most Chinese families found it problematic to communicate about early stage dementia across generations. They had no communication, failed communication, or conflicted communication that devastated their family relationships. Therefore, in addition to addressing what and how it happened and the consequences in Chapter 4, I mainly answered the question of why—namely, why communication become problematic and what factors may have led to this outcome in the previous section.

Communication (溝通), according the traditional Chinese interpretation, means digging a ditch to connect two parallel rivers. Later, it has been used to refer to connecting two opposing parties and addressing their divergences. In this sense, communication in the context of an interpersonal relationship also involves two parties: message senders and receivers. The aim of communication is to successfully connect sender and receiver by ensuring the message is clearly seen and spoken by the sender and heard by and able to move the receiver. Through the lens of sender and receiver, the current study revealed that dementia communication can become problematic if the message fails to be delivered from the message sender to receiver, amid particular cultural, social, and historical contexts. Thus, the general research findings reflect this phenomenon as “disconnected communication,” subsumed by four sequential abstract concepts: unseeable (message sender), unspeakable (message sender), unhearable (message receiver), and unmovable (message receiver), as illustrated in **Figure 8**. The first assumption is that communication becomes disconnected if family members have failed in at least one of these four categories, thus making the subsequent steps unfeasible. For example, if the message sender did not see the patient, they would not be able to



speak, be heard by receiver, and move the receiver to act. If the family sees the patient but does not vocalize the problem, the message receiver will not be able to hear the message and act. If the sender has seen and spoken the problem but is not heard by the receiver, the receiver can still be left unmoved to action. If the sender has seen and spoken the problem and has been heard by the receiver, the receiver can still be unmoved. These four scenarios are defined as “disconnected communication”, in which the message sender and receiver either make each other or the PLwD invisible during these communication processes and become disconnected.

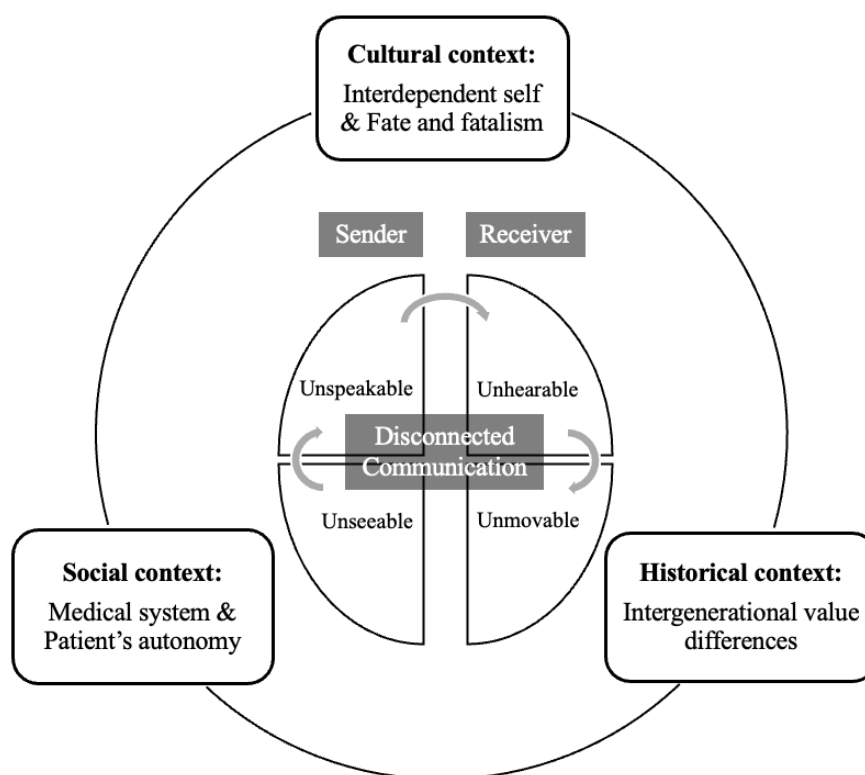


Figure 8. Theorization of Disconnected Communication

Alternatively, the second assumption is that a desirable family communication pattern is seeable (message sender), speakable (message sender), hearable (message receiver), and movable (message receiver), wherein family members can empathetically see each other, speak to each other, hear each other, and be moved to act, which is defined as “connected communication”, as illustrated in **Figure 9**. With nobody putting the invisibility cloak on each other in the family, the message between the sender and

receiver can be successfully connected. There are five hypothetical pathways for the eight dimensional constructs, seeable and unseeable, speakable and unspeakable, hearable and unhearable, movable and unmovable, as shown in **Table 12**. Quantitative methods can be used to test these pathways and explore other possibilities in future research.

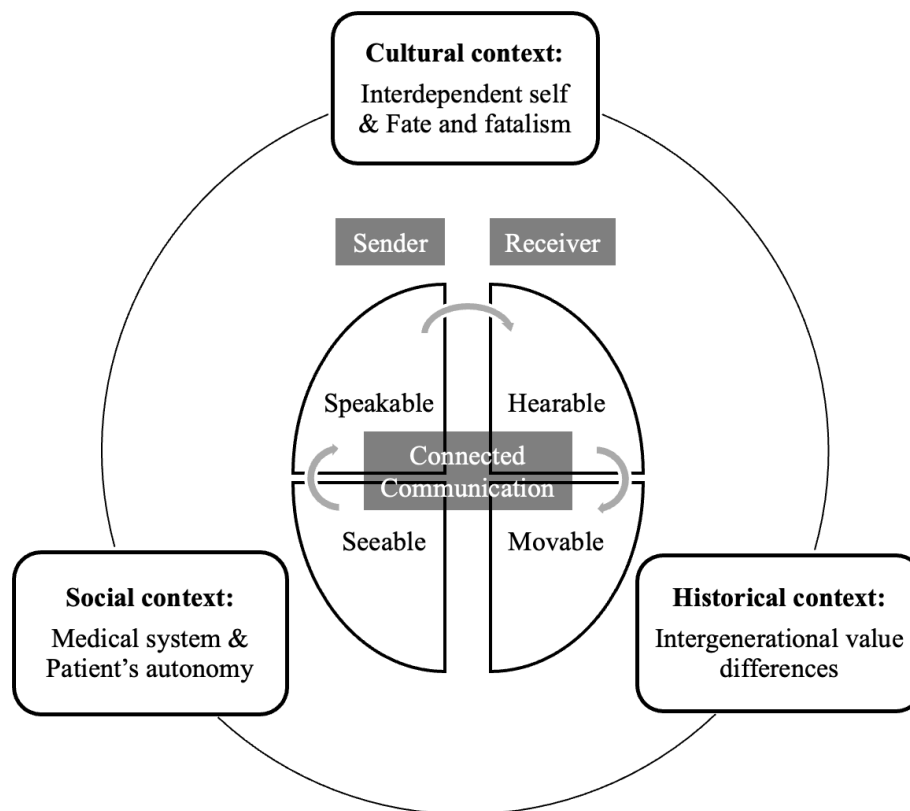


Figure 9. Theorization of Connected Communication

In the next sections, I first engage the theory of connected communication with the current research findings, previous research, and existing theoretical perspectives. Then, I explain how my research practice supports my scientific findings and offer considerations for future research.

Table 12. Summary of the Theorization, Core Categories, Properties, and Codes

Theorization	Core categories				Properties	Codes
	Message sender		Message receiver			
Disconnected Communication	<u>Unseeable</u>	Unspeakable	Unhearable	Unmovable	Rejecting illness claims	Family making dementia invisible PLwD making dementia label invisible
					Dismissing self-consciousness	Covering up Framing intentionally Blurting out
	Seeable	<u>Unspeakable</u>	Unhearable	Unmovable	Conducting self-censorship	Internalized hierarchy Designated role
					Adjusting communication strategies	Communicating directly Communicating informally Communicating indirectly Communicating matter-of-factly rather than emotionally Keeping to oneself
	Seeable	Speakable	<u>Unhearable</u>	Unmovable	Not seeing personhood Not receiving the message Ignoring the message	Power imbalance Negative family history Lacking sense of responsibility
	Seeable	Speakable	Hearable	<u>Unmovable</u>	Pro-child rather than pro-older adult Deeply rooted ideologies No traumatic events	
Connected Communication	Seeable	Speakable	Hearable	Moveable		



5.1.3. Intergenerational Family Communication about Dementia

This study investigated an overarching research question: how intergenerational family members communicate about dementia-related issues, with three sub questions. Guided by constructive grounded theory, the analysis theorized the problematic communication process as “disconnected communication”. It first filled research gaps by examining three family communication tasks in early stage dementia: how families make initial decisions to diagnose dementia, how they inform others about the diagnostic result and discuss the uncertainties, and how they deal with emotional arousal. From the side of the message sender, at the very beginning, when dementia symptoms gradually appear, it is important for family members to recognize the initial symptoms and the necessity of medical checkups. Only if both family members and the PLwD do not reject the illness claim can they successfully get the diagnosis. In this journey, adult grandchildren play a significant role because many of them are the first to notice their grandparents’ symptoms and suggest their parents take their grandparents for medical checks. Grandchildren also increasingly take their grandparents for dementia checks in the hospital.

Upon getting the diagnosis, because most diagnostic results are directly delivered to family members without the presence of the PLwD, family members have the right to decide whether to disclose to the patient. However, the level of disclosure is determined by how family members perceive the self-consciousness of the PLwD. Families that fully acknowledge the patient’s self-consciousness disclose it directly and empathetically without using euphemisms, whereas others that dismiss their self-consciousness tend to cover it up, frame it as memory loss, or blurt it out. In addition, many PLwD deny their dementia, even to insiders, due to stigma and concerns about saving face; thus, family members might not dare communicate about dementia with the patient. Compared to the rich research on the personhood and relationship aspects of communication, dementia research in China has seldom raised these issues, which are also largely neglected by family members and avoided by patients. Rather than co-constructing narratives about self, identity, and positioning through communication.



Chinese people share the assumption that dementia is an arrangement of fate, thus rendering family communication unnecessary in face of supernatural power. It can also be postulated that for people holding an interdependent view of self, it is more important to consider how others see them rather than how they see themselves. Thus, they see themselves through the lens of others and are more aware of the stigma and concerns about saving face and how that influences their identity, rather than the inner changes associated with dementia. These unseeable behaviors directly cut off the possibility to reach the next three steps of constructing connected communication: speakable, hearable, and movable. They also prematurely deprive the right of PLwD to engaging in family communication about dementia. However, consistent with previous research, in the current study, early stage PLwD largely retained their ability to feel and communicate, despite often requiring their communication partners to scaffold the conversation and be more patient. They also had the agency and desire to engage in decision making (Adler, 2010'; Hamann et al., 2011). However, despite the increasing recognition of dementia care triads among doctors, patients, and caregivers, patients are often left out of discussions regarding their diseases and conditions.

Moving toward prognosis communication, family members who see the problems in everyday care tend to initiate communication, mostly about the division of caregiving tasks, desired caregiving approaches, and nursing home decisions. Based on previous categorization, most family communication in this study also revolved around important decision making among family members, but I went beyond the caregiving dyads and involved the younger generation. However, rather than directly broaching these topics, the message sender tends to conduct self-censorship and adjust their communication strategies based on their evaluation of the speakability of the subject matter; in other words, whether what they said can be heard and lead to action by the message receiver. Thus, instead of pursuing effectiveness and persuasiveness as emphasized in Western culture, Chinese people often assess their relative positioning and designated role in the family to determine communication strategies that are consistent with the goals of Chinese communication: maintaining relationships,



respecting status differences, and preserving harmony. Family members with more equalitarian relations tend to communicate directly with less self-censorship, whereas family members who acknowledge the power relationship tend to censor themselves more severely and gradually lower the volume of their voice to the point that they are not heard by the message receiver.

However, communication about feelings and emotions is rare, both among family members and with PLwD, despite the diagnosis and prognosis often taking an emotional toll on the entire family unit. This is consistent with the previous literature showing a surprising lack of research on the emotional aspect of dementia communication. Most family members do not see the self-consciousness of PLwD, and PLwD tend not to reveal themselves to family members or have lowered ability to communicate—thus, their feelings are left uncommunicated. Nonetheless, a growing number of adult grandchildren are more attentive to the PLwD and their partner's feelings and emotions, given their designated role in the family and the desire to reciprocate care and love to their grandparents. They understand the PLwD's and their partner's needs to have an emotional outlet in the face of dementia and actively solve communication breakdowns between their grandparents and parents. However, when it comes to expressing their own emotional needs and seeking support, this is intentionally avoided and people usually turn to communicating about matters of fact. However, the suppression of personal feelings is less common among the grandchild generation, whose members have a higher desire for self-expression and are less likely to tolerate ambiguity in communication. Their prominent role in the family due to the one-child policy and neofamilism allows them to express themselves more freely without suffering significant consequences compared to previous generations. They also tend to feel more distressed and depressed when their message about their feelings is not heard by the message receiver.

From the side of the message receiver, despite the message being sent by the sender, such as initial communication about medical checks, prognosis communication, and



emotional communication, it can be left unheard by the receiver. Consistent with previous studies showing Chinese communication is receiver centered, this study showed that whether what has been said can make a difference depends on the listening skills and sensitivity of the listener. However, they may not see the personhood of the PLwD and take any message concerning them as irrelevant noise; they might not receive the message because it is too implicit or they do not try to interpret it, although it is highly difficult to examine the intention behind their inaction; or they choose to ignore the message due to the power imbalance between the message sender and receiver, negative family history, or a lack of a sense of responsibility. This also corresponds with previous research showing that although increased longevity means prolonged years of shared family life, it may also mean prolonged years of conflicts, especially in the context of dementia caregiving (Bengtson, 2001). Nonetheless, negative family history mainly influences the care provided by the middle generation, and it is less likely to affect the grandchild generation, which is often protected from knowing about and engaging in family conflicts. This can explain why in some families, grandchildren are more attentive to their grandparent's dementia compared to other relatives. In the current study, previous family conflicts and changing family relations in dementia care were also widely discussed and contemplated among younger generations to make sense of what is happening in the family (Chirico et al., 2021).

Finally, although the message can be heard by the receiver, some may still be reluctant to act. In line with neofamilism (Yan, 2016), the family can be pro-child rather than pro-older adult; thus, members might allocate more resources and attention to the grandchildren's development rather than the grandparent's disease. Alternatively, the receiver can have deeply rooted ideologies that are not easily challenged or changed. But most commonly, families are unwilling to take action related to scheduling medical checkups, improving caregiving, or providing psychological support because in early stage dementia, patients usually do not exhibit severe symptoms that require immediate intervention. Family members can easily hide it and initiate communication when it becomes more severe. This is in line with Silverstein et al.'s (1995) study showing that



extended kinship networks represent a latent rich resource of help in times of need. However, from the perspective of communication, it is not adequate to activate support only in crisis situations. Recognizing the importance of early detection, diagnosis, and intervention and providing care consistent with the preferences of PLwD, it is necessary for the three generations to get together and communicate dementia-related issues before a crisis comes up.

To the best of my knowledge, this is the first study to explore the communicative role and unique responsibility of adult grandchildren in dementia caregiving. Like a large body of evidence has demonstrated, the study also showed that the grandparent–grandchild relationship is a continuous source of mutual assistance, especially among those with a positive relationship history, regardless of geographical distance and socioeconomic changes (Michałek-Kwiecień, 2022; Scharf, 2016; Stelle et al., 2010). As the only child in the family, the grandchild’s relationship with grandparents is less influenced by patrilineal preference. Therefore, their emotional intimacy and higher education level make them more attentive to the initial symptoms of dementia and supportive of subsequent medical checks and treatment. Although previous research (e.g., Venters & Jones, 2021) suggested that adult grandchildren are more likely to share care responsibilities with the extended family, no one has examined their role in communication amid complex family dynamics among G1, G2, and G3. As illustrated in this process, adult grandchildren engage in major events during early-stage dementia and inevitably have a higher discursive power in the family. Their role is no longer restricted to secondary tasks, as suggested in previous research, but rather is extended to communicating more salient issues with family members as capable and equal communicators (Tatangelo et al., 2018). They also seek strategies to retain the independence and psychological well-being of PLwD as much as possible (McNaney et al., 2017). But they often experience conflicting intergenerational views, such as different understanding of dementia and caregiving approaches due to different intergenerational value orientations determined by historical context. Nonetheless, as they enter established adulthood, it is highly likely that they will exert greater influence



on family decisions during the long haul of the disease journey.

In this study, I also took the further step to examine the influence of Chinese communication culture on family communication about dementia across three generations. I went beyond the traditional questionnaire investigation and explored the intricacies and subtleties of Chinese communication through in-depth interviews and fieldwork observations. This filled research gaps regarding contextualized health communication in dementia care and nonimmigrant families. Based on the literature review and what has been found in the current study, dementia communication in the Chinese context can be summarized by four key characteristics. First, message senders are highly accommodative to achieving the communication goals. Second, the message delivered is implicit and subject to the interpretation of the message receiver. Third, communication is hierarchical, with little communication involving people in less powerful positions. Fourth, communication is commonly activated in times of crisis rather than proactively. These characteristics pose barriers to actualizing connected communication, which is fundamental to dementia care. However, it is evident that the three generations hold different views of communication and practice speaking rules differently. Whereas members of the older generation (G1) seldom express themselves and tend to obey authority, members of the middle generation (G2), in the transition from the old to the new era, have a more independent view of self and thus, embrace more open communication. Members of the younger generation (G3) have experienced the most rapid societal changes and are the most likely to express themselves and even challenge authority. Despite these different value orientations, the current study showed some families shared connected communication across generations with more egalitarian intergenerational relationships, emphasizing emotional bonds rather than submission to the patriarchal norms, which indicates an intimate shift in modern Chinese families. Compared with previous examinations of health communication in the fields of cancer, schizophrenia, and end-of-life in Western societies, this study revealed the nuances of Chinese intrafamilial communication across three generations in the early stage of dementia, which can significantly benefit its long-term



management and the well-being of the entire family unit.

In summary, this study has successfully filled three research gaps: the family communication process in early stage dementia, the role of adult grandchildren in family communication, and how Chinese culture shapes family communication across three generations. In the next section, I will demonstrate the theoretical contributions of this study.

5.1.4. Theoretical Contributions

This study has made significant theoretical contributions to understanding the abstract process of intergenerational family communication about dementia. Previously, I examined three theoretical perspectives in family communication: family system theory, the theory of intergenerational ambivalence, and communication accommodation theory. Because they are not sufficient to predict family communication about dementia accurately, this study adopted inductive reasoning and theorized about the communication process. The abstraction of disconnected communication, with its four subsuming categories, has the potential to integrate these three perspectives. First, although family system theory explains how family emotions patterns develop and are transmitted across generations, this study revealed that the family communication pattern regulates emotional expression across generations, with historical and cultural roots in the family hierarchy and designated roles. Several key concepts in Bowen's theory were discussed in the current study, such as triangles, emotional cutoffs, and multigenerational transmission processes. Going beyond managing family dysfunction through the lens of emotional symptoms, this study contributed an alternative intervention strategy by solving disconnected family communication patterns to address ambiguities associated with dementia. Second, this study further proved the coexistence of intergenerational solidarity and conflict in Chinese families; in other words, ambivalence. Such ambivalence is not only generated from the family structure but also from conflicting views, both of which are important to understand in contemporary intergenerational relationships. Furthermore, this study contributed to



this theoretical perspective by adding the dimensions of ambivalence. The underlying psychological process between the selection of seeable or unseeable, speakable or unspeakable, hearable or unhearable, and movable or unmovable reflects ambivalent feelings that arise from complex family dynamics. Thus, it is important to address ambivalent feelings by empowering family members to move from unseeable to seeable, from unspeakable to speakable, from unhearable to hearable, and from unmovable to movable in social work practice. Finally, going beyond accommodative communication behavior in a high-context culture, this study identified an analytic hurdle to distinguishing message sender and receiver while scrutinizing overaccommodation, underaccommodation, and nonaccommodation behavior, especially in view of their power relations. This study also examined in more detail the accommodation strategies on the continuum from direct communication to no communication and their relationship with the level of certainty and self-censorship. In summary, the theorization of disconnected communication has substantially contributed to existing theoretical frameworks, with a stronger analytic reach in explaining family communication about dementia.

Although dementia can be challenging for everyone involved, maintaining connected communication among family members can make a significant difference in the disease journey. Thus, it should be a focus of future social work interventions to improve the well-being and quality of life for families living with dementia, which will be discussed in the next section.

5.2. Practical Implications

Social research always seeks to make sense of the world from particular perspectives to inform our thinking, decision making, and action taking. This study has important implications for PLwD, family members, and societies deeply influenced by Confucianism and familism. This study upholds two core values in dementia care. The first is the benefit of early detection, diagnosis, and intervention, and the second is open prognosis and emotional communication involving early stage dementia. They can be



actualized with connected communication in the family if they see each other, vocalize the problems, hear what others say, and engage in proactive action. However, in reality, this can be challenging to implement because family members tend to put the invisibility cloak on themselves and each other during communication. Therefore, it is necessary to empower adult grandchildren, adult children, and PLwD in micro-level social work practice and consider the Chinese culture specificities.

First, adult grandchildren should be empowered to speak with communication and persuasion strategies. With their attentiveness to the initial dementia symptoms and more advanced knowledge in dementia care, they are in the right position to broach these communication topics when other family members do not have the ability to detect or vocalize them. However, in the Chinese communication context emphasizing hierarchy and designated roles, they should be empowered to adopt appropriate communication strategies that can maximize the possibility of being heard and moving family members to act. Additionally, the message should be clear and explicit, leaving no room for the message receiver to ignore, misinterpret, or find it difficult to understand. They should also learn how to emphasize the consequences of not handling dementia issues early so that other family members can no longer neglect them and wait for crisis-induced communication. Despite their prominent role in the family and higher desire for self-expression, grandchildren should understand that they are still regulated by family communication rules. Social workers should pay more attention to adult grandchildren in dementia care, given their increasingly high level of involvement.

Second, adult children should be empowered to see the personhood of the PLwD and hear the messages delivered by the adult grandchildren. On one hand, they should not deprive the PLwD's right to know their medical conditions and respect their autonomy in decision making despite their cognitive deficits. Before disclosing dementia to PLwD, it is desirable for doctors to have a one-on-one consultation with PLwD, which allows doctors to get the patient's informed consent to learn whether they want to be informed and whether they want their family members to know. If PLwD agree that they or family



members should be informed, a truthful disclosure with a full explanation should be provided without any euphemisms. Rather than covering up the diagnosis to protect PLwD from potential harm, it is more desirable to adopt an alternative by way of providing realistic hope. Although dementia is incurable, there is significant variation in disease manifestation and progression, and effective nonpharmaceutical treatment can delay the progression or maintain the current level of dementia.

After obtaining the diagnosis, they should engage PLwD in prognosis communication to learn about their care preferences and openly discuss how caregiving tasks should be allocated among multiple caregivers and their future care arrangements, rather than waiting until problems arise. Situated in this interdependent culture, dementia is not only an incurable disease but also has strong social implications that can undermine the patients' social life. Thus, the disclosure of the diagnosis should be followed by outlining a management plan to narrow not only pharmacology-based strategies, but also nonpharmacological interventions that can be provided by family caregivers and social organizations. The PLwD's need to maintain social connections should be fully recognized, which is not only cognitively beneficial but also satisfies their core needs as interdependent people. On the other hand, as the sandwich generation, adult children often encounter greater challenges in balancing caregiving, work, and other commitments. Therefore, they should recognize the role of adult grandchildren as capable, independent communicators and involve them in the disease journey to improve care together. Rather than being dominant decision makers, adult children can connect multiple generations and collaboratively arrange care throughout the family unit, which is also beneficial to PLwD because they will have more communication opportunities with extended family.

Third, early stage PLwD should be empowered to see their rights and autonomy in the medical settings and speak their needs and feelings. Social workers should educate people with strong dementia stigma that a medical diagnosis is something they have, not who they are, thus making the diagnosis more acceptable and visible in the family.



When they still largely retain their communication abilities, they should be made aware of the need to communicate with their family members about their care preferences and financial arrangements, rather than waiting until problems arise or their communication abilities falter. Nevertheless, such communication can be difficult to implement because formal family meetings are not culturally practiced in China. Thus, this requires the assistance of social workers to facilitate communication or become a mediator to deliver hard-to-say messages. In addition, they should be encouraged to share their feelings and emotions if they want to be understood, particularly with grandchildren if they are emotionally close. It is of utmost importance to empower PLwD to see themselves and not let the dementia label replace their identity. This will enable family members to speak with them openly and learn about their needs in the long haul of the disease journey to provide quality care.

In summary, with joint effort that empower adult grandchildren to be speakable, adult children to be seeable and hearable, and PLwD to be seeable and speakable, connected communication can be established, eventually making family members movable regarding desired actions and changes to benefit the entire family unit.

5.3. Reflection

In this section, I first reflect on the rigor of the research process with lessons learned. Then, I discuss several questions raised during my fieldwork that can be heuristic for future research.

5.3.1. Reflection on the Research Process

As mentioned in Chapter 3, this qualitative study adopted the trustworthiness, auditability, credibility, and transferability (TACT) framework to ensure scientific rigor. In this section, I first address how I achieved scientific rigor in the research process. Then, I will show how I immersed myself in the research practice and the challenges I encountered.



Trustworthiness

Trustworthiness requires the researcher to examine how they established confidence in the quality of the investigation process and the outcome of the research. It includes three dimensions: sources and quality of data, dependable outcomes, and researcher experience. The current study involved PLwD, whose narrative may not be accurate due to their cognitive problems. Therefore, in the fieldwork, I reached out to family members of PLwD and conducted interviews with them to examine whether what has been said by the PLwD is correct. In this way, I also investigated what the PLwD could memorize and why they remember certain things rather than others, thereby deriving insightful analysis. Nonetheless, what has been said by family members may not always be dependable either, because I have a power difference with the interviewees. Therefore, I conducted interviews with social workers, who have rich knowledge about the PLwD's experience in the past several years and had more time to establish rapport with them. With accounts from PLwD, family members, and social workers, I triangulated the authenticity of the interview content and reflected on inconsistencies. With multiple sources of data, I ensured the findings were situated in the views of participants and yielded dependable outcomes.

In addition, I found empathy to be a key factor that strengthened the interviewees' trust in me and enhanced data quality. Originally, I perceived that my role as a researcher would cause a power imbalance between me and my interviewees. However, in the research process, I sensed their genuine interest in talking with me because communication is such an underexplored area, not only in academic research but also in everyday dementia care. The interviewees rarely had the opportunity to talk about the communication aspect of their caregiving journey with someone who can understand their dilemma empathetically. Many caregivers were even reluctant to end the interview and expressed a desire to talk more. Especially when I reached out to the grandchild generation through social media, I originally assumed it would be highly difficult to recruit in this way, because they did not have any knowledge about me and the research topic can be sensitive. But to my surprise, the grandchildren of my age



readily accepted my interview invitation. In our conversations, I felt less like our relationship involved researcher and interviewee, but rather two friends who shared the common worries of our generation and can empathetically understand each other. The common worry was not only the barriers they face while trying to communicate about dementia, but in fact applies to our everyday life in and outside family. Thus, despite not knowing each other before, we developed a rich, heartfelt exchange that may have never happened with their family members. Some of them also asked me to send them my thesis once it is ready because they are interested in learning more about other people's experiences with this subject matter. I was deeply grateful for their trust and giving me the opportunity to learn about their family life that is traditionally not shared with outsiders. It is a privilege as a researcher, and I have continuously told myself to use these interviews well and ensure my analysis truthfully reflects their experience and makes scientific contributions that can help the larger community of families dealing with dementia.

Reflecting on my positioning in the research, I acknowledge my prior assumptions that most dementia communication would be problematic or nonexistent. Although I have not had dementia-related experience in my family, I did witness the challenges my family faced regarding my grandmother's terminal care, such as being unwilling to challenge the decision-maker role of my uncle or provide him with any care suggestions, but only to follow what he said. As a granddaughter, I am not the key care provider and I do not have a say in the family. In addition, we never asked my grandmother about her care preference when she was still able to talk several years ago. All I could do is to think of entertaining activities my grandmother could do while bedridden, although these activities may not be what she wanted. This experience before my master's study began was a key driver for me to explore the communication aspect of care provision and the experience of younger family members like me when dealing with their grandparents' disease. Therefore, during the pilot interviews, I assumed such communication would be problematic, but what I found is that there are indeed families that openly communicate across three generations, even with the PLwD. That was when



I recognized my personal bias and knew I should not bring my negative experiences into the investigation process. Therefore, I set aside my personal opinions and maintained neutrality in the following three rounds of interviews to ensure trustworthiness.

Auditability

Auditability requires the researcher to document and describe the research process fully for future audits. It includes three dimensions: transparent data collection, systematic data analysis, and data verification. Therefore, I have kept records of all audio recordings, conversations on social media, and pictures to ensure the transparency of the recruitment and data collection process. In the analysis process, I also carefully and systematically saved the fieldnotes, memos, codes, and drafts to document the thought process that led to my conclusions. These efforts can ensure an easier verification process by either other researchers or research participants.

Credibility

Credibility requires the researcher to ensure the findings are dependable, relevant, and congruent. Therefore, I verified my findings with the source of the data, a process also referred to as member checking. In the research findings chapter, I also used direct quotations to substantiate my analysis. Because all interviews were conducted in Chinese, all quotes featured in this study were translated with ChatGPT. Because manual translation is inefficient, such translation traditionally would be conducted with translation tools such as Google Translate. However, with the prevalence of generative artificial intelligence, more accurate translation is possible. Therefore, I compared the translation between Google Translate and ChatGPT and was surprised to find that although both can provide credible translation, ChatGPT was better able to understand what the quote was about and its context, thereby providing a more context-specific translation in terms of family communication about dementia. In addition to providing translation as requested, ChatGPT also offered comments about the quotes that reflected its view on what is going on. For example, when I entered a quote about a family



member carelessly blurting out the dementia diagnosis, ChatGPT commented: “It is important to approach discussions about dementia with sensitivity and empathy, especially when speaking to the person directly affected.” Such interactive communication with ChatGPT not only provided me with more insights and perspectives, but also offered an opportunity to communicate with others about my research and generate new ideas. Thus, I translated all the quotes with ChatGPT and considered its feedback about the quotes to improve my analysis. Nonetheless, no direct translation is perfect, and there are always nuances AI cannot capture. Therefore, I double checked every translation and modified it accordingly to ensure it accurately reflected the message delivered in the original Chinese.

Transferability

Transferability requires the researcher to situate the study in a particular context so that it can be transferred to other similar settings. At the beginning of the findings section, I delineated the cultural, social, and historical contexts of this study and provided details on the demographic information of the study participants. Therefore, other researchers can assess the integrity of the research outcomes and transfer findings to other contexts and study samples. As discussed in the methodology section, this study used grounded theory to produce a substantive theory addressing delimited problems in intergenerational dementia communication. Nonetheless, with the given context and study sample, the theorization process, categories, properties, and codes can apply to multiple substantive areas, such as family communication about other diseases, conflict resolution, and cultural change, which has the potential to develop into a formal theory with more evidence.

Immersion

In addition to ensuring scientific rigor, the grounded theory approach requires researchers to immerse themselves fully in the interviewees’ world. While conducting the analysis and writing down my thoughts, my emotions flowed with the participants and were triggered by the connotations of their experiences. While exploring the



concept of the interdependent self and how it affects interpersonal communication behavior, I felt exhausted due to constraints imposed by China's collectivist and high-context culture. The complexities of their interactions regulated by social norms made me feel tired, but it was also a mirror for myself because I am also living in this context. Although I do not have family members with dementia, writing this section prompted me to reflect on my interpersonal struggles using the lens of interdependence. To my surprise, after taking it into account, I realized what I am struggling with and how it can create conflict, indeed solving some of my puzzles and making me feel relieved. Therefore, although revealing the psychological process of the interdependent self is painful as a writing practice, I hope my analysis can prompt families with dementia to rethink their experiences with new perspectives and relieve their burden through enhanced understanding of the interpersonal relationship.

In comparison, while writing about the shortcomings of the current medical system and the deprivation of patient autonomy, I felt angry, and my writing speed was substantially quickened due to its strong argumentation nature. I was shocked when I discovered these apparent violations of human rights, and I felt a strong urgency to bring these issues to the attention of the public and hope they can be addressed through social work practice. On that day of writing, I was emotionally aroused and had an unprecedented desire to seek social justice for marginalized people.

Later, when I began to write the chapter on disconnected communication, especially the section on censorship of speakability and adjusting communication strategies, I was deeply touched and even felt depressed. This occurred not only because it applies to my life, but also because I realized how little understanding we have of our own communication practice and how often we misunderstand others unnecessarily. I even wanted to go back into the field and tell my interviewees not to communicate in that way. This emotional feedback during the analysis and writing stage was unexpected, but it also made me realize how deeply I, as a novice to qualitative research, had immersed myself into this research topic and embraced the interviewees' world, even



to the extent that it reshaped my worldview.

Challenges

The complexity of carrying out this research was far greater than what I expected while writing the proposal. Originally, I planned to recruit all participants online, but this turned out to be unfeasible in the case of dementia. Therefore, after struggling with online recruitment, I decided to conduct fieldwork, which was a big step away from my original plan. With the support of my supervisor, a professor from Shanghai, and my peer classmates, I got in touch with a dementia care nongovernmental organization and became a volunteer in a very short of time. I went into the field immediately to build rapport with participants, provide services, organize groups, and conduct interviews. Luckily, it turned out to be a wise decision, because I not only had the chance to meet more potential interviewees, but I also fully immersed myself in the patients' world. I could talk with them about their experiences regularly, observe their home environment and daily routine, and observe their social interactions with others. Thus, what I obtained extended far beyond the 1-hour interview, revealing a holistic picture of these people and their interpersonal and social environment. In addition to the topic of communication, they also raised other common concerns that I had never considered before, which are discussed in the next section and could be further explored in my academic career.

In this research process, things usually did not happen in the way I expected. For example, most older people in Shanghai speak Shanghainese; thus, I had to ask their family members to translate and seek help from my Shanghai friends. Once, an interviewee suddenly got sick before our scheduled meeting, so I lost a valuable case and felt deep regret for not interviewing him earlier. There were also times when the PLwD spoke to me in Mandarin, but I could not understand a word. Despite these challenges, there were also pleasant surprises, such as their unconditional trust and opportunities to meet potential interviewees via various activities. Although I was shy to reach out during the first encounter and I did get several rejections, most family



members were supportive and passionate about talking with me. At the end of my fieldwork, I had established strong rapport with some families, and we have maintained contact and greet each other occasionally. This is the first anthropological approach I have adopted in my research journey, and I was amazed by its power in connecting human experiences that we share but often remain hidden and insulated. This experience has greatly inspired me to engage with vulnerable groups more deeply and meaningfully in the future.

In addition, the process of abstracting ideas and theorizing my findings has been painful. Beyond individual experiences, researchers have to break through the ordinary, challenge the taken-for-granted understanding, and explore larger analytic stories. Through numerous discussions with my supervisor, other professors, and peer classmates, I gradually peeled back the experiences described in this study layer by layer to define the phenomena in abstract terms and demonstrate the abstract relationships. Additionally, the coding process was challenging, because it required me to use lifeless language that better fits the academic and social world than the participants' expression. In this process, I had to constantly ask myself: Am I seeing this in the data because of my own personal experience? Is the idea arising from the interviewee or is it being directed by me? What are they trying to say, and what are they remaining silent about? Thus, most importantly, I had to tolerate ambiguity during the iterative process of theorizing, an ability that I had to train myself to develop. My previous experience in quantitative research had made me more accustomed to getting the results immediately after running the data analysis. Thus, the coding and analyzing process took longer than expected, and I made the decision to postpone my thesis submission date by a month to ensure more time for analysis and writing. Looking back, the journey of exploring this research topic was full of inspiration, surprises, and struggles, but I eventually got through the process. I feel proud of myself and deeply appreciative of those who generously offered help that enabled me to open the door of a new world.



5.3.2. Reflection on the Research Topic

In addition to the prior reflection on the research process, in this section, I discuss several questions raised during my fieldwork. Although these topics are not necessarily related to communication per se, they indicate a larger story about person-centered care, intergenerational solidarity, and government support.

The first consideration is that despite constant advocacy for person-centered care, it remains unclear how family members should execute it and what practical challenges they face in this process. With a patient-centered approach, it is important to identify goals and priorities in the context of the patient's world and develop a mutually agreeable plan of management. This requires family members to take the perspective of the patient and fully respect their autonomy. However, it is difficult to ensure that the family members are empathetically going inside the world of the PLwD, especially when they perceive to have done so but in reality, they are still in their own world and doing things they perceive as good for the PLwD but only benefit them. During my fieldwork, many family caregivers told me about how they coped with dementia symptoms, such as forgetfulness, suspicion, repetition, and bad temper. Coincidentally, most of them, even the social workers, pointed out a common solution, which is to infantilize the PLwD. They would coax them whenever they were unreasonable and always agreed with them even if what they said is wrong, never trying to correct or argue with them. They never treated them based on their original identity, like grandmother, mother, or wife, but only as a child in kindergarten with a lower level of intelligence. This strategy turned to be effective in calming down PLwD because it directly reduced negative triggers; thus, family members said it was in the best interest of the PLwD. It also helps relieve the psychological burden of caregivers because they do not need to exhaust themselves in reasoning or try to communicate with PLwD as with normal people. Therefore, infantilization is seemingly a win-win situation. However, is it truly what PLwD need? Is it in line with the requirement of person-centered care?



You cannot treat her as if she were your grandmother or mother. No. Just treat her as if she were someone younger than you, like a child in kindergarten. (Partner 2, 69)

After coaxing her, then I felt mentally relieved. (Partner 2, 69)

Alternatively, one adult daughter rejected this idea of infantilizing her mother and intentionally trained her to logically think and learn about her reality, even if she could not fully understand. The interviewee acknowledged that dementia could cause confusion for PLwD, and if she as the caregiver kept agreeing what her mother said and adding to her confusion, her mother would decline more quickly and eventually lose consciousness. She faithfully enacted the principle of person-centered care by recognizing her mother's personhood and the need to maintain her consciousness through proactive training, instead of agreeing with her confusion or mistakes to reduce her own trouble. This points to a common concern in an interdependent culture: People holding an interdependent view of self attempt to read the minds of others and take it as a genuine reflection of the reality. Although this is culturally expected, in the context of an incurable and complex disease such as dementia, it can be problematic and cause unexpected relational consequences. Similar to previous research arguing that the surrogates of PLwD cannot consistently and accurately identify their values and thoughts (Black et al., 2013), an adult grandson in this study further pointed out that if family members only do what they think is best for the patient, this is in fact a selfish act. Therefore, to overcome the tendency to regard what family members perceive as the reality, it is highly important to empower patients to express themselves, especially in the early stage of dementia. Given the patients' declining communication abilities, it is also important for the communication partners to scaffold the conversation and ensure enough time and patience. However, during my interviews with PLwD, some caregivers intentionally or unintentionally took away their opportunity to communicate. This might have been because the PLwD spoke slowly and the caregivers did not want it to affect the interview efficiency, they did not want them to say something wrong in



front of an outsider, or simply because they knew the answer but the PLwD may not. How to avoid overcompensating for PLwD's communication loss is worth consideration in future research. In any case, the caregivers did not enter the patient's world or understand what they needed, despite perceiving that they did. Gradually, PLwD held themselves back in the conversation and became disassociated from their own subject matter.

When I communicate with her [PLwD], I will emphasize some practical things. However, I noticed that unlike me, [some caregivers] tend to use whites lies. I believe that certain things should not be overly kind. You need to let them [PLwD] know the current situation, even if they don't understand it. You should encourage them to learn and think. (Child 4, 46)

Dementia is a confusion of space, time, and even logic. If you don't exercise her [PLwD] on purpose, it's highly likely that she will become more confused and not know the situation she is in at all. (Child 4, 46)

Everyone in the family is selfishly trying to do what they think is good for the other person, and this is the biggest problem. (Grandchild 3, 27)

On the other hand, patient autonomy, which includes the right to know (or not) and make informed decisions about therapeutic interventions, can also be challenging to implement. In reality, it can be difficult to have advanced awareness and delineate the boundaries between intentional and unintentional denial. On one hand, some family members only discover the patient's strong denial attitudes toward dementia after getting the diagnosis, leaving them no room to communicate in advance, in a decontextualized environment, to discuss care preferences, whether about dementia or another issue. On the other hand, as I observed in the fieldwork setting, it can be very tricky to determine whether PLwD are intentionally denying their dementia diagnosis or they have in fact forgotten their diagnosis or even experienced memory decline



because of their severe forgetfulness. Researchers discovered that anosognosia, a term describing the state of PLwD's unawareness of their cognitive deficits, is a common but underrated symptom that significantly influences family dementia caregivers (de Ruijter et al., 2020). Family members may mistake unawareness as unwillingness to talk about dementia and do not dare to carelessly broach the topic, even though the PLwD may accept such communication.

The second consideration is whether the generations are getting closer or further apart. Although research evidence has shown greater family interdependence with increasing multigenerational households in modern Chinese families, it is worth thinking about whether such solidarity has only been strengthened among nuclear families, with generations in fact growing apart and causing latent sources of help not to be activated even in times of need. During my fieldwork, a spousal caregiver, aged 86, reflected on how much caregiving support he could obtain in the 1990s from his extended family. However, as time passed and his relatives from the older generation passed away, he perceived it as increasingly difficult to seek support from the younger generation due to scattered geographical locations and the increasing independence of the nuclear family. He restricted the dementia issue to his own nuclear family and seldom sought support from other relatives, which he perceived as an unwanted burden to them. The growing attention to the nuclear family also made it difficult to care for relatives far from this core circle of intimacy. Similarly, another adult child caregiver mentioned although she has an older sister, they both agree to not to bother each other and only try to manage their respective nuclear families well. Beyond the nuclear family, they did not have spare time and energy to help each other; thus, they upheld this principle of independence even in the face of their mother's dementia. In future research, it is worth examining whether the nuclear family is increasingly independent and to what extent people keep salient issues such as dementia away from their broader circle of family members.

Now my own daughter is the only one left. My brothers and sisters have all passed

away. I only have the younger generation. The relatives we meet now are from the next generation; some are in Beijing, some are in Dalian. There's nothing they can do. Everyone just lives their own lives, managing their own affairs. (Partner 3, 86)

Everyone in our family is very independent, managing their own affairs. "Don't bother me, just focus on living your own life well. I don't have the ability to help you, that's just how it is. The most important thing for you is to take good care of your own family." We are independent, very independent. (Child 4, 46)

The third consideration is whether government subsidies can sufficiently support the needs of integrated dementia care. Undoubtedly, the middle generation (G2) has greater time constraints in providing dementia care and needs more integrated familial, social, and institutional support. Family members are less likely to take full responsibility for dementia care, as favored by traditional culture (Zhang et al., 2019). In this fieldwork, I touched on this topic with some interviewees in Shanghai, and they described their experience of receiving government support. Although this does not represent an overview of dementia-related policies in China, which is not the focus of this study, their experiences offer insight into the barriers they face in obtaining seamless social support. In terms of community-based care, although in recent years, dementia care services have significantly grown in Shanghai, they do not cover all neighborhoods and community-based activities are usually provided every other day. Therefore, caregivers usually have to move across neighborhoods to attend the activities, making transportation a barrier to seeking social activities. Some family members also have to move PLwD across different organizations to ensure they have somewhere to go every day, rather than providing full day care every other day at home. This also has negative effects for PLwD because they have to become familiar with a new environment every other day, which can be difficult for PLwD. Nonetheless, the implementation of long-term care insurance in China has provided family members with a new solution, allowing them to hire in-home helpers to assist with daily activities several times a week. However, some family caregivers said it takes a long time to qualify for such



service and they later found the PLwD resisted this formal care, causing them to relinquish such service. While community-based service may not fully capture the intensity of dementia care, in-home service may fail to match the patient's unique caregiving needs.

What to do in the future? Actually, it will rely more on government subsidy and community support. There's no way around it, because the family components are becoming smaller and the structure is also shrinking. (Child 4, 46)

In addition to service support, caregivers also encountered challenges in applying for government subsidies. For example, one caregiver went back and forth to apply for a dementia subsidy but was rejected. Eventually, he found out that the people responsible for allocating the subsidy did not know Alzheimer's disease is a form of dementia. After he clarified this point, they requested that the caregiver provide the formal diagnosis record, but when he went to retrieve it from the hospital, he was told the system had been upgraded and all previous records were lost. Therefore, he took his mother for another diagnostic test, whose fee was equivalent to 5 months of the subsidy he was seeking. After going through all these procedures, the subsidy was announced to the neighborhood publicly for 2 weeks to see if anyone had concerns. Because dementia is associated with stigma, the caregiver also questioned the necessity of doing so and the potential harm it may cause to his mother. Although these are only individual experiences, there is an apparent gap between informal and formal care that can be addressed through policy advocacy.

In summary, in this section, I first presented how my research findings have been supported by a rigorous research process and how I immersed myself in the research setting to increase the study's authenticity. I also reflected on the challenges I encountered while executing the research proposal. Furthermore, I reflected on several considerations revealed during my fieldwork, such as how we should understand person-centered care, whether generations are getting closer or further apart, and how



policies can support families with dementia. These reflections can be insightful for future research.

5.4. Strengths and Limitations

The current study has three major strengths. First, rather than focusing on one intergenerational relationship, such as between aging parents and their adult children, a common formula in the existing literature, this is the first study to capture the younger generation's role in the family disease journey and highlight their willingness and ability to be further involved in family communication and important decisions. As an understudied area both nationally and internationally, the dynamics among three generations should be of interest to societies with multigenerational family environments, such as Southern Europe, India, and South America. Second, although previous research has indicated positivity bias such that families with more positive relations can be overrepresented, this study sufficiently captured families experiencing worsened family relations after getting a dementia diagnosis through fieldwork recruitment. Third, the current study involved early stage PLwD and their family members, responding to the global call to involve PLwD and caregivers in dementia research (e.g., China, Scotland, G7, World Health Organization) and make policies more relevant to their interests. This is particularly meaningful for early stage PLwD, who largely retain cognitive and communication abilities to make decisions. Although it can be difficult to check the authenticity of what people with dementia say in an interview due to their memory loss, in this fieldwork, I double checked my findings with social workers by thoroughly learning the background and disease experience of each PLwD. I also interviewed their family members whenever possible. Thus, it becomes easier to distinguish what the PLwD had memorized, thus increasing the reliability of the research findings.

This study also had several limitations. First, because in-depth interviews rely on recalling prior behaviors, emotions, and feelings, they may result in retrospective bias of experiences in early stage dementia. Notably, this research recruited families with



middle- to late-stage dementia due to the low diagnosis rate of dementia in China, which may increase the difficulty among interviewees to recall what happened many years ago. Nonetheless, adding families with middle- to late-stage dementia enabled me to observe the increasing communication intensity from early to later stages due to the preference for crisis-induced communication, adding valuable insights. Second, due to different economic development levels, there is a clear rural–urban divide in China, with inequalities in access to care and health care expenditures (Ying et al., 2020). Medical resources (e.g., hospitals, doctors, beds) are also unevenly distributed among east, middle, and west regions (Chai et al., 2020). Families in the most and least developed areas may have significantly different access to dementia education and diagnostic screening resources. Although this study also recruited families from rural areas, the fieldwork was conducted in Shanghai, one of China’s most developed cities; thus, many interviewees had relatively better socioeconomic status and access to health care resources. Due to time constraints, I could not reach out to more interviewees from relatively more deprived areas, which should be a focus of future research. Third, women were overrepresented in the sample for two reasons. According to the Qian-Gua Database, 90.41% of Xiaohongshu’s active users are women, and the main user group is urban white-collar and elite career women (Guo, 2022). Therefore, the initial recruitment strategy through social media may have resulted in an overrepresentation of female and urban residents. While recruiting interviewees in the daycare center, I noted significantly more female participants than male participants, also resulting in the overrepresentation of female interviewees. Fourth, as one of the focuses of this study is the perspectives and behaviors of the younger generation, involving only six adult grandchildren interviewees may limit the diversity presented in the Chinese young population. Nonetheless, because grounded theory does not pursue generalizability but instead theoretical sufficiency, this current sample adequately addressed the research questions. Fifth, during the second and third rounds of data collection, I did not include PLwD to verify my research findings since I had already left the field while conducting data analysis. Due to the cognitive impairment, it was difficult to talk with PLwD about the research findings using online chat, thus they were not included. Future research



can narrow the sample to families with early stage dementia, balance the sample in terms of the urban–rural divide and gender representation, interview and re-interview more adult grandchildren and PLwD. Future research can also test the ‘connected communication’ model using quantitative methods, which was not feasible in the current study considering the limit of the scope.

5.5. Conclusion

In conclusion, this study thoroughly explored the family communication process about early stage dementia across generations. Guided by constructive ground theory, this process has been theorized as disconnected communication with four subsuming categories—unseeable, unspeakable, unhearable, and unmovable—to explain the studied phenomena. Situated in a certain cultural, social, and historical context, these study results call for connected communication among family members through mutual awareness of disease conditions, active engagement of the adult grandchild generation, open prognosis discussion, and emotional communication among family members and with PLwD. This study can also inform other Confucian societies with aging populations, such as Japan, Korea, and Vietnam, on how adult family members can communicate more effectively regarding health issues, especially age-related diseases and conditions. Although managing dementia can be challenging, embracing connected communication in the family can make a significant difference throughout the disease journey.



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Appendix I: Interview guide

1.1 Interview guide with adult grandchild

	孙辈	Adult grandchild
确诊前的家庭沟通 Family communication before diagnosis	1.先讲一下祖辈确诊认知障碍症前后的具体情况。 2.你是什么时候发现祖辈有认知障碍症的症状的？回忆一下自己当时的想法和感受。当时你有和其他家人或者朋友聊过你的想法和感受吗？如果有的话，你是如何开启这个对话的？你们聊了什么，怎么聊的？(提示：地点，时间，家人，话题，感受) 3.是谁决定带祖辈去医院检查诊断？为什么？其他家人的态度是什么？寻求医生帮助的前后都发生了什么？ 4.如果是由你决定带祖辈去医院检查，为什么？有哪些事情/哪些人影响了你的决定？ 5.能说说你和祖辈/父母的关系怎么样嘛？确诊前，你们多久联系一次？是打电话/面对面/视频通话还是用其他方式联络？这个病对你们之间的关系和交流有什么影响？	1. Tell me about what happened when you first learned that your grandparent was first diagnosed with dementia. 2. When did you first notice your grandparent having signs of dementia? If you recall, what were you thinking and feeling then? Have you talked to anyone about your thoughts and feelings such as with your family or friends? If so, how did you initiate the talk, and what and how did you talk about them? (Prompt: place, time, people, topic, affect) 3. Who decided to take your grandparent to the hospital seeking a formal diagnosis? Why? What is the attitude of other family members? What happened before and after seeking medical help? 4. If the diagnosis was initiated by you, why was this? Did anyone/anything influence your decisions? 5. Can you tell me about your relationship with your grandparent? How often did you communicate with him/her before diagnosis? In-person, by phone, via video call, or other means? How has their diagnosis affected your relationship and communication with him/her?
确诊后的家庭沟通 Family communication after diagnosis	1.确诊后你有什么想法和感受？你有和家人或朋友聊过吗？如果有的话，你是如何开启这个对话的？你和他们聊了什么，怎么聊的？(提示:地点，时间，家人，话题，感受) 2.你的祖辈知道自己有认知障碍症吗？如果知道的	1. Can you tell me your thoughts and feelings when you learned about the diagnosis? Have your thoughts and feelings changed? Have you ever talked to your family or friends about this? If so, how did you initiate the talk, and what and how did you talk about them? (Prompt: place, time, people, topic, affect)

话，最初你们家人是怎样告诉他的？如果不知道，你们家人又是如何做出这个决定的？

3.你或家人是怎样将确诊的消息告诉其他亲人和朋友的？告诉了谁，为什么决定告诉他们，他们的反应又是怎样的？

4.确诊后，有哪些其他家人参与进来？他们都做了什么(探究家庭结构，家庭关系，以及潜在的矛盾)？

5.认知障碍症给祖辈和家人带来了哪些变化？你和父母、祖辈有没有沟通过这些变化(例如病情，家庭关系)？

2. Does your grandparent know he/she has dementia? If yes, how did your family inform him/her initially? If not, how and why did your family make this decision?

3. How did you or your family tell other relatives and friends about the diagnosis? Who was told, why did you decide to tell them, and what was their reaction?

4. Who in your family was involved in the disease journey after the diagnosis? What did they do? (Focus on family structures, family relationships, and potential conflict)

5. How has dementia affected or changed your grandparents and family members? Have you discussed these changes (e.g., illness, family relationships) with your parents and grandparents?

目前,家人之间如何沟通情绪

Currently, how family communicate about emotions

1.回想最近一次，你和祖辈之间最快乐的一次相处(>30分钟)。当时发生了什么事情，在哪里，有哪些人，什么时间，你们都聊了什么？

2.回想最近一次，你和祖辈之间让你最痛苦/沮丧/焦躁的一次相处(>30分钟)。当时发生了什么事情，在哪里，有哪些人，什么时间，你们都聊了什么？你或家人是用什么方法来处理沟通上的困难的？

3.回想最近你和祖辈的一次日常相处(>30分钟)。当时发生了什么事情，在哪里，有哪些人，什么时间，你们都聊了什么？

1. Can you recall the most recent, happiest interaction (>30 minutes) between you and your grandparent. What happened, where, who, when, what did you talk about?

2. Can you recall the most recent, painful/frustrated/anxious interaction (>30 minutes) between you and your grandparent. What happened, where, who, when, what did you talk about? What strategies did you or your family use to manage the communication difficulties?

3. Can you recall the most recent, normal daily interaction (>30 minutes) between you and your grandparent. What happened, where, who, when, what did you talk about?



1.2 Interview guide with adult child

	子辈	Adult child
确诊前的家庭沟通 Family communication before diagnosis	1.先讲一下您父亲/母亲确诊认知障碍症前后的具体情况。 2.你是什么时候发现您父亲/母亲有认知障碍症的症状的？回忆一下自己当时的想法和感受。当时你有和其他家人或者朋友聊过你的想法和感受吗？如果有的话，你是如何开启这个对话的？你们聊了什么，怎么聊的？(提示：地点，时间，家人，话题，感受) 3.是谁决定带父亲/母亲去医院检查诊断？为什么？其他家人的态度是什么？寻求医生帮助的前后都发生了什么？ 4.能说说你和父亲/母亲的关系怎么样嘛？确诊前，你们多久联系一次？是打电话/面对面/视频通话还是用其他方式联络？这个病对你们之间的关系和交流有什么影响？	1. Tell me about what happened when you first learned that your parent was first diagnosed with dementia. 2. When did you first notice your parent having signs of dementia? If you recall, what were you thinking and feeling then? Have you talked to anyone about your thoughts and feelings such as with your family or friends? If so, how did you initiate the talk, and what and how did you talk about them? (Prompt: place, time, people, topic, affect) 3. Who decided to take your parent to the hospital seeking a formal diagnosis? Why? What is the attitude of other family members? What happened before and after seeking medical help? 4. Can you tell me about your relationship with your parent? How often did you communicate with him/her before diagnosis? In-person, by phone, via video call, or other means? How has their diagnosis affected your relationship and communication with him/her?
确诊后的家庭沟通 Family communication after diagnosis	1.确诊后你有什么想法和感受？你有和家人或朋友聊过吗？如果有的话，你是如何开启这个对话的？你和他们聊了什么，怎么聊的？(提示：地点，时间，家人，话题，感受) 2.你的父亲/母亲知道自己有认知障碍症吗？如果知道的话，最初你们家人是怎样告诉他的？如果不知道，你们家人又是如何做出这个决定的？ 3.你或家人是怎样将确诊	1. Can you tell me your thoughts and feelings when you learned about the diagnosis? Have your thoughts and feelings changed? Have you ever talked to your family or friends about this? If so, how did you initiate the talk, and what and how did you talk about them? (Prompt: place, time, people, topic, affect) 2. Does your parent know he/she has dementia? If yes, how did your family inform him/her initially? If not, how and why did your family make this decision?



的消息告诉其他亲人和朋友的？告诉了谁，为什么决定告诉他们，他们的反应又是怎样的？

4.确诊后，有哪些其他家人参与进来？他们都做了什么(探究家庭结构，家庭关系，以及潜在的矛盾)？

5.认知障碍症给父亲/母亲和家人带来了哪些影响或变化？你和父亲/母亲、子女有没有沟通过这些变化(例如病情，家庭关系)？

3. How did you or your family tell other relatives and friends about the diagnosis? Who was told, why did you decide to tell them, and what was their reaction?

4. Who in your family was involved in the disease journey after the diagnosis? What did they do? (Focus on family structures, family relationships, and potential conflict)

5. How has dementia affected or changed your parents and family members? Have you discussed these changes (e.g., illness, family relationships) with your parents and children?

目前,家人之间如何沟通情绪

Currently, how family communicate about emotions

1.回想最近一次，你和父亲/母亲之间最快乐的一次相处(>30 分钟)。当时发生了什么事情，在哪里，有哪些人，什么时间，你们都聊了什么？

2.回想最近一次，你和父亲/母亲之间让你最痛苦/沮丧/焦躁的一次相处(>30 分钟)。当时发生了什么事情，在哪里，有哪些人，什么时间，你们都聊了什么？你或家人是用什么方法来处理沟通上的困难的？

3.回想最近你和父亲/母亲的一次日常相处(>30 分钟)。当时发生了什么事情，在哪里，有哪些人，什么时间，你们都聊了什么？

1. Can you recall the most recent, happiest interaction (>30 minutes) between you and your parent. What happened, where, who, when, what did you talk about?

2. Can you recall the most recent, painful/frustrated/anxious interaction (>30 minutes) between you and your parent. What happened, where, who, when, what did you talk about? What strategies did you or your family use to manage the communication difficulties?

3. Can you recall the most recent, normal daily interaction (>30 minutes) between you and your parent. What happened, where, who, when, what did you talk about?



1.3 Interview guide with grandparent (Knowing the diagnosis)

	祖辈(知晓病情)	Grandparent (Knowing the diagnosis)
确诊前的家庭沟通 Family communication before diagnosis	1.你是什么时候发现自己有认知障碍症的症状的？回忆一下自己当时的想法和感受。当时你有和其他家人或者朋友聊过你的想法和感受吗？如果有的话，你是如何开启这个对话的？你们聊了什么，怎么聊的？(提示:地点，时间，家人，话题，感受) 2.是谁决定带您去医院检查诊断？其他家人的态度是什么？回想一下当时他们是怎么跟您说的，以及您的想法和感受。 3.能说说你和子女/孙辈的关系怎么样嘛？确诊前，你们多久联系一次？是打电话/面对面/视频通话还是用其他方式联络？这个病对你们之间的关系和交流有什么影响？	1. When did you first notice you had signs of dementia? If you recall, what were you thinking and feeling then? Have you talked to anyone about your thoughts and feelings such as with your family or friends? If so, how did you initiate the talk, and what and how did you talk about them? (Prompt: place, time, people, topic, affect) 2. Who took you to the hospital seeking a formal diagnosis? What is the attitude of other family members? If you recall, how did they talk to you? What were you thinking and feeling then? 3. Can you tell me about your relationship with your grandchildren and children? How often did you communicate with them before diagnosis? In-person, by phone, via video call, or other means? How has your diagnosis affected your relationship and communication with them?
确诊后的家庭沟通 Family communication after diagnosis	1.确诊后你有什么想法和感受？你有和家人或朋友聊过吗？如果有的话，你是如何开启这个对话的？你和他们聊了什么，怎么聊的？(提示:地点，时间，家人，话题，感受) 2.最初您的家人是怎样告诉您这个病情的？其他亲人和朋友知道吗？你有没有和他们聊过这个病？ 3.确诊后，您和哪些家人的沟通变多、或者变少了？为什么？ 4.您觉得这个病给您自己和家人带来了哪些影响或变化？您和子女、孙辈有没有沟通过这些变化(例	1. Can you tell me your thoughts and feelings when you learned about the diagnosis? Have your thoughts and feelings changed? Have you ever talked to your family or friends about this? If so, how did you initiate the talk, and what and how did you talk about them? (Prompt: place, time, people, topic, affect) 2. How did your family first tell you about the diagnosis? Do other relatives and friends know? Have you talked to them about the disease? 3. After you were diagnosed, which family members did you communicate with more or less? Why? 4. How do you think dementia has



如病情, 家庭关系)?

affected or changed you and your family? Have you discussed these changes with your children and grandchildren (e.g., illness, family relationship)?

目前, 家人之间如何沟通情绪
Currently, how family communicate about emotions

1. 回想最近一次, 你和家人之间最快乐的一次相处(>30 分钟)。当时发生了什么事情, 在哪里, 有哪些人, 什么时间, 你们都聊了什么?

2. 回想最近一次, 你和家人之间让你最痛苦/沮丧/焦躁的一次相处(>30 分钟)。当时发生了什么事情, 在哪里, 有哪些人, 什么时间, 你们都聊了什么? 你或家人是用什么方法来处理沟通上的困难的?

3. 回想最近你和家人的一次日常相处(>30 分钟)。当时发生了什么事情, 在哪里, 有哪些人, 什么时间, 你们都聊了什么?

1. Can you recall the most recent, happiest interaction (>30 minutes) between you and your family. What happened, where, who, when, what did you talk about?

2. Can you recall the most recent, painful/frustrated/anxious interaction (>30 minutes) between you and your family. What happened, where, who, when, what did you talk about? What strategies did you or your family use to manage the communication difficulties?

3. Can you recall the most recent, normal daily interaction (>30 minutes) between you and your family. What happened, where, who, when, what did you talk about?



1.4 Interview guide with grandparent (Not knowing the diagnosis)

	祖辈(不知晓病情)	Grandparent (Not knowing the diagnosis)
确诊前的家庭沟通 Family communication before diagnosis	1.日常生活中您最喜欢和子女/亲人/孙辈聊些什么? 会和哪些家人或朋友分享自己的想法和感受? 2.能说说你和子女/孙辈的关系怎么样嘛? 你们多久联系一次? 是打电话/面对面/视频通话还是用其他方式联络? 现在和过去有什么不同? 3.最近有觉得自己记忆力下降了吗? 有没有和家人或朋友聊起这件事? 如果有的话, 你是如何开启这个对话的?你们聊了什么, 怎么聊的? (提示: 地点, 时间, 家人, 话题, 感受) 4.有没有打算看医生检查一下记忆问题?	1. What do you like to talk about with your children/relatives/grandchildren in daily life? Which family members or friends do you share your thoughts and feelings with? 2. Can you tell me about your relationship with your grandchildren and children? How often do you communicate with them? In-person, by phone, via video call, or other means? What is the difference between current and past communication? 3. Have you ever felt that your memory has declined recently? Have you talked to anyone about your thoughts and feelings such as with your family or friends? If so, how did you initiate the talk, and what and how did you talk about them? (Prompt: place, time, people, topic, affect) 4. Have you planned to see a doctor and screen your memory problems?
确诊后的家庭沟通 Family communication after diagnosis	1.您觉得记忆力下降您自己和家人带来了哪些影响或变化? 您和子女、孙辈有没有沟通过这些变化?	1. How do you think your declined memory has affected or changed you and your family? Have you discussed these changes with your children and grandchildren?
目前, 家人之间如何沟通情绪 Currently, how family communicate about emotions	1.回想最近一次, 你和家人之间最快乐的一次相处(>30 分钟)。当时发生了什么事情, 在哪里, 有哪些人, 什么时间, 你们都聊了什么? 2.回想最近一次, 你和家人之间让你最痛苦/沮丧/焦躁的一次相处(>30 分钟)。当时发生了什么事情, 在哪里, 有哪些人, 什么时间, 你们都聊了什	1. Can you recall the most recent, happiest interaction (>30 minutes) between you and your family. What happened, where, who, when, what did you talk about? 2. Can you recall the most recent, painful/frustrated/anxious interaction (>30 minutes) between you and your family. What happened, where, who, when, what did you talk about? What strategies did you or your family use to

么？你或家人是用什么方法来处理沟通上的困难的？

3.回想最近你和家人的一次日常相处(>30 分钟)。当时发生了什么事情，在哪里，有哪些人，什么时间，你们都聊了什么？

manage the communication difficulties?

3. Can you recall the most recent, normal daily interaction (>30 minutes) between you and your family. What happened, where, who, when, what did you talk about?



Appendix II: Ethics approval

THE UNIVERSITY OF HONG KONG



August 7, 2023

Miss Shicheng XU
MPhil Student
Department of Social Work and Social Administration

Dear Miss XU,

Application for Ethics Approval
HREC's Reference Number: EA230303

I refer to your application for ethics approval of your project entitled "Communicating dementia across three generations within Chinese families".

2. I am pleased to inform you that the application has been approved by the Human Research Ethics Committee (HREC) regarding the ethical aspect of the above-mentioned research project, and the expiration date of the ethical approval is August 6, 2027.

3. Please be reminded of the following points concerning the approved project:

- (a) The HREC reference number of your project (EA230303) has to be shown in all materials sent to potential and actual participants to enable participants to link the materials to an approved project.
- (b) You should report to the HREC any amendments and new information on the project using the prescribed form ('Application for Amendment of an Approved Project' downloadable from the Research Services website (<http://www.rss.hku.hk/integrity/ethics-compliance/hrec>)). Application for extension should be submitted well before the initially approved expiration date.
- (c) Any deviation from the study protocol or compliance incident that has occurred during a study and may adversely affect the rights, safety or well-being of any participant or breaches of confidentiality should be reported to the HREC within 15 calendar days from the first awareness of the deviation/incident by the Principal Investigator.

Yours sincerely,

Secretary
Human Research Ethics Committee

THE REGISTRY 教務處
POKFULAM ROAD, HONG KONG. TEL: (852) 2859 2111 FAX: (852) 2858 2549

